



Republic of the Philippines
DEPARTMENT OF HEALTH

2027 BUDGET
BUDGET DELIBERATIONS

Bureau of Local Health Systems Development

Local Health Systems Development and Assistance

SUMMARY

BUDGET CUE CARDS

Local Health Systems Development Assistance

OVERVIEW OF THE 2027 NEP BUDGET PROPOSAL

662 M 1,383 M 1,497 M ▲ +6.5%

2025 GAA

2026 GAA

2027 NEP

*Compute the % change from the
2026 GAA to the 2027 NEP.*

[Provide brief description/s on what the total proposal hopes to achieve and how it supports the overall health goals.]

Brief Description on the total proposal:

To provide the Local Government Units and the Local Health Boards with technical assistance to strengthen their local health systems pursuant to Republic Act 7160 and to assist the 110 Universal Health Care Integration Sites (UHC-IS) in the integration of their local health systems into Province-Wide and City-Wide Health Systems as mandated by Republic Act No. 11223.

To provide support in the implementation of the 2025-2028 Indigenous People's Health Strategic Plan, reduce inequity in the delivery and access to primary care services in Unserved and Underserved Communities/IP and provide support including capacity development of BHWs as Health Promotion Officers on the ground.

To provide support for the delivery of Primary Care Services in the community through capacity building and logistical support.

Local Health Systems Development and Assistance

BREAKDOWN OF THE 2027 NEP BUDGET

CENTRAL OFFICE PROPOSAL in '000 PESOS

PS	25,655	MOOE	104,187	CO	XXX	TOTAL	129,842
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CHD PROPOSALS in '000 PESOS

PS	XX	MOOE	1,367,840	CO	XXX	TOTAL	1,367,840
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TOTAL NEP PROPOSAL PER EXPENSE CLASS in '000 PESOS

PS	25,655	MOOE	1,472,027	CO	XXX	TOTAL	1,497,682
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Local Health Systems Development and Assistance

OVERVIEW OF THE 2027 BUDGET PROPOSAL

1.434B

TIER 1 PROPOSAL

4.103B

TIER 2 PROPOSAL

5.537B

TOTAL PROPOSAL

TOTAL PROPOSAL ('000)
CENTRAL OFFICE

PER EXPENSE CLASS
CENTRAL OFFICE

394,315		25,655	PS		369,315	MOOE		0	CO
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TOTAL PROPOSAL ('000)
CHDs

PER EXPENSE CLASS
CHDs

5,142,558B			PS		5,142,558	MOOE		0	CO
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Local Health Systems Development and Assistance

► PHYSICAL TARGETS & ACCOMPLISHMENTS

INDICATORS	2025		2026	2027
	TARGET	ACCOMP	TARGET	TARGET
Number of UHC Integration Sites that Achieved the Target Number of Key Result Areas for a Particular Level in the Local Health Systems Maturity Levels	At least 58 UHC IS achieved at least 90% of Level 3 LHSML KRAs	31 UHC IS achieved at least 90% of the Level 3 KRAs <i>58 UHC IS achieved at least 70% of Level 3 KRAs</i>	At least 58 UHC IS achieved at least 90% of Level 3 LHSML KRAs	At least 58 UHC IS achieved at least 90% of Level 3 LHSML KRAs
Percent of Partners with Fulfilled Technical Assistance Agenda	100%	452/460 (98.26%)	100%	100%
Number/Percentage of identified PK sites	4,926 brgys	8,432 brgys	16,804 brgys	29,408 of brgys
Number of organized PK teams	1:1 bgry to PK team ratio	8,432 teams	1:1 bgry to PK team ratio	1:1 bgry to PK team ratio

Note: Accomplishments are reported as of 31 December 2025.

PHYSICAL & FINANCIAL TARGETS & ACCOMPLISHMENTS

► ABSORPTIVE CAPACITY

662 M

2025 Current Allotment



94% (620 M)

Obligation Rate



78% (484 M)

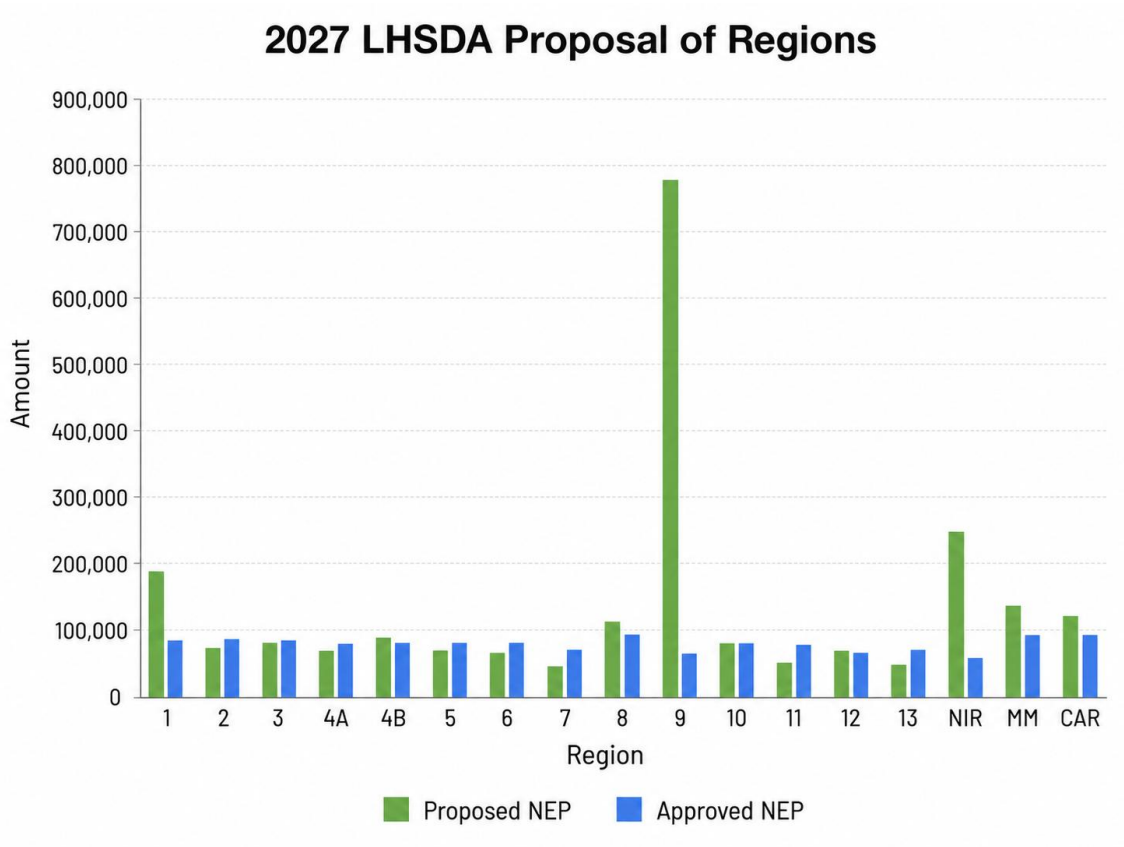
Disbursement Rate



Note: Data presented are current as of 31 December 2025.

Local Health Systems Development and Assistance

SUMMARY OF FY 2027 CHD BUDGET PROPOSALS



PROBLEM DEFINITION

The proposal aims to provide technical assistance to LGUs and LHBs to strengthen their LHS in accordance to RA 7160 particularly the 109 UHC IS in integrating their LHS into Province/City-wide Health Systems as mandated by RA 11223. Additionally, the proposal also supports the flagship program Purokalusugan by providing incentives, capacity building and kits to Barangay Health Workers who are the frontline health workers serving the communities including unserved and underserved areas and vulnerable groups such as Indigenous People/Indigenous Cultural Communities

TIER 1 PROPOSAL

Budget	Proposed Activity / Intervention	Expected Beneficiaries and Target Coverage
214 M	UHC Grants for LGU including Fixed Tranche	110 UHC IS and 120 LGUs
1.2B	Support for CHD and PCDOHOs in Health Systems Strengthening: LIPH Development, UUC for PHC, IP, BHWs, PuroKalusugan, LGU HSC, Good Practices in Health	18 CHDs/MOH-BARMM

TIER 2 PROPOSAL

Budget	Proposed Activity / Intervention	Expected Beneficiaries and Target Coverage
64M	Support to P/CDOHO and local health financing activities; UHC Summit	17 CHDs
200M	UHC Awards	LGUs
3.8B	Support for BHWs in PuroKalusugan	277,767 BHWs 70% of barangays (29,409 PK Sites)

PROBLEM DEFINITION

The proposal aims to provide technical assistance to LGUs and LHBs to strengthen their LHS in accordance to RA 7160 particularly the 109 UHC IS in integrating their LHS into Province/City-wide Health Systems as mandated by RA 11223. Additionally, the proposal also supports the Primary Care service delivery by providing incentives, capacity building and kits to Barangay Health Workers who are the frontline health workers serving the communities including unserved and underserved areas and vulnerable groups such as Indigenous People/Indigenous Cultural Communities.

NEP-BASED PROPOSAL

Budget	Proposed Activity / Intervention	Expected Beneficiaries and Target Coverage
1.4 B	Support for LGU, CHD and PCDOHOs in Health Systems Strengthening: LIPH Development, UUC for PHC, IP, BHWs, PuroKalusugan, LGU HSC, Good Practices in Health	17 regions, including their respective LGUs and BHW Federations
41 M	Fund transfer to BARMM (Fixed Tranche of Provinces 14M, Financial Grants for UHC-IS 6M, BHWs incentive, IP Health 3M, LGU Scorecard 2M and Primary Care Service Delivery Support 16M	MOH-BARMM including their respective LGUs and BHW Federations
10 M	Conduct of UHC Summit	110 UHC Integration Sites, NGAs, CSO
12 M	DOH Representatives Convention	538 DOH Reps, LHSDA Chiefs and Program Managers from 17 CHDs and MOH-BARMM

NEP-BASED PROPOSAL

Budget	Proposed Activity / Intervention	Expected Beneficiaries and Target Coverage
25.7 M	Salary of permanent BLHSD Staff	26 permanent staff
41 M	Orientations, Workshops, Consultations of different LHS focus areas, Salary of COS, other operating expenses	17 CHDs and MOH-BARMM and other stakeholders on local health systems

Local Health Systems Development and Assistance

NEP-BASED PROPOSAL

Region	Amount	Region	Amount
Metro Manila	93 M	NIR	58.4 M
CAR	94 M	Central Visayas	70 M
Ilocos	85.8 M	Eastern Visayas	92 M
Cagayan Valley	87 M	Zamboanga Pen.	72.7 M
Central Luzon	83.9 M	Northern Mindanao	79.9 M
CALABARZON	78.5 M	Davao	77.5 M
MIMAROPA	84 M	SOCCKSARGEN	70 M
Bicol	82.7 M	CARAGA	73 M
Western Visayas	83 M	Support for BARMM	41 M

MAJOR INSIGHTS

[Provide brief description on the components of regional proposal and the distribution of funds (or why the regions received the same allocation/why a particular region received the highest allocation)]

The allocation will be used for the following: grants for UHC IS, performance-based incentives for BHW Federations, support for DOH Representatives, support to CHD for the integration of local health systems, LIPH/AOP development/monitoring, local health financing, implementation of other local health systems focus areas such as unserved and underserved communities, IP/ICC, LGU HSC, Good Practices in Health and support for primary care service delivery.

The regional allocation is based on the number of LGUs, number of UHC Integration Sites, number of unserved and underserved communities, fund utilization rate, presence of IP/ICC and number of DOH Representatives in the region.

Local Health Systems Development and Assistance

IMPLEMENTATION CHALLENGES & WAYS FORWARD

IMPLEMENTATION CHALLENGES	WAYS FORWARD
Increase in PK target to 70% of barangays in 2027	Included in Tier 2 proposal
DOH Rep (Support to PCDOHO) budget used for utilities, rental, internet subscription, etc.	Transfer to GMS

Local Health Systems Development and Assistance

FOLLOW THROUGH FROM CSO CONSULTATION

CSO RECOMMENDATION	PLAN OF ACTION	ACTIONS TAKEN
Prioritize passage of Magna Carta of BHWs	Magna Carta of BHWs is a priority bill of DOH and is included in the HEAL. Active participation in Senate/Congressional hearings. Submission of position paper.	Attended Senate hearing on Magna Carta. Submitted position paper and data on BHW incentives and demographic profile.

FINANCIAL BUDGET CUE CARDS

BLHSD

LOCAL HEALTH SYSTEMS DEVELOPMENT AND ASSISTANCE

BUDGET
TRENDS

► COMPARISON OF BUDGET ALLOCATIONS, 2022-2027 (IN THOUSAND PESOS)

YEAR	ALLOCATION			
	PS	MOOE	CO	TOTAL
2022 GAA	11,025	492,880	0	503,905
2023 GAA	11,318	600,592	170,000	781,910
2024 GAA	12,502	616,532	0	629,034
2025 GAA	24,493	637,688	0	662,181
2026 GAA	23,297	1,383,131	0	1,406,428
2027 NEP	25,655	1,472,027	0	1,497,682

► COMPARISON OF 2025 GAA, 2026 GAA AND 2027 NEP BY EXPENSE CLASS (IN THOUSAND PESOS)

EXPENSE CLASS		2025 GAA A	2026 GAA B	2027 NEP C	% CHANGE D = (C - B)/B
	PS	24,493	23,297	25,655	.090%
	MOOE	637,688	1,383,131	1,472,027	6.5%
	CO	0	0	0	0
TOTAL		662,181	1,406,428	1,497,682	1%

LOCAL HEALTH SYSTEMS DEVELOPMENT AND ASSISTANCEORIGINAL
BUDGET PROPOSAL

► COMPARISON OF 2027 BUDGET PROPOSAL AND 2027 NEP BY EXPENSE CLASS (IN THOUSAND PESOS)

EXPENSE CLASS	2027 TOTAL BUDGET PROPOSAL A	2027 NEP B	% CHANGE $C = (B - A) / A$
 PS	0	25,655	0%
 MOOE	4,103,846	1,472,027	-64%
 CO	0	0	0
TOTAL	4,103,846	1,497,682	34%

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

ABSORPTIVE
CAPACITY

► BUDGET UTILIZATION PERFORMANCE, 2025–2026 (IN THOUSAND PESOS)

YEAR	BUDGET ALLOTMENT	OBLIGATION	OBUR	DISBURSEMENT	DBUR
2025 GAA As of Dec. 31, 2025	662,181	620,261	94%	484,257	78%
2025 GAA with ConAp As of June 30, 2026	40,229	33,218	82%	18,199	55%
2026 GAA As of June 30, 2026	1,406,428	449,333	32%	206,453	46%

Note: Data presented shall be based on data from FMS.

Name of Budget Line Item

FY 2025 GAA FINANCIAL ACCOMPLISHMENTS

► ABSORPTIVE CAPACITY

662M

2025 GAA Allotment

94% (620M)

Obligation Rate

78% (484 M)

Disbursement Rate

[Provide brief description of 2025 GAA budget utilization performance as of December 31, 2025, reasons for low obligation and disbursement (if unmet), and ways forward]

- **NARRATIVE:**
- **REASONS FOR LOW OBLIGATION** (if below 95%):
 - Activities will be conducted on the 3rd and 4th quarter of 2024
 - Unliquidated previous transfers (fixed tranche)
 - Still on process of liquidation (UHC-IS Grant)
 - TOPs/MOAs still in process to download Fixed tranche, GPH incentive and PBI to LGUs.
- **REASONS FOR LOW DISBURSEMENT** (if below 85%):
 - DV/Payment for some activities conducted are still on process
 - Awaiting other documents needed prior downloading of remaining grants
 - Some obligated activities are scheduled in Q4.
- **WAYS FORWARD:**
 - For Coordination with LGUs for their Liquidations of the Fund Transfer
 - Implement close monitoring and weekly tracking of fund status to ensure targets are met within the remaining period.

Note: Data presented are as of 31 December 2025 and **shall be based on data from FMS.**

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

FY 2025 CONTINUING APPROPRIATIONS

FINANCIAL ACCOMPLISHMENTS

► **ABSORPTIVE CAPACITY**



[Provide brief description of 2025 CONAP budget utilization performance as of June 30, 2026, overall status on procurement, and ways forward]

- **DESCRIPTION**
- **STATUS OF 2025 CONAP PROCUREMENT ACTIVITIES**

STATUS	AMOUNT (in '000)	TARGET COMPLETION DATE	JUSTIFICATION
Completed/Fully-Delivered			
Ongoing implementation			
Undergoing Bidding			
Not yet started			

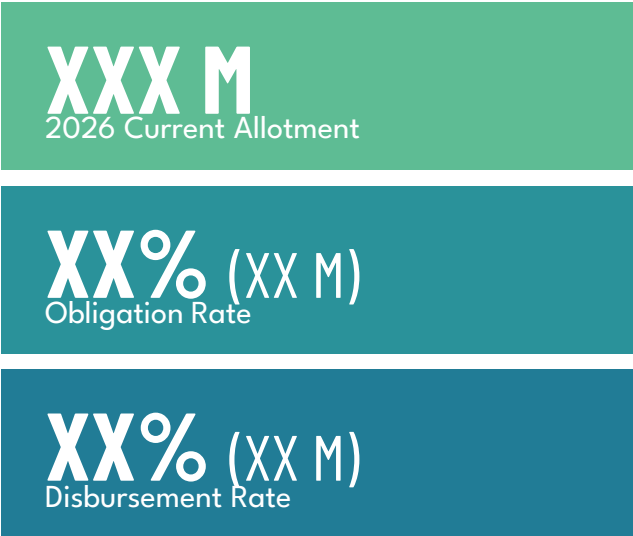
- **WAYS FORWARD:**

Note: Data presented are as of 30 June 2026 and shall be based on data from FMS.

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

► **ABSORPTIVE CAPACITY**



[Provide brief description of 2026 GAA budget utilization performance as of June 30, 2026, overall status on procurement, and ways forward]

- DESCRIPTION

- STATUS OF 2026 PROCUREMENT ACTIVITIES

STATUS	AMOUNT (in '000)	TARGET COMPLETION DATE	JUSTIFICATION
Completed/Fully-Delivered			
Ongoing implementation			
Undergoing Bidding			
Not yet started			

- WAYS FORWARD:

Note: Data presented are as of 30 June 2026 and shall be based on data from FMS.

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

BUDGET ALLOCATION
BY PROGRAM COMPONENT

► COMPARISON OF BUDGET ALLOCATION BY PROGRAM COMPONENT: 2026 GAA VS. 2027 PROPOSAL (IN THOUSAND PESOS)

COMPONENT	2026 GAA A	2027 NEP B	% CHANGE C = (B - A)/A
[Program Component 1]	XX	XX	XX
[Program Component 2]	XX	XX	XX
[Program Component 3]	XX	XX	XX
TOTAL	XX	XX	XX

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

DETAILED BREAKDOWN BY
PROGRAM COMPONENT

► DETAILED BREAKDOWN BY PROGRAM COMPONENT BASED ON 2027 PROPOSAL: INSERT PROGRAM COMPONENT

PARTICULARS	BENEFICIARIES	NO. OF BENEFICIARIES	QUANTITY	UNIT COST	TOTAL
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
XX	XX	XX	XX	XX	XX
TOTAL					XX

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

PROPOSED SPECIAL PROVISIONS

► SPECIAL PROVISIONS

TITLE	2026 GAA VERSION	2027 VERSION (Highlight any notable differences)	STATUS OF 2026 SPECIAL PROVISION
XX	XX	XX	XX
XX	XX	XX	XX
XX	XX	XX	XX



Republic of the Philippines
DEPARTMENT OF HEALTH

2027 BUDGET
BUDGET DELIBERATIONS

Reference slides

You may also prepare additional cue cards covering other relevant data/issues that your office anticipates may arise during the budget deliberations.

FAQs

1. Barangay Health Workers -

a. What kind of support is provided by DOH for BHWs?

DOH's plans for improving barangay-level healthcare:

- The DOH's Universal Health Care (UHC) strategy focuses on strengthening Primary Health Care (PHC) as the foundation of community health.
- Emphasis on capacity-building of BHWs, including:
 - Standardized training and certification
 - Continuous professional development through manuals and inter-agency programs (TESDA, CHED)
- Expansion of community health services through strategies such as PuroKalusugan, which delivers comprehensive health services directly at the barangay/purok level.
- Services cover disease surveillance, health promotion, elderly care, mental health, nutrition, and Philhealth programs.
- Leveraging technology to reach Unserved and Underserved Communities for Primary Health Care, including offline training modules on mobile devices and tablets using the BHW Connect system as platform
- If passed into law, implementation of the Magna Carta for BHWs, ensuring standardized benefits, security of health services, and sufficient staffing to improve coverage and quality of care.

FAQs

1. Barangay Health Workers -
B. How many BHWs are there
 - 277,767 (Registered and Accredited BHWs)
 - Source: BHW Connect as of Dec 2025

Note: as of this date the BHW connect web site under system hold by KMITs due to security threat

DOH estimated budget allocation for honorarium under the Magna Carta for BHWs:

Income Class	No of BHWs	Honorarium/mo	Allocation/mo	Total/annum
3rd	35,645	3,000	106,935,000	1,283,220,000
4 th	27058	3,000	81,174,000	974,088,000
5th	7541	3,000	22,623,000	271,476,000
Total	70,244		210,732,000	2,528,784,000

The proposed bill, will mandate the DOH to support the provision of honorarium for BHWs in 3rd, 4th, and 5th income class municipalities, to be included in the General Appropriations Act (GAA) of the DOH budget, as approved by the Department of Budget and Management. Specific allocations for other responsibilities and programs will be detailed in the Implementing Rules and Regulations (IRR)

INSERT NAME OF OFFICE HERE

FAQs

Name of Budget Line Item



Q1

[Insert frequently asked question here]

A1

[Insert clear and concise answer here]

FAQs

1. Barangay Health Workers -
 - a. What kind of support is provided by DOH for BHWs?
 - b. How many BHWs are there
2. Special Health Fund
3. LHS-ML

INSERT NAME OF OFFICE HERE

FAQs

Name of Budget Line Item



Q1

[Insert frequently asked question here]

A1

[Insert clear and concise answer here]

INSERT NAME OF OFFICE HERE

FAQs

Name of Budget Line Item



Q1

[Insert frequently asked question here]

A1

[Insert clear and concise answer here]

LEADERSHIP AND GOVERNANCE FOR HEALTH (LEADGOV4HEALTH)

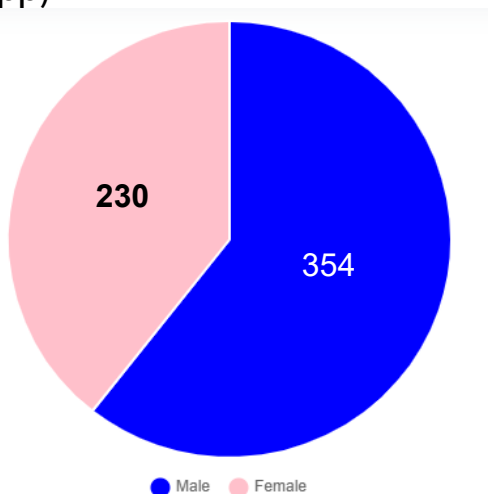
Q1

Number of DOH Representatives by Sex and Position

A1

There are 584 DOH Representatives as of April 30, 2026 (Source: DOH Rep Assistant App)

Assistant App)



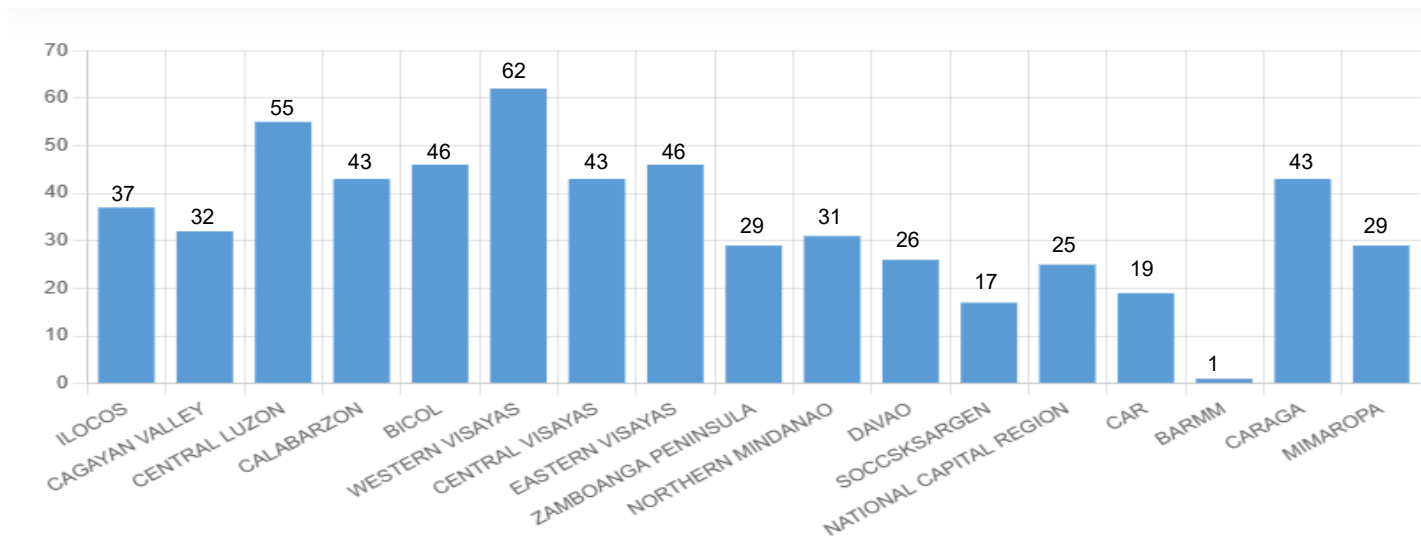
Distribution of DOH Representatives by Sex

Distribution of DOH Representatives by Position

LEADERSHIP AND GOVERNANCE FOR HEALTH (LEADGOV4HEALTH)

Q2
A2

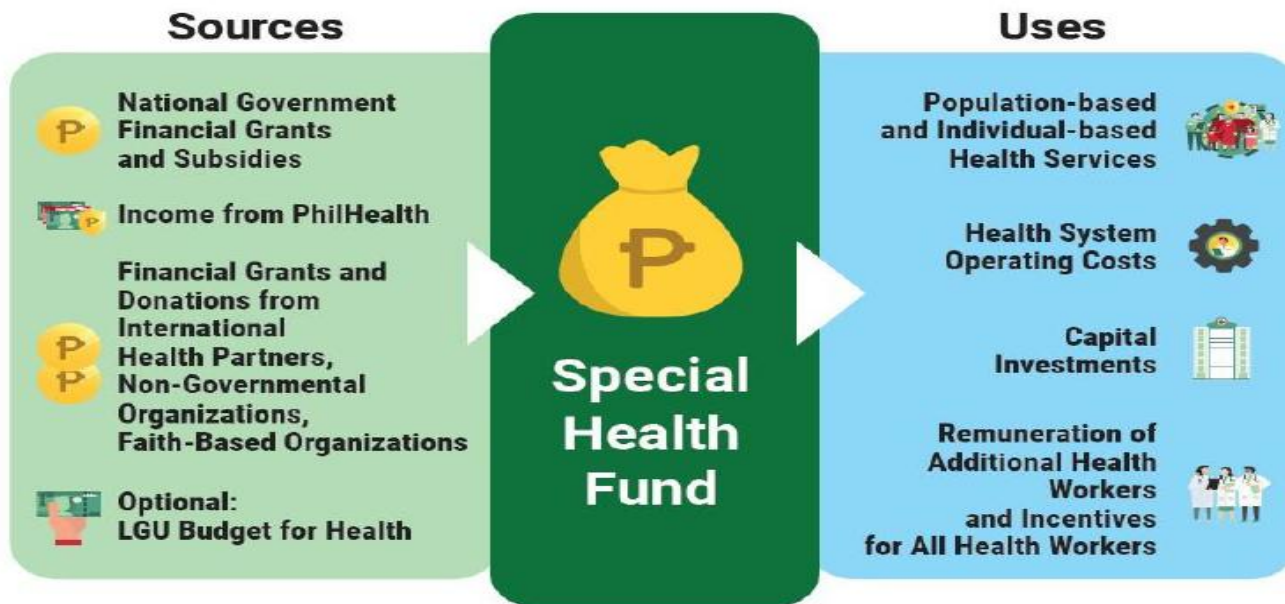
Number of DOH Representatives by Regional Distribution



LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

Q1

Sources of Funds and Uses of SHF



LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

Q2 Allowable and Non-allowable Expenditures for SHF

Allowable Expenditures and Transactions for SHF

1. Population-based health services
2. Individual-based health services
3. Health Systems Operating Costs
4. Capital investments based on health facility development plan
5. Remuneration of additional health workers until such time that the LGU implemented incremental creation of plantilla positions
6. Incentives for all health workers, including BHWs, Bgy. Nutrition Scholars

Expenses Chargeable Against the LGU General Fund (Not pooled in SHF)

1. Remuneration of existing human resources for health personnel under **plantilla** positions
2. Petroleum, oil, and lubricants of **vehicles not intended** for transporting patient, commodities, specimen, and **not used** for health service delivery
3. Payment for utilities including but not limited to electricity, water, telephone, internet, and other communication services
4. Monthly rentals or purchase of office for the LHB, MSU, and health offices/facility
5. Purchase of office supplies not exclusive for health service delivery
6. Land acquisition and land development for the construction/upgrading of health facilities

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

Q3 Why can't the local health facilities operating as Local Economic Enterprise (LEE) utilize the SHF?

- An LEE is a venture wholly or partially owned by the LGU that generates income by providing goods and services to the public. It aims to generate revenue to support LGU functions, fund public services, and create self-sustaining operations.
- Section 325(a) of the LGC provides that the appropriations for salaries, wages, representation and transportation allowances of LEE personnel shall not be included in the LGU annual budget. Their personal services (PS) shall be charged to the respective budgets of the LEEs.
- As per UHC Act **Section 21**, Philhealth income shall accrue to the SHF of the P/CWHS and shall be used for remuneration of **additional** health workers
- Since many health facilities operating as LEEs rely on PhilHealth payments as the main source of their budget, the accrual of PhilHealth payments to the SHF would affect the PS expenditures and disrupt the operations of the LEE.

List of 2025 Unserved and Underserved Areas (UUA)

Distribution of UUC for PHC Barangays by Region (as of 2025 per DC No. 2025-0549)

PROGRAM PROFILE

SUMMARY	
REGION	NUMBER
BANGSAMORO AUTONOMOUS REGION IN MUSLIM MINDANAO (BARMM)	400
CORDILLERA ADMINISTRATIVE REGION (CAR)	609
REGION I (ILOCOS REGION)	262
REGION II (CAGAYAN VALLEY)	227
REGION III (CENTRAL LUZON)	129
REGION IV-A (CALABARZON)	195
MIMAROPA REGION	458
REGION V (BICOL REGION)	757
REGION VI (WESTERN VISAYAS)	397
REGION VII (CENTRAL VISAYAS)	82
REGION VIII (EASTERN VISAYAS)	358
NEGROS ISLAND REGION (NIR)	141
REGION IX (ZAMBOANGA PENINSULA)	523
REGION X (NORTHERN MINDANAO)	440
REGION XI (DAVAO REGION)	437
REGION XII (SOCCSKSARGEN)	304
REGION XIII (CARAGA)	268
TOTAL	5987

TECHNICAL BUDGET CUE CARDS

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

PROGRAM PROFILE



PROGRAM/ACTIVITY/PROJECT DESCRIPTION

Briefly describe the PAP, including its scope, target beneficiaries, and key components.



OBJECTIVES AND INTENDED OUTCOMES

Clearly state the primary goal or intended outcome.

INSERT NAME OF OFFICE HERE

PROGRAM PROFILE

Name of Budget Line Item



LEGAL AND POLICY BASIS

Cite the relevant laws, issuances, policies, or frameworks that mandate or support the implementation of the PAP (e.g., RA, EO, DOH AO, UHC Law, SDG, PDP, etc.).

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

PROGRAM PROFILE



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

Describe the major steps, workflows, or mechanisms involved in implementing the PAP. Include details such as planning cycles, coordination with partners or stakeholders, fund flow, monitoring processes, or reporting arrangements.

For the following PAPs, but not limited to those listed below, kindly include the procedures on how individuals can access or avail of the services:

- **DPCB: Cancer Assistance Fund**
- **MPO: Medical Assistance for Indigent and Financially-Incapacitated Patients**
- **HHRDB: DOH Pre-Service Scholarship Program**
- **HFSRB: Regulation of Health Facilities and Services**
- **FDA: Regulation of Health Establishments and Products**
- **BOQ: Provision of Quarantine Services and International Health Surveillance**

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

PROGRAM PROFILE



BASIS OF PRIORITIZATION OR TARGETING

Explain how areas, populations, or activities were selected (e.g., disease burden, equity, vulnerability, existing data).

INSERT NAME OF OFFICE HERE

Name of Budget Line Item

PHYSICAL TARGETS & ACCOMPLISHMENTS

► **PHYSICAL TARGETS & ACCOMPLISHMENTS**

INDICATORS	2025		2026		2027
	TARGET	ACCOM P	TARGET	ACCOM P	TARGET
[Insert Major Indicator 1]	XX	XX	XX	XX	XX
[Insert Major Indicator 2]	XX	XX	XX	XX	XX
[Insert Major Indicator 3]	XX	XX	XX	XX	XX

Note: Accomplishments are reported as of 30 June 2026.

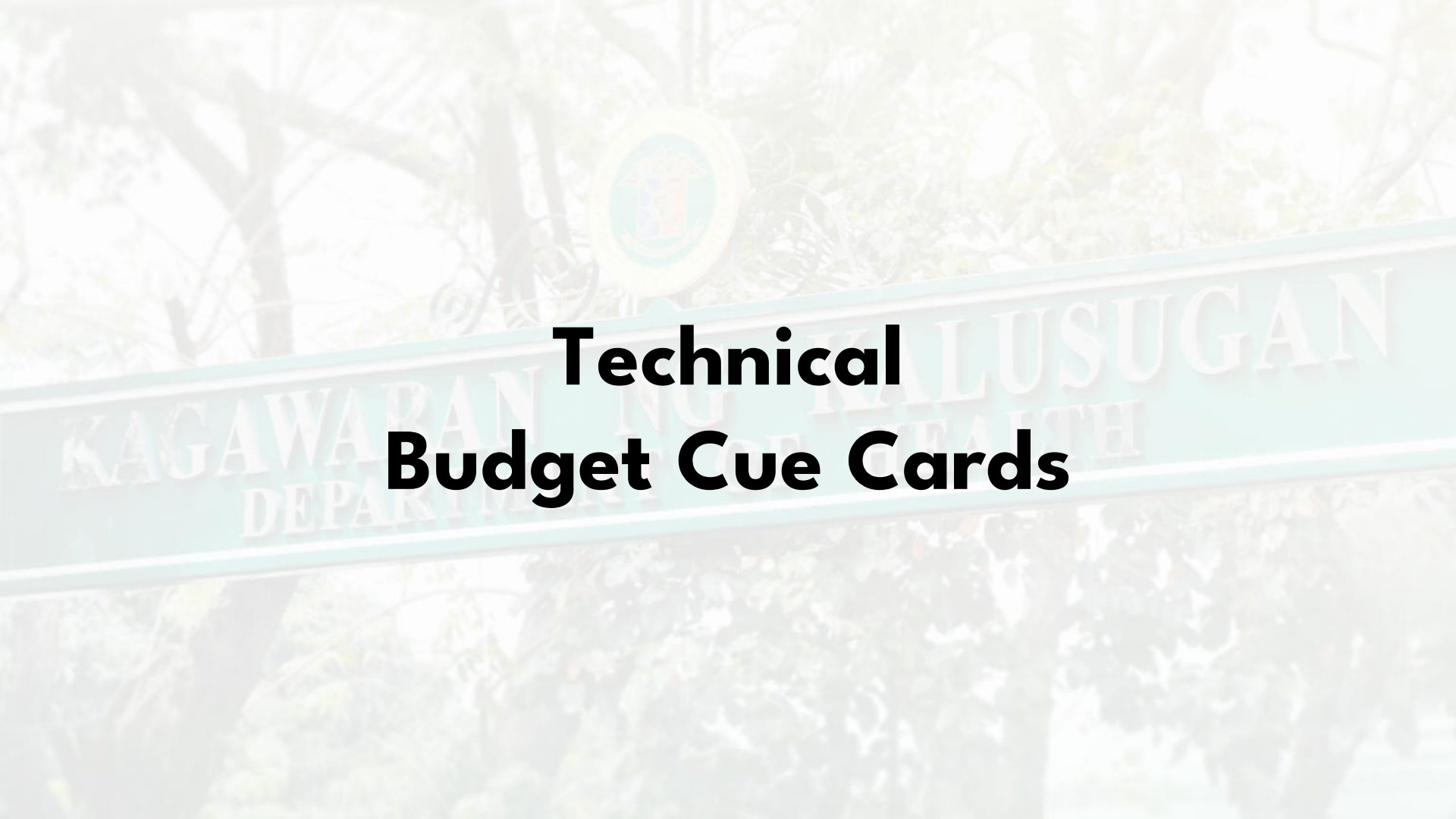
[Provide brief description/s of the targets and accomplishments. Provide justification on unmet targets, if any]



Summary Slides

Local Health Systems Development and Assistance

A dedicated fund designed to support the strengthening and development of local health systems. It provides financial and technical assistance to local government units to improve the delivery of primary care services, enhance health governance and local planning, monitor local health systems performance and facilitate the implementation of Universal Health Care (UHC) initiatives at the community level.



Technical Budget Cue Cards

PROGRAMS

1. Local Health Systems Integration

- a. Local Health Systems Maturity Level and Information System
- b. Health Care Provider Network (HCPN)

2. Local Health Planning and Financing

- a. Local Investments Plan for Health/ Annual Operational Plan (LIPH/AOP) and Information System
- b. Special Health Fund
- c. LGU Health Accounts

3. Leadership and Governance for Health (LeadGov4Health)

- a. Support for Local Health Boards
- b. Support for DOH Representatives

PROGRAMS

4. Equity for Health and Special Concerns

- a. PuroKalusugan
- b. Indigenous Peoples' Health
- c. Unserved and Underserved Communities for Primary Health Care (UUC for PHC)) *(formerly known as Geographically Isolated and Disadvantaged Areas (GIDA) and Socio-Economically Disadvantaged Areas (Urban Poor))*
- d. Support for Barangay Health Workers

5. LGU Performance and Monitoring

- a. LGU Health Scorecard
- b. Seal of Good Local Governance (Health Compliance and Responsiveness Area)
- c. Good Practice in Health and Replication

LOCAL HEALTH SYSTEMS INTEGRATION

PROGRAM PROFILE

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

- To provide the general mechanisms by which the LGUs (provinces, cities and municipalities) can integrate their local health systems (LHS) into Province-wide and City-wide Health System (P/CWHS)
- Serve as the general framework in the monitoring and evaluation of the progress of the LGUs that committed to the LHS integration, and serve as guide in prioritizing resources and support
- To be used in complementation with the LGU Health Scorecard and other existing DOH monitoring and evaluation systems

► OBJECTIVES AND INTENDED OUTCOMES

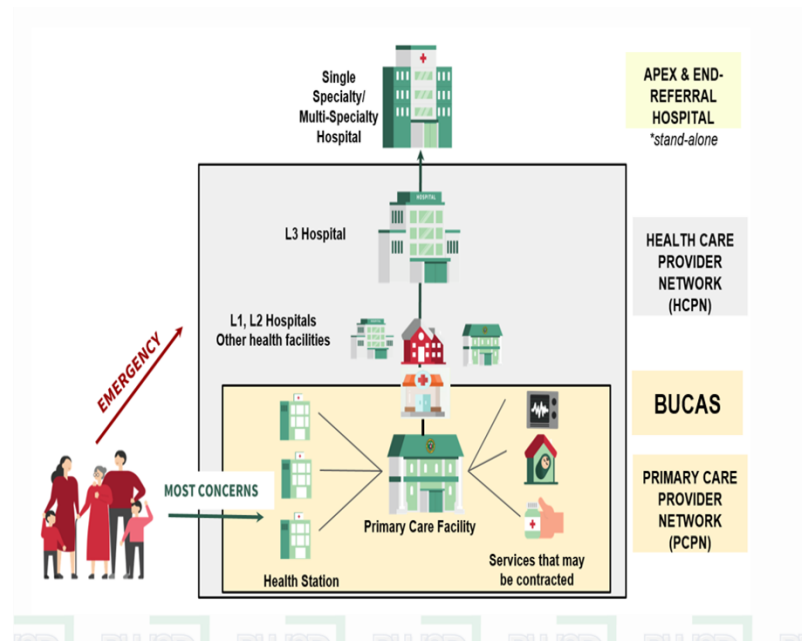
- LGUs that have committed to integrate their local health systems into P/CWHS to exhibit **managerial** and **technical** integration in **2020-2022** and **financial** integration in **2023-2025**
- By **2026**, issuance of an Executive Order (EO) mandating the integration of all local health systems upon the positive recommendation of an independent study commissioned by the Joint Congressional Oversight Committee and positive recommendation of the SOH

LOCAL HEALTH SYSTEMS INTEGRATION

PROGRAM PROFILE

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

- **Managerial Integration** - refers to the consolidation of administrative, technical and managerial functions of the PWHS/CWHS over its resources such as health facilities, human resources for health, health finances, health information systems, health technologies and equipment and supplies
- **Technical Integration** - the functional and efficient linking of health service provision from primary to tertiary care, when appropriate, across different levels of facilities, care settings, across a comprehensive spectrum of care.
- **Financial Integration** - consolidation of financial resources, through the **Special Health Fund**, and its utilization exclusively for health services and health system development under a single planning and investment strategy by the P/CWHS.



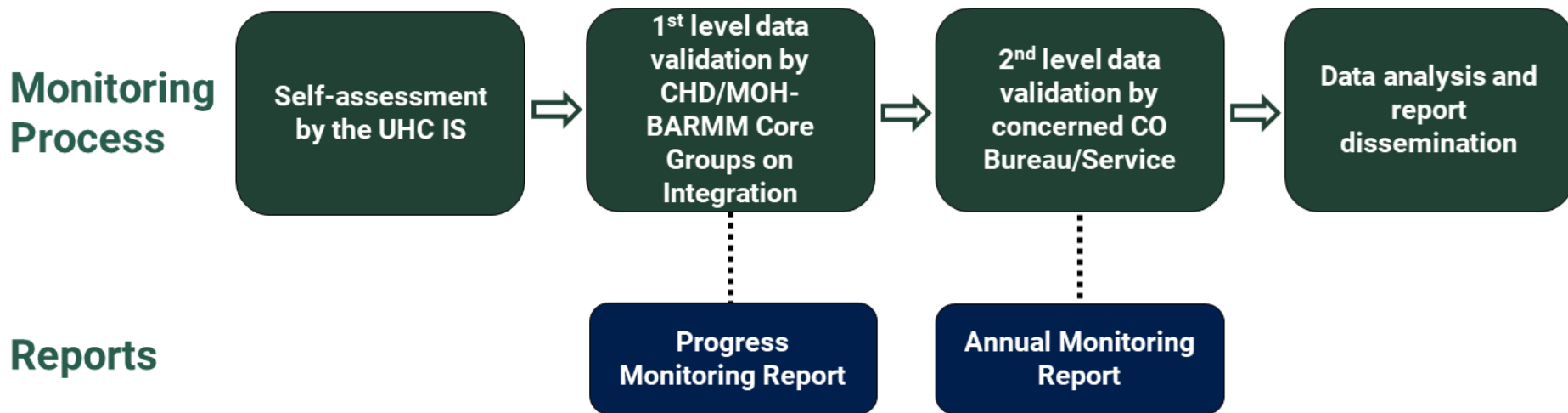


LEGAL AND POLICY BASIS

- Section 33, Article III of the Local Government Code (RA 7160)
- Section 19, Chapter V of the Universal Health Care Act, *‘Integration of Local Health Systems into Province-wide and City-wide Health Systems (P/CWHS)’*
- Section 41(d), Chapter XI of the Universal Health Care Act, *“In the first six (6) years from the enactment of this Act, the National Government Shall provide technical and financial support to LGUs that commit to province-wide integration”*
- AO No. 2020-0021, *“Guidelines on Integration of the Local Health Systems into P/CWHS”*
- AO No. 2020-0019, *“Guidelines on the Service Delivery Design of HCPN”*
- AO No. 2020-0037, *“Guidelines on Implementation of the Local Health Systems Maturity Level (LHS ML)”*
- DM No. 2021-0277: FY 2021 LHS ML Annual Monitoring and Updated LHS ML Monitoring Tool
- DM No. 2025-0599: LHS ML Monitoring Tool and Related Activities for F.Y. 2026 Onwards

LOCAL HEALTH SYSTEMS INTEGRATION

► KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS (LHS MATURITY LEVEL)



► KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS (LHS MATURITY LEVEL)

Building Blocks	Characteristics
Leadership and Governance (BLHSD)	Unified Governance of the LHS (<i>LG 1</i>) Strategic and Investment Planning (<i>LG 2</i>)
Financing (BLHSD)	Financial Management (<i>Fin 1</i>)
Health Workforce (HHRDB)	HRH Management and Development (<i>HW 1</i>)
Information (KMITS, EB)	Information Management (<i>Info 1</i>) Epidemiology and Surveillance System (<i>Info 2</i>)
Medicines, Products, Vaccines and Technology (SCMS, PD, PS)	Procurement and Supply Chain Management System (<i>MPVT 1</i>)
Service Delivery (DPCB, HEMB, HPB, HFSRB, BLHSD)	Referral System (<i>SD 1</i>) DRRM-H System (<i>SD 2</i>) Health Promotion Programs and Campaigns (<i>SD 3</i>)

- Components
 - Levels of Progression
 - Preparatory Level (Level 1)
 - Organizational Level (Level 2)
 - Functional Level (Level 3)
 - Key Result Areas (KRAs)
 - Province - 71 KRAs
 - HUCs/ICCs - 69 KRAs
 - Means of Verification (MOVs)
 - Province - 115 MOVs
 - HUCs/ICCs - 113 MOVs

AO No. 2020-0037: Guidelines on the Implementation of the LHS ML
 DM No. 2021-0277: FY 2021 LHS ML Annual Monitoring and Updated LHS ML Monitoring Tool
 DM No. 2025-0599: LHS ML Monitoring Tool and Related Activities for FY 2026 onwards

LOCAL HEALTH SYSTEMS INTEGRATION

► BASIS OF PRIORITIZATION OR TARGETING

UHC Integration Sites are Provinces/HUCs/ICCs which have committed to integrate LHS into P/CWHS by signing a tripartite MOU with DOH and PhilHealth

PROGRAM PROFILE

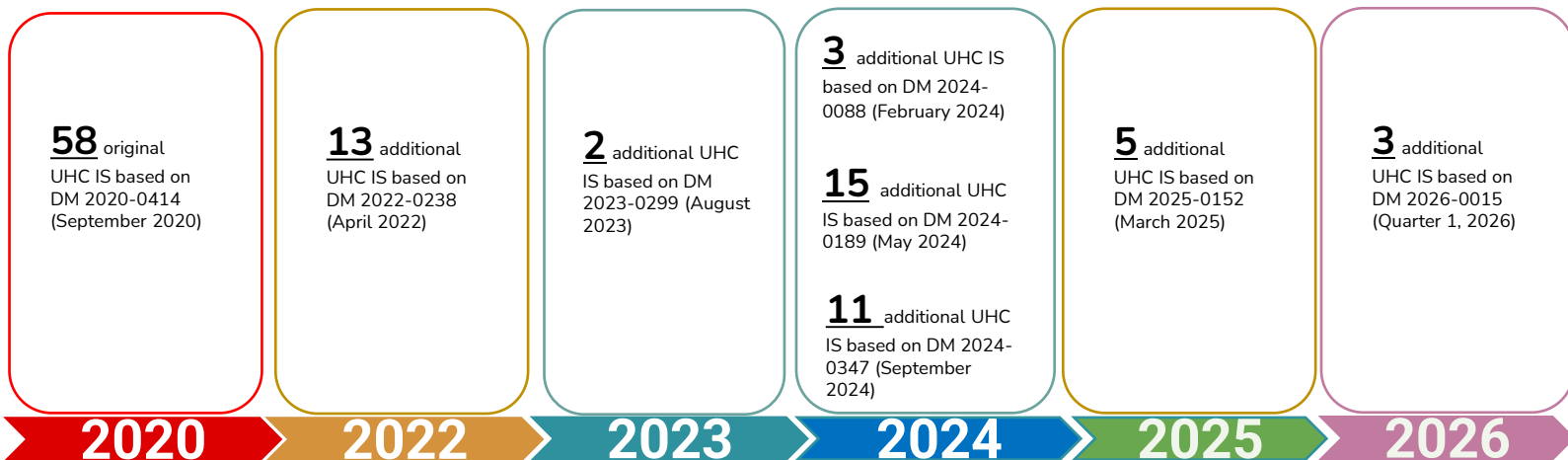
NON-UHC INTEGRATION SITES

- NCR – Makati City
- Region III – Angeles City, Olongapo City
- NIR – Negros Oriental***
- Region VIII – Tacloban City
- Region IX – Sulu**
- BARMM – Lanao del Sur*, Tawi-Tawi*, Cotabato City

**With ongoing discussions in recognizing non-devolved LGUs in BARMM as part of region-wide local health systems integration*

***with letter of intent to SOH to become a UHC IS*

****with pending application*

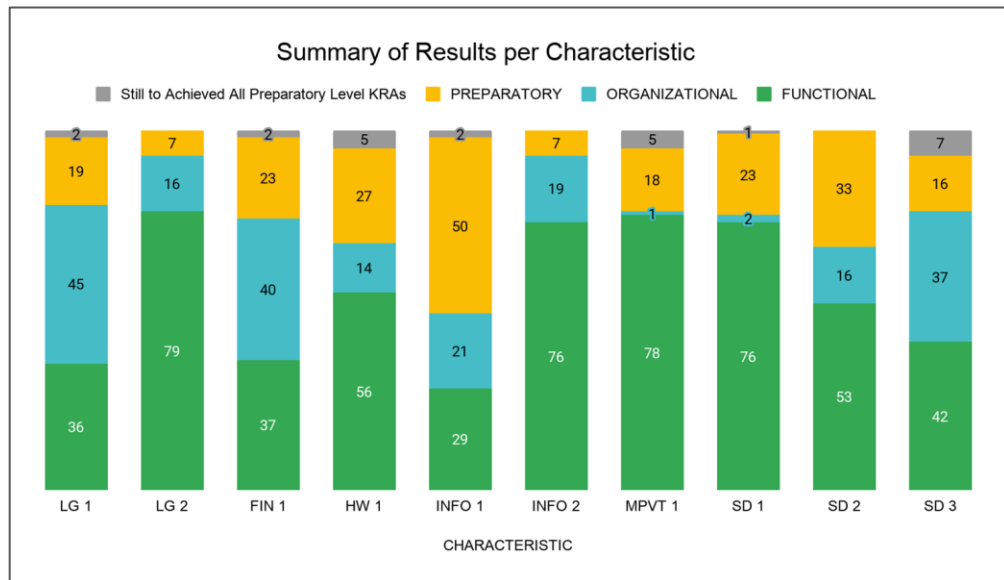


Total: 110 UHC IS (77 provinces, 33 HUCs/ICCs) out of 120 LGUs

► PERFORMANCE INDICATORS AND TARGETS

Year	Target	Accomplishment
2022 (Year-end Monitoring)	At least 58 UHC IS are able to achieve 100% of the <u>Preparatory Level</u> Key Result Areas (KRAs)	53 UHC IS <i>*4/13 Addl UHC IS were able to achieve 100% Prep Level</i>
2023 (Year-end Monitoring)	At least 58 UHC IS are able to achieve 100% of the <u>Preparatory Level</u> KRAs	69 UHC IS
	At least 58 UHC IS are able to achieve at least 70% of the <u>Organizational Level</u> KRAs	63 UHC IS
2024 (Year-end Monitoring)	At least 58 UHC IS are able to achieve <u>100%</u> of the <u>Organizational Level</u> KRAs	33 UHC IS achieved 100% of the Level 2 KRAs <i>36 UHC IS achieved 70-99% of the Level 2 KRAs</i>
	At least 58 UHC IS are able to achieve <u>at least 70%</u> of the <u>Functional Level</u> KRAs	40 UHC IS achieved at least 70% of the Level 3 KRAs <i>18 UHC IS achieved 50-69% of Level 3 KRAs</i>
2025 (Year-end Monitoring)	At least 58 UHC IS are able to achieve <u>at least 90%</u> of the <u>Functional Level</u> KRAs	31 UHC IS achieved at least 90% of the Level 3 KRAs <i>58 UHC IS achieved at least 70% of Level 3 KRAs</i>
		<i>91 UHC IS achieved 100% of Level 1 KRAs 37 UHC IS achieved 100% of Level 2 KRAs, and 85 UHC IS achieved at least 70% of level 2 KRAs 24 UHC IS achieved 100% of Level 1 and 2 KRAs, and at least 90% of Level 3 KRAs</i>

► PERFORMANCE INDICATORS AND TARGETS



Based on the 2025 LHS ML Annual Monitoring Report

Note: Reported accomplishments for **2025** are based on data as of **December 31, 2025**.

LG1 - Unified Governance

LG2 - Strategic and Investment Planning

Fin1 - Financial Management

HW1 - HRH Management & Development

Info1 - Information Mgt System

Info2 - Epidemiology and Surveillance System

MPVT1 - Procurement & Supply Chain Mgt

SD1 - Referral System

SD2 - DRRM-H

SD3 - Health Promotion

LOCAL HEALTH SYSTEMS INTEGRATION



BUDGET SUMMARY

PROGRAM PROFILE

LHSDA Budget Line Item	CY 2021	CY 2022 (DM # 2021- 0458)	CY 2023 (DM # 2022- 0436)	CY 2024 (DM # 2023- 0350)	CY 2025 (DM # 2024- 0323)	CY 2026 (DM # 2025- 0381)	CY 2027 (NEP)
Support to CHD Operations	17,052,917 (DM # 2020-0443)	37,261,373	45,911,335	45,722,358	58,196,111	212,953,747	212,953,747
Hiring of Technical Support Staff	47,181,355 (DM # 2020-0443)	56,418,163	71,053,330	71,053,330	71,053,330	76,202,125	76,202,125
Financial Grant for UHC IS	47,407,142 (DM # 2020-0443) 145,760,315 (DO # 2021-0664)	179,646,876	220,001,750	220,001,750	231,417,993	74,368,362	74,368,362

Note: Financial assistance to BARMM UHC IS (Maguindanao and Basilan) issued through a separate order as fund transfer

No. of UHC IS (excluding MOH BARMM):

2021 - 58 UHC IS; 2022 - 71 UHC IS; 2023 - 73 UHC IS; 2024 - 102 UHC IS; 2025 - 104 UHC IS; 2026 - 110 UHC IS

HEALTH CARE PROVIDER NETWORK

PROGRAM PROFILE

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

- Health Care Provider Network (HCPN) refers to a group of primary to tertiary care providers, whether public or private, offering people-centered and comprehensive care in an integrated and coordinated manner with primary care provider acting as the navigator and coordinator of health care within the network
- **HCPN Demonstration** - Joint program with PhilHealth; A mechanism of live-testing environments for new policy approaches and interventions implemented under controlled and supervised conditions

► OBJECTIVES AND INTENDED OUTCOMES

- To expedite the demonstration of functional integration of LHS through pilot implementation of major financial reforms such as PhilHealth network contracting, All Case Rates-based Global budget (ACR-GB), prospective payments, integrated procurement process, and digitized supply chain and management system and integrated health information system in HCPN
- To implement PHIC Konsulta+SDG reform, GAMOT (outpatient drug benefit package), ACR-GB

HEALTH CARE PROVIDER NETWORK

PROGRAM PROFILE

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

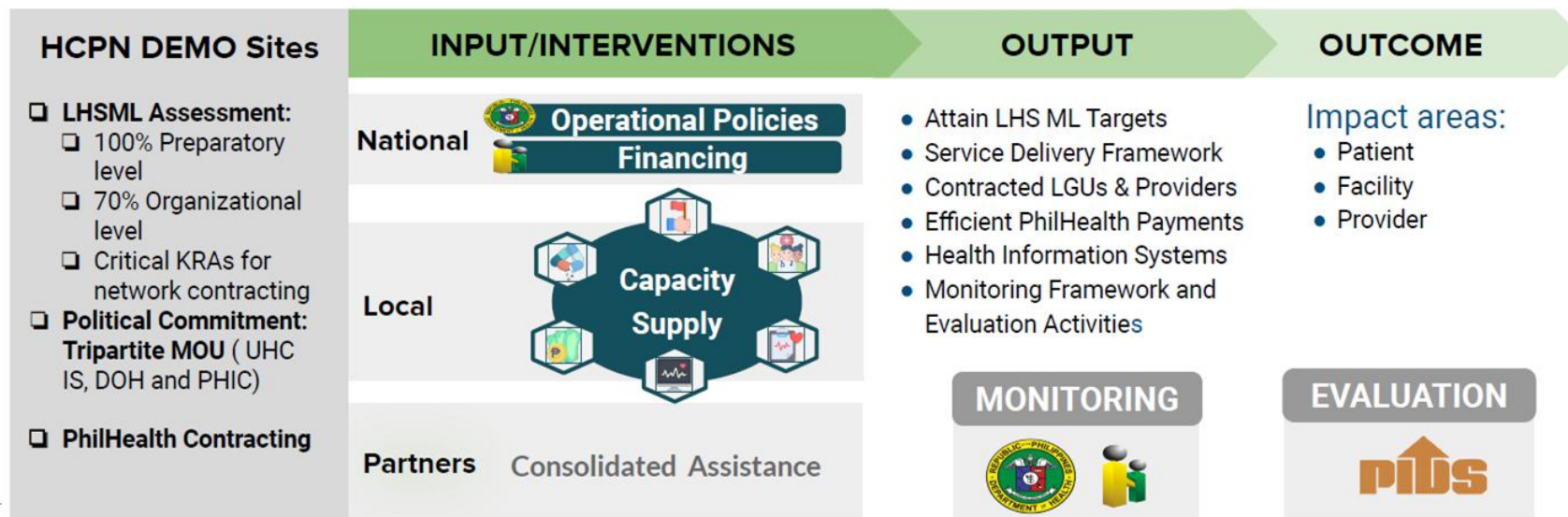
- **Features of a Public-led HCPN (*Province-/City-wide Health Systems*)**
 - Composition of public HCPNs: BHS, Health Centers, RHUs, Municipal, District, City, and Provincial Hospitals
 - Managing Entities: Provincial/ City Health Boards, to be assisted by the Provincial/ City Health Office and Management Support Unit
 - Establishment of a Special Health Fund (SHF)
 - Establishment of management support systems (strategic and investment planning, human resource for health management and development, information management, financing, procurement and supply chain management, functional referral system, function disaster risk reduction management, functional epidemiological surveillance system, health promotion programs/campaign)
- **Contracting Mechanisms**
 - DOH for **population-based** health services (through **Terms of Partnership**)
 - PhilHealth for **individual-based** health services (through **Service-Level Agreement**)
 - PhilHealth Income of the Public HCPN to accrue to the **Special Health Fund**

HEALTH CARE PROVIDER NETWORK

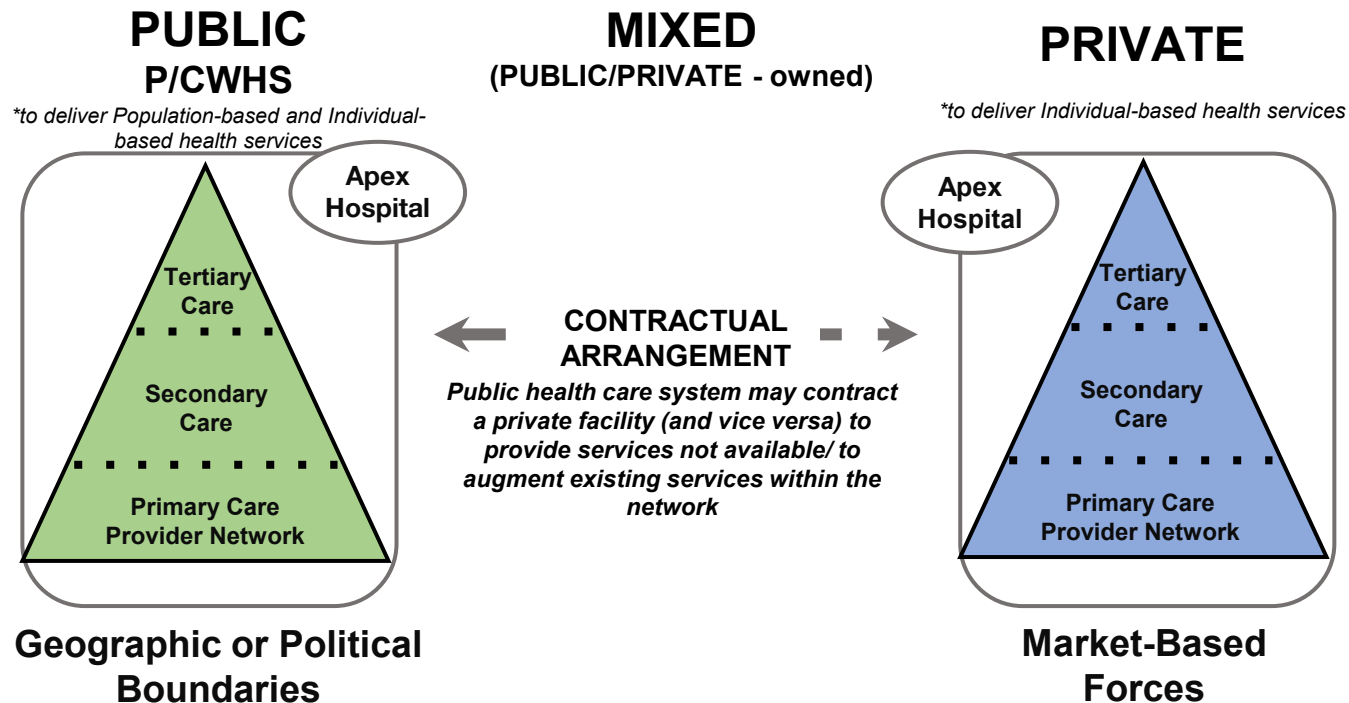
PROGRAM PROFILE

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

- If the UHC Integration Sites assessed as ready and eligible for network contracting were provided with appropriate and adequate inputs/ interventions (including prospective payments from PhilHealth), they are expected to produce significant results and achieve functional health systems



HEALTH CARE PROVIDER NETWORK



HEALTH CARE PROVIDER NETWORK

Key Activities and Policies

*Guided by the Local Health Systems
Maturity Level (LHSMML) Characteristics,
these interventions are critical to the
HCPN Demonstration Program*

LHSMML Characteristic	Responsible Office	Activities and/or Policies
LG 1 – Unified Governance of the Local Health Systems	BLHSD, HHRDB	LHB Capacity Development 1. Capacity Development Program for DOH Representatives as Local Technical Experts (<i>led by BLHSD, funded by PBSP</i>) 2. Development of the Local Health Board Guidebook (<i>led by BLHSD, funded by PBSP</i>) 3. LHB Capacity Development Program (<i>led by HHRDB, funded by ADB</i>)
	BLHSD	Contracting of Province/City-Wide Health Systems (P/CWHS) 1. Terms of Partnership (UHC IS Grants to SHF)
LG 2 – Strategic and Investment Planning + Fin 1 – Financial Management	UHCST	Engagement of service delivery networks of the HCPN Demonstration Site 1. PhilHealth Circular on HCPN Contracting 2. Conduct of Site Sensitization and Orientation of HCPN Reforms
	BLHSD	Transfer of DOH Funds to the Special Health Fund 1. Overall program management on Special Health Fund 2. Manual of Procedures for the Special Health Fund 3. Financial monitoring of Special Health Fund (with other agencies)
	UHCST, PHIC	Transfer of PhilHealth Funds to the Special Health Fund 1. PhilHealth Circular on Prospective Payment Mechanisms (3PM) for HCPNs 2. Financial monitoring of Special Health Fund (with other agencies)

HEALTH CARE PROVIDER NETWORK

Key Activities and Policies

*Guided by the Local Health Systems
Maturity Level (LHSMML) Characteristics,
these interventions are critical to the
HCPN Demonstration Program*

LHSMML Characteristic	Responsible Office	Activities and/or Policies
Info 1 – Information Management System	KMITS, PHIC	Validated EMR Systems that links Primary care to secondary and tertiary care providers within the P/CWHS 1. Policy on province/city-wide interoperability of Health Information System (electronic medical records, logistics and supply chain, e-prescription/medication, etc) 2. Specific guidelines on province/city-wide health enterprise architecture planning and data governance blueprint and service delivery framework/management 3. Specific guidelines on province/city-wide procuring connectivity and implementing cybersecurity infrastructure in health facilities
MPVT 1– Procurement and Supply Chain Management	PS, SCMS, PD	Availability of essential medicines and commodities 1. TA in ensuring functionality of electronic logistics and supply chain, warehousing and pharmacies particularly in Primary Care Facilities 2. TA in pooled/ integrated procurement
	UHC ST	Digitalized PHIC benefit and payment information system 1. Information system on Konsulta+SDG, GAMOT, ACR GB, 3PM 2. Guidelines and TA on GAMOT (outpatient drug benefit package)

HEALTH CARE PROVIDER NETWORK

Key Activities and Policies

*Guided by the Local Health Systems
Maturity Level (LHSMML) Characteristics,
these interventions are critical to the
HCPN Demonstration Program*

LHSMML Characteristic	Responsible Office	Activities and/or Policies
SD 1 – Referral System	HFDB	Monitoring of P/CWHS Referral Systems 1. Publication of revised issuance on the monitoring of referral systems within HCPNs Integration of BUCAS Centers in the P/CWHS 1. Development of standards for the implementation of BUCAS Centers 2. Inclusion of BUCAS Centers in HCPN
	DPCB	Development and Roll out of Clinical Management Pathways 1. CMP for Primary Care Services (HIV, TB) 2. CMP for Sepsis
	UHCST, PHIC	Outpatient and inpatient benefit expansion 1. Integration of packages in primary care 2. Expansion of benefit packages

HEALTH CARE PROVIDER NETWORK

How is HCPN different from existing integration efforts?

	UHC IS	PCPN Konsulta Sandbox	HCPN Demo
Goal	To guide progression of local health systems towards integration	To pilot prospective payments for a primary care provider network , through the Special Health Fund	To simulate an integrated province-wide health system with PhilHealth financing as primary incentive
Sites profiles	Committed to integrate their local health systems Varying levels of progression	With capacity for providing primary care for at least 20% of the population	High LHSML scores, with health partner, and not involved in other sandboxes Accredited health facilities Registered population

HEALTH CARE PROVIDER NETWORK

► **LEGAL AND POLICY BASIS**

- UHC Act IRR
 - Section 18.2: PhilHealth shall endeavor to contract health care provider networks for the delivery of individual-based health services
 - Section 18.10: PhilHealth shall adopt any or a combination of closed-end, prospective payment mechanisms, such as capitation, global budget, case-based payment, per diem or daily charges, and other appropriate mechanisms
 - Section 19: Integration of local health systems into province-wide and city-wide health system
- RA No. 7875, as amended, NHIA of 2013: Section 16 (j) PhilHealth has the power to negotiate and enter into contracts regarding payment mechanisms and delivery of health services, among others
- DOH Administrative Order No. 2020-0019 Guidelines on the Service Delivery Design of Health Care Provider Networks
- DOH Administrative Order No. 2020-0021 Guidelines on the Integration of the Local Health Systems into Province-/City-wide Health Systems

**KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS****Preparatory**

UHC Integration Sites (IS) to continue integrating their LHS into P/CWHS as guided by LHS ML

DOH to assess the readiness of the UHC Integration Sites based on the level of LHS maturity and service standards compliance

Application

UHC Integration Sites to comply with 10 Network requirements of PhilHealth

Simultaneous submission of network requirements and member facilities' requirements to PhilHealth

Release of Notice to Proceed with Contracting by PhilHealth

**Negotiation and
Approval**

Alignment of computed contract amount and discussion on key clauses of SLA by UHC IS and PhilHealth

Review and finalization of MOA and SLA based on agreement

Post-Contracting

DOH and Philhealth to monitor and evaluate of contracted HCPN

HEALTH CARE PROVIDER NETWORK



BASIS OF PRIORITIZATION OR TARGETING

- Level of LHS maturity based on:
 - LHS ML 2025 Year-end Report
 - Service capacity and compliance to standards set forth in AO 2020-0019 “Guidelines on Service Delivery Design of HCPNs) and AO 2020-0021 (Guidelines on Integration of the LHS into P/CWHS)

76 UHC Integration Sites deemed ready and qualified for PHIC network contracting as of April 1, 2026

Annex A: List of Universal Health Care Integration Sites Qualified for Health Care Provider Network Contracting and DOH UHC Fund for ZBB in LGU Hospitals per Universal Health Care Health Service Cluster

UHC Health Service Cluster	Region	Updated List of UHC IS Eligible for HCPN Contracting
Area I (Northern and Central Luzon=21)	CAR	1 Baguio City
		2 Mountain Province
		3 Benguet
		4 Kalinga
		5 Ifugao
		6 Apayao
		7 Abra*
	Region 1	8 Pangasinan
		9 Ilocos Sur
		10 Ilocos Norte
		11 La Union
		12 City of Dagupan
	Region 2	13 Santiago City
		14 Quirino
		15 Cagayan
		16 Nueva Vizcaya
		17 Isabela
		18 Batanes
	Region 3	19 Tarlac
		20 Bataan
		21 Bulacan
		22 Pampanga
Area II (NCR and Southern Luzon=16)	NCR	23 Paranaque City
		24 Valenzuela City
		25 Marikina City*
		26 Pasig City*
	CaLaBaRZon	27 Quezon

Area III (Visayas=14)	MiMaRoPa	28 Lucena City
		29 Laguna
		30 Batangas
		31 Palawan
		32 Puerto Princesa City
		33 Romblon
		34 Occidental Mindoro*
		35 Oriental Mindoro
	Region 5	36 Marinduque
		37 Sorsogon
Area IV (Mindanao=23)	Region 6	38 Catanduanes
		39 Aklan
		40 Iloilo
		41 Guimaras
		42 Capiz
	NIR	43 Antique
		44 Negros Occidental
	Region 7	45 Bohol
		46 Mandaue City
	Region 8	47 Lapu Lapu City*
		48 Southern Leyte
		49 Northern Samar
		50 Western Samar
		51 Biliran
		52 Eastern Samar
	Region 9	53 Zamboanga del Norte
		54 Zamboanga del Sur
		55 Zamboanga Sibugay
	Region 10	56 Cagayan De Oro City
		57 Lanao del Norte
		58 Misamis Occidental*
		59 Misamis Oriental*

Region 11	60 Camiguin*
	61 Davao Oriental
	62 Davao de Oro
	63 Davao del Norte
	64 Davao City
	65 Davao del Sur*
Region 12	66 South Cotabato
	67 Sarangani
	68 Sultan Kudarat
	69 General Santos City*
CARAGA	70 Surigao del Norte
	71 Agusan del Sur
	72 Butuan City
	73 Agusan del Norte*
	74 Surigao del Sur*
BARMM	75 Basilan
	76 Maguindanao

* FY 2025 LHS-ML Year End Report

HEALTH CARE PROVIDER NETWORK

► BASIS OF PRIORITIZATION OR TARGETING

Achieved 100% Preparatory Level KRAs
(16 KRAs)

Characteristics	KRAs (16)
Unified Governance of the Health System (LG1)	1.1 Commitment to integrate LHS into P/CWHS
	1.2 Presence of TWG to Assist the Provincial/ City Health Board (P/CHB) on Matters Relating to LHS Integration
Strategic and Investment Planning (LG2)	1.1 Baseline Assessment of LGUs' Health Capacity and Corresponding Investment Needs on LHS Integration
Financial Management (Fin1)	<i>Fin1 Preparatory Level KRAs are subsumed under LG1, particularly the commitment of the LGU to integrate its local health system (KRA 1.1), and membership and functions of the expanded P/CHB (KRA 1.2).</i>
HRH Management and Development (HW1)	1.1 Baseline Assessment, Gaps Analysis and Investment Needs for the Dev't and Strengthening of HRH Management & Dev't System
Information Management System (Info1)	1.1 Baseline Assessment, Gaps Analysis & Investment Needs for the Dev't & Strengthening of Information Management System
Epidemiology and Surveillance System (Info2)	1.1 Baseline Assessment, Gaps, Analysis & Investment Needs for the Dev't and Strengthening of Epidemiology & Surveillance System
Procurement and Supply Chain Management (MPVT1)	1.1 Baseline Assessment, Gaps Analysis and Investment Needs for the Dev't and Strengthening of the Procurement and Supply Chain Management System
	1.2 Presence of a Pharmacy and Therapeutics Committee
Referral System (SD1)	1.1 Assessment of Existing Two-Way Referral System
	1.2 Groupings/ Clustering of Health Care Providers & Facilities
	1.3 Identification of Potential Apex Hospital/s
Disaster Risk Reduction Management in Health (SD2)	1.1 Baseline Assessment & Gaps Analysis on DRRM-H System Implementation
Health Promotion Programs and Campaigns (SD3)	1.1 Baseline Assessment on Functional Health Literacy
	1.2 Presence of a Health Promotion Committee
	1.3 Barangay Health Workers (BHWs) as Barangay Health Promotion Officers
	1.4 Implementation of Population-wide Health Promotion Program & Campaigns



HEALTH CARE PROVIDER NETWORK

► BASIS OF PRIORITIZATION OR TARGETING

Achieved 100% Preparatory Level KRAs
(16 KRAs)

Achieved at least 70% of the
Organizational Level KRAs (25/36
KRAs)

Characteristics	KRAs (36)
Unified Governance of the Health System (LG1)	2.1 Organized Province-wide and City-wide Health System <i>(applicable to only 58 UHC IS)</i>
	2.2 Expanded Membership and Functions of P/CHB
	2.3 Strengthened Provincial/ City Health Office
	2.4 Established Management Support Unit (MSU)
Strategic and Investment Planning (LG2)	2.1 Local Investment Plans for Health (LIPH) as the Strategic and Investment Plan of the P/CWHS
	2.2 LIPH/ Annual Operational Plan (AOP) concurred by Concerned Center for Health Development (CHD)
Financial Management (Fin1)	2.1 Opening of Special Health Fund (SHF) Depository Bank Account
	2.2 Presence of Separate SHF Book at Provincial/ HUC/ ICC Level
	2.3 Presence of Subsidiary Ledger for SHF in Trust Fund of Component LGUs <i>(applicable to only 58 UHC IS)</i>
	2.4 Presence of Personnel in the MSU to Assist the Board in the Management of the SHF
HRH Management and Development (HW1)	2.1 Presence of P/CWHS HRH Plan
	2.2 Updated National Health Workforce Registry (NHWR)
	2.3 Presence of Harmonized HRH Management and Development Systems and HRH Performance Assessment System
	2.4 Presence of DOH-PRC Certified Primary Care Workers
Information Management System (Info1)	2.1 Presence of P/CWHS Health Information Management/ Information and Communication Technology (ICT) Development Plan
	2.2 Functional Electronic Medical Records (EMR) System among Health Facilities within P/CWHS
	2.3 Engagements with Medical Specialists/ Facilities within P/CWHS
	2.4 Presence of Dedicated ICT Personnel in the P/CHO



HEALTH CARE PROVIDER NETWORK

► BASIS OF PRIORITIZATION OR TARGETING

Achieved 100% Preparatory Level KRAs
(16 KRAs)

Achieved at least 70% of the
Organizational Level KRAs (25/36
KRAs)

Characteristics	KRAs (36)
Epidemiology and Surveillance System (Info2)	2.1 Epidemiology and Surveillance Units (ESU) with dedicated personnel competent on Basic Epidemiology, Disease Surveillance and Event-based Surveillance
	2.2 Presence of Technical Guidelines/ MOP on Epidemiology and Surveillance System
Procurement and Supply Chain Management (MPVT1)	2.1 Presence of P/CWHS Procurement Plan
	2.2 Presence of Technical Guidelines/ MOP on Harmonized Supply Chain Management System
	2.3 Presence of Pharmacist/ Pharmacy Assistant
	2.4 Presence of Dedicated Personnel on Supply Chain Management in P/CHO
Referral System (SD1)	2.1 Presence of Technical Guidelines/ Manual on Referral System
	2.2 Adoption & Implementation of Clinical Practice Guidelines (CPGs) and Other Case Management Protocols in P/CHWS
	2.3 Presence of P/CWHS Health Facility Development Plan
	2.4 Presence of Public Health Unit in LGU Hospitals <i>(applicable to only 65 UHC IS)</i>
	2.5 Partnership with Apex Hospital/s
	2.6 Registration of Indigents to a Primary Care Provider (PCP) within the P/CHWs
	2.7 Presence of Technical Guidelines on Customer Feedback Mechanism
Disaster Risk Reduction Management in Health (SD2)	2.1 Organized Province-wide/ City-wide DRRM-H System
	2.2 Monitoring of P/CWHS Performance on DRRM-System
Health Promotion Programs and Campaigns	2.1 Presence of Health Promotion Unit (HPU) in the P/CHO
	2.2 Presence of P/CWHS Health Promotion Framework Strategy (HPFS)
	2.3 Implementation of Population-wide Health Promotion Programs (at least 3 HPFS priority areas)



HEALTH CARE PROVIDER NETWORK

► BASIS OF PRIORITIZATION OF TARGETING

Achieved 100% Preparatory Level KRAs
(16 KRAs)

Achieved at least 70% of the
Organizational Level KRAs (25/36
KRAs)

Achieved all non-negotiable KRAs

- LG1 2.1 Organized Province-wide Health System (Signed and notarized inter-LGU MOA)
- LG1 2.2 Expanded Membership and Functions of the P/CHB
- Fin1 2.1 Opening of SHF Depository Bank Account

HEALTH CARE PROVIDER NETWORK



BASIS OF PRIORITIZATION C TARGETING

Achieved 100% Preparatory Level KRAs (16 KRAs)

Achieved at least 70% of the Organizational Level KRAs (25/36 KRAs)

Achieved all non-negotiable KRAs

Achieved/ ongoing status for the additional KRAs

- LG1 2.3 Strengthened P/CHO
- LG1 2.4 Established MSU
- LG2 2.1 LIPH as strategic and investment plan of P/CWHS
- LG2 2.2 LIPH/AOP concurred by CHD
- Fin1 2.2 Presence of Separate SHF Book of Accounts
- Fin1 2.3 Presence of Subsidiary Ledger for SHF in Trust Fund of Component LGUs
- Fin1 2.4 Presence of Personnel in the MSU to assist the Board

HEALTH CARE PROVIDER NETWORK



BASIS OF PRIORITIZATION C TARGETING

Achieved 100% Preparatory Level KRAs (16 KRAs)

Achieved at least 70% of the Organizational Level KRAs (25/36 KRAs)

Achieved all non-negotiable KRAs

Achieved/ ongoing status for the additional KRAs

- Info 1 2.2 Functional EMR among health facilities within the P/CWHS
- MPVT1 2.3 Presence of Pharmacist/ Pharmacy Assistant
- SD1 2.1 Presence of Technical Guidelines/ Manual of Referral System
- SD1 2.4 Presence of Public Health Unit in Public Hospitals
- SD1 2.5 Partnership with Apex Hospital/s
- SD1 3.2 Registration of all constituents to a PCP in P/CWHS

LOCAL HEALTH SYSTEMS INTEGRATION AND HCPN

► BLHSD PROGRAM ACCOMPLISHMENTS (2023-2026)

	2023	2024	2025	2026
Policy/ Reports	- DC No. 2023-0049/0133: FY 2022 LHS ML Year-End Monitoring Report - AO No. 2023-0003: Policy Framework for Private Sector Engagement for the P/CWHS in Support of UHC	- DC No. 2024-0094: FY 2023 LHS ML Year-End Monitoring Report - DM No. 2024-0343: Guide in the validation of LHS ML LG1 Characteristic - DM No. 2024-0237: List of UHC IS Qualified for HCPN Contracting - DM No. 2024-0476: Guidelines on the Demonstration of the Functionality of Selected Public HCPNs	- DC No. 2025-0125: FY 2024 LHS ML Year-End Monitoring Report - DM No. 2025-0599: LHS ML Monitoring Tool and Related Activities for FY 2026 Onwards - DC No. 2025-0419: Dissemination of the Compendium of Implementation Journeys of Select UHC IS (Vol 1)	- DC No. 2026-0122/ 0122-A: LHS ML CY 2025 Year End Report - DC No. 2026-0171: Updated List of Universal Health Care Integration Sites Qualified for Health Provider Network Contracting based on the FY 2025 Local Health Systems Maturity Level Year End (LHS ML) Report - DC No. 2026-0336 Dissemination of the Compendium of Implementation Journeys of Select UHC IS (Vol 2) - AO No. 2026-0009 Implementing Guidelines of DOH UHC Fund for ZBB in LGU Hospitals

LOCAL HEALTH SYSTEMS INTEGRATION AND HCPN

► BLHSD PROGRAM ACCOMPLISHMENTS (2023-2026)

	2023	2024	2025	2026
Technical Assistance	- Capacity Development Design for DOH Representatives <i>“Leading Change in Local Health Systems Integration for UHC: Navigating Complexities and Driving Results”</i> - HCPN Demonstration Site Consultative Workshop - Consultative Meeting on the Local Economic Enterprise (LEE) on Health of the Province of Benguet and Aklan - Participated in various UHC Fora/Summit as resource speaker	- Orientation on the Community of Practice for the HCPN Demonstration Site - FY 2024 LHSML Mid-Year Results and Updates on HCPN Demonstration Program - Local Health Systems Maturity Levels Second Consultative meeting - Participated in various UHC Fora/Summit as resource speaker	- DOH and PhilHealth HCPN Alignment Meeting & Strategic Planning - Sub-allotment of Funds to Selected CHDs for the Provision of Financial Grant in Support of Additional UHC IS - Sub-Allotment of Funds to Selected CHDs for the Implementation of Local Health Systems-Related Activities - Stakeholders Forum on the Implementation of the Special Health Fund - Learning Exchange Sessions on Local Health Systems Integration - First National UHC Summit	- Sub-Allotment of Funds to Selected CHDs for the Implementation of Local Health Systems-Related Activities - Local Health Systems Maturity Levels 6th Consultative meeting - HCPN Coordination Meetings - Policy Dialogue on Private Sector Engagement in Province/City-Wide Health Systems - Participated in various UHC Fora/Summit as resource speaker

LOCAL HEALTH SYSTEMS INTEGRATION AND HCPN

► BLHSD PROGRAM ACCOMPLISHMENTS (2023-2025)

	2023	2024	2025	2026
Monitoring	Monitoring of Selected UHC IS (Valenzuela City)	Monitoring of Selected UHC IS (Province of Quirino)	Monitoring of Selected UHC IS (Provinces of Batanes and Sorsogon)	Monitoring of Selected UHC IS (Negros Occidental, Bacolod City, Lanao del Norte, and Iligan City)

LOCAL HEALTH SYSTEMS INTEGRATION

► **CHDs/ MOH BARMM PROGRAM CHALLENGES AND RECOMMENDATIONS**
(2026)

CHDs	CHALLENGES	Recommendations
CARAGA, Bicol, SOCCSKSARGEN, BARMM	Delayed issuance of policies, guidelines, advisories, and Means of Verification (MOV) requirements affecting planning, compliance, and implementation.	Release policies and MOV requirements at least six months prior to implementation and validation activities.
Ilocos, Cagayan Valley, CALABARZON	Varying interpretations of policies among CHDs, LGUs, and stakeholders leading to inconsistent implementation.	Conduct regular policy calibration, harmonization sessions, and learning exchanges.
SOCCSKSARGEN, Davao, CARAGA, Zamboanga	Lack of operational guidance for implementation of SHF, HCPN, ZBB, and other UHC reforms.	Develop implementation manuals, FAQs, operational toolkits, and technical advisories.
MIMAROPA, Bicol	Fragmented technical guidance from different national offices resulting in duplication and confusion at the local level.	Strengthen coordination and alignment among Central Office bureaus and services and institutionalize integrated technical assistance mechanisms.

LOCAL HEALTH SYSTEMS INTEGRATION

► CHDs/ MOH BARMM PROGRAM CHALLENGES AND RECOMMENDATIONS (2026)

<i>Governance</i> CHDs	CHALLENGES	Recommendations
Zamboanga, Northern Mindanao, Davao, NCR, Bicol, Western Visayas	Frequent changes in local leadership and shifting political priorities affecting continuity of UHC implementation, integration initiatives, and policy support.	Institutionalize UHC commitments through ordinances, executive orders, local investment plans, and development plans to ensure continuity across administrations.
Zamboanga, Eastern Visayas, Western Visayas,,NIR, Bicol	Delayed enactment of local ordinances, executive issuances, and governance policies needed to support integration and operationalization of PWHS.	Provide model ordinances, strengthen technical assistance, and facilitate policy development workshops for LGUs.
Northern Mindanao, CAR, BARMM	Weak functionality and inconsistent performance of governance mechanisms such as PHBs, LHBs, TWGs, and Management Support Units (MSUs).	Conduct governance strengthening interventions, clarify roles and responsibilities, and institutionalize performance monitoring mechanisms.

LOCAL HEALTH SYSTEMS INTEGRATION

► CHDs/ MOH BARMM PROGRAM CHALLENGES AND RECOMMENDATIONS (2026)

<i>Governance</i> CHDs	CHALLENGES	Recommendations
Central Visayas, NCR, CALABARZON	Varying levels of commitment among LGUs and component municipalities affecting the pace and quality of integration efforts.	Strengthen advocacy activities, leadership engagement sessions, and regional UHC summits involving governors, mayors, and local legislators.
SOCCSKSARGEN, MIMAROPA	Resistance to organizational and operational changes required under integrated health systems.	Implement structured change management programs, stakeholder engagement activities, and communication campaigns.

LOCAL HEALTH SYSTEMS INTEGRATION

► CHDs/ MOH BARMM PROGRAM CHALLENGES AND RECOMMENDATIONS (2026)

<i>Human Resource for Health</i> CHDs	CHALLENGES	Recommendations
Northern Mindanao, SOCCSKSARGEN, Bicol, CAR, Ilocos, Cagayan Valley, Central Visayas	Persistent shortage of healthcare workers across all levels of care, affecting service delivery, health system integration, monitoring, and implementation of UHC reforms.	Expand HRH financing mechanisms and maximize the use of SHF, PhilHealth revenues, and national support programs for workforce augmentation.
Northern Mindanao, Davao, SOCCSKSARGEN, Bicol	High turnover of HRH, UHC focal persons, DMOs, TWG members, and contractual personnel leading to loss of institutional memory and disruptions in implementation.	Institutionalize succession planning, mentoring systems, turnover protocols, and knowledge management mechanisms.

LOCAL HEALTH SYSTEMS INTEGRATION

► CHDs/ MOH BARMM PROGRAM CHALLENGES AND RECOMMENDATIONS (2026)

<i>Human Resource for Health</i> CHDs	CHALLENGES	Recommendations
CAR, Ilocos, Cagayan Valley	Personnel Services (PS) cap and fiscal limitations restricting the hiring and retention of health personnel despite increasing service demands.	Review existing HRH financing policies and explore exemptions or alternative financing arrangements for critical health positions.
Northern Mindanao, SOCCSKSARGEN	Weak workforce planning, staffing analysis, and HRH monitoring systems resulting in gaps in deployment and workforce management.	Develop standardized HRH planning tools, strengthen workforce analytics, and institutionalize regular workforce assessments
Bicol, Ilocos, MIMAROPA	Limited access to continuing professional development and competency-building opportunities for health workers and UHC implementers.	Implement continuous competency development programs and structured learning pathways for UHC implementers and health workers.

LOCAL HEALTH SYSTEMS INTEGRATION

► CHDs/ MOH BARMM PROGRAM CHALLENGES AND RECOMMENDATIONS (2026)

<i>Information Management</i> CHDs	CHALLENGES	Recommendations
CAR, Northern Mindanao, CALABARZON	Delayed reporting, data quality issues, and limited use of digital information for planning and monitoring.	Strengthen digital governance systems and promote data-driven decision making.

LOCAL HEALTH SYSTEMS INTEGRATION

► CHDs/ MOH BARMM PROGRAM CHALLENGES AND RECOMMENDATIONS (2026)

CHDs	CHALLENGES	Recommendations
Bicol, CAR, NCR, SOCCSKSARGEN	Weak referral systems characterized by poor compliance with referral protocols, weak feedback mechanisms, and inconsistent implementation across facilities.	Strengthen referral governance, monitoring, and accountability mechanisms within HCPNs.
NCR, CAR	Continued bypassing of primary care facilities by patients resulting in congestion of hospitals and inefficient utilization of health services.	Intensify primary care advocacy, strengthen patient navigation systems, and improve public awareness on referral pathways.
Davao, CAR, Central Visayas	Absence of integrated patient tracking and navigation systems affecting continuity of care and referral completion.	Implement interoperable patient navigation and referral tracking systems across HCPNs.
CAR, Bicol, Central Luzon	Limited coordination and service integration among facilities within HCPNs.	Strengthen HCPN operationalization and formalize service coordination arrangements among network members.

HEALTH CARE PROVIDER NETWORK

Status of Province-Wide and City-Wide Health Systems (Public HCPN)

Organized HCPN
(With Inter-LGU MOA)

63

Total HCPN Contracted by
PhilHealth

None

*PhilHealth implemented a Strategic Pause
in HCPN Contracting*

With Previous PhilHealth
Contracting

4

(PCPN Sandbox: Bataan, South Cotabato, Quezon, Baguio City)

LOCAL INVESTMENT PLANS FOR HEALTH

PROGRAM PROFILE

► **PROGRAM/ACTIVITY/PROJECT DESCRIPTION**

- Provides policies, guidelines and strategic direction in the development of local health investment plans for health/annual operational plans (LIPH/AOP) to guide the LGUs in crafting these plans and mobilize resources in support of the implementation and achievement of UHC
- To institutionalize local health planning as a sectoral endeavor involving not just the LGUs and DOH, but also key stakeholders, towards a whole-of-system, whole-of-government, whole-of-society approach in the development, implementation and monitoring and evaluation of LGU health plans, with the LIPH as the costed strategic plan for UHC implementation.

► **OBJECTIVES AND INTENDED OUTCOMES**

1. To develop policies, procedures and tools to guide the development of LGU investment plans for health and annual operational plans
2. To develop capacities of key DOH and LGU players on LIPH and AOP processes, fostering bottom-up planning, and considering health needs of UUC for PHC, IP and other vulnerable populations
3. To improve matching of national resources and technical assistance based on the LGU needs identified in the LIPH/AOP



LEGAL AND POLICY BASIS

- Section 22 of the UHC Law (RA 11223) and its IRR state that province/city-wide investment plans for health shall serve as the basis for financial and non-financial grants from the national government to account for complementation of public and private health care providers and public or private health sector investments.
- AO No. 2020-0022 - *“Guidelines on the Development of Local Investment Plans for Health”*
- AO No. 2020-0018 - *“Guidelines on Contracting Province-Wide and City-Wide Health Systems”*
 - The Terms of Partnership is the legal instrument for contracting P/CWHS, formalizing the agreement between the DOH CHD and LGUs in the implementation of the AOP

► KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

1. Issuance of policies and guidelines on the development of 3-year LIPH and Annual Operational Plans
2. Continuing advocacy, consultation, partnership and collaboration with key stakeholders on local health planning (and financing)
3. Advocacy for LGUs to increase LGU budget for health
4. Capacitation of key DOH and LGU players on the LIPH and AOP processes
5. Provision of technical and financial assistance for LIPH and AOP development & monitoring
6. Harmonization and alignment of national and local health planning and budgeting processes and plans
7. Monitoring of AOP implementation

LOCAL INVESTMENT PLANS FOR HEALTH

► BASIS OF PRIORITIZATION OR TARGETING

Fixed Tranche for LIPH/AOP development and monitoring

From LHSDA Line Item (incorporated in CHDs; through fund transfer to MOH-BARMM)

Allocation of Fixed Tranche to LGUs may vary based on:

- Population/Demographics
- No. of Unserved/ Underserved Areas/GIDA
- No. of Component LGUs
- LIPH/AOP needs (Burden of disease, HS performance)
- Local Health Systems Maturity Level (LHS ML) Performance
- Utilization of funds

Total Amount of Fixed Tranche has not changed in the last 5 years

LOCAL INVESTMENT PLANS FOR HEALTH

► PERFORMANCE INDICATORS AND TARGETS

List all indicators used to measure progress, along with baseline and target values.

INDICATORS	TARGET S	2024	TARGETS	2025	TARGETS	2026	2027
		ACCOMPL.		ACCOMPL.		ACCOMPL.	TARGETS
Number of LGUs with LIPH/AOP	120	92% (111/120) with 2024 AOP	120	99% (119/120) with 2025 AOP	120	100% (120/120) with LIPH and 3 AOPs 2026, 2027 AOP - 97.5% 2028 AOP - 96%	120
Number of LGUs with signed TOP	120	73% (87/120) with signed TOP	120	88% (106/120) with signed TOP	120	48% (57/120) with signed TOP	120

Note: Reported accomplishments for **2025** are based on data as of **December 31, 2025**, while those for **2026** reflect data as of **June 30 2026**.

LOCAL INVESTMENT PLANS FOR HEALTH

PROGRAM PROFILE

► BLHSD PROGRAM ACCOMPLISHMENT (2024-2026)

	2024	2025	2026
Policy/ Reports	- DM 2024-0413 on Enhanced Process & Tools for 2026-2028 LIPs and AOPs - DM 2024-0483 on Template for the TOP for the Implementation of the AOP beginning 2025 onwards - DM 2024-0266 Guidance to CHDs and MOH-BARMM on Validation of LHS ML Strategic Investment Planning (LG2) and Financial Management (Fin1) Characteristics - DM 2024-0095 Guidance on Local Investment Plan for Health Indicator for Municipalities and Component Cities in the LGU HSC	- DM 2025-0285 and DM 2024-0266 on Guidance to CHDs & MOH-BARMM on Validation of LG2 and Fin1 Characteristics for LHS ML monitoring - DM 2025-0410 Guidance on the LIPH Indicator in the 2025 LGU HSC - DM 2025-0541 Development of LGU Supplemental AOPs	- DM 2026-0200 Guidance to CHDs and Ministry of Health (MOH)-BARMM_ on Validation of LG2 and Fin1 Characteristics for the Local Health System Maturity Level LHS ML Year 2026 Monitoring (and Advisory on extended timelines) - DM 2026-0179 Guidance on the LIPH Indicators in the 2026 LGU HSC Implementation (DM 2026-0179-A on extended timelines)

LOCAL INVESTMENT PLANS FOR HEALTH

PROGRAM PROFILE

► BLHSD PROGRAM ACCOMPLISHMENT (2024-2026)

	2024	2025	2026
Technical Assistance	- Annual Provision of Fixed Tranche and support for the development, implementation and monitoring of LIPH/AOP through direct allotment (CHD-incorporated funds) and fund transfer to MOH-BARMM - Orientation of CHDs and MOH-BARMM and relevant stakeholders on LIPH/AOP policies and processes - Orientation on the use of the LIPH Information System and Helpdesk Chat Group		
			Orientation on LIPH IS ver. 3.0 - March 13-15 Orientation on LG2 & Fin1 for 2026 LHS ML monitoring - April 6
Monitoring	AOP monitoring in CARAGA Central Luzon Zamboanga Peninsula	AOP monitoring in Central Visayas SOCCSKSARGEN Consultation & Review of LIPH and SHF implementation	NIR: May 18-22 Northern Mindanao: Aug 18-21 CAR: Sept 22-25 Consultation & Review of LIPH and SHF implementation

STATUS OF SUBMISSION OF 2026–2028 LIPHs AND ANNUAL OPERATIONAL PLANS (AOPs)

As of June 30, 2026



The submission and concurrence of the 2026–2028 Local Investment Plans for Health (LIPH) and the corresponding Annual Operational Plans (AOPs) continue to make strong progress nationwide.



120
Total LGUs



119 (99%)
LIPH Submitted



117 (97.5%)
AOP 2026 Submitted



117 (97.5%)
AOP 2027 Submitted



116 (96%)
AOP 2028 Submitted

LUZON						
CHD	No. of LGUs	LIPH 2026–2028	AOP 2026	AOP 2027	AOP 2028	REMARKS
NCR	17	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Initial appraisal completed (Sept. 2025)
CAR	7	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Concurrence completed (Dec. 2025)
ILOCOS	5	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Concurrence completed (Dec. 5, 2025)
CAGAYAN VALLEY	6	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Concurred Nov. 18; submitted to BLHSD via Google Drive
CENTRAL LUZON	9	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Completed
CALABARZON	6	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Submitted and concurred (Nov. 24, 2025)
MIMAROPA	6	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Completed
BICOL	7	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Completed

VISAYAS AND MINDANAO						
CHD	No. of LGUs	LIPH 2026–2028	AOP 2026	AOP 2027	AOP 2028	REMARKS
NIR	4	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Concurred (Dec. 15, 2025)
WESTERN VISAYAS	6	✓ 100%	✓ 100%	✓ 100%	✓ 100%	Concurred (by Dec. 19, 2025)
CENTRAL VISAYAS	5	✓ 100%	⚠ 3/5 (60%) (2 LGUs waiting for signature and waiting for submission)	⚠ 3/5 (60%) (2 LGUs waiting for signature and waiting for submission)	⚠ 3/5 (60%) (2 LGUs waiting for signature and waiting for submission)	2 LGUs waiting governor's signature and waiting for submission
EASTERN VISAYAS	8	✓ 100%	✓ 100%	✓ 100%	✓ 100%	All concurred before Dec. 31, 2025
ZAMBOANGA PENINSULA	6	⚠ 5/6 (83%)	⚠ 5/6 (83%)	⚠ 5/6 (83%)	⚠ 5/6 (83%)	Sulu – Ongoing vetting and appraisal
NORTHERN MINDANAO	7	✓ 100%	✓ 100%	✓ 100%	✓ 100%	All plans submitted and concurred
DAVAO REGION	6	✓ 100%	✓ 100%	✓ 100%	✓ 100%	All submitted, concurred and uploaded in GDrive
SOCCSKSARGEN	5	✓ 100%	✓ 100%	✓ 100%	✓ 100%	All LGU LIPH plans submitted and concurred
CARAGA	6	✓ 100%	✓ 100%	✓ 100%	✓ 100%	All plans submitted and concurred
MOH-BARMM	4	N/A	N/A	N/A	N/A	For separate reporting



NATIONWIDE OVERALL

120
Total LGUs

119 (99%)
LIPH Submitted

117 (97.5%)
AOP 2026 Submitted

117 (97.5%)
AOP 2027 Submitted

116 (96%)
AOP 2028 Submitted

STATUS LEGEND

- ✓ Completed (100%)
- ⚠ In Progress / Partial
- ⚫ Not Applicable

KEY TAKEAWAYS:

- 99% (119/120) of LGUs have submitted their 2026-2028 LIPHs.
- AOP 2026-2028 submissions are at a 96%
- A few LGUs remain pending administrative requirements
- Continuous coordination with CHDs is underway to achieve 100% completion

Note: Percentages may not total to 100% due to rounding.

Operational Policies for LIPH and AOP

DM 2024-0413, Enhanced Process and Tools for LGU Development of 2026-2028 LIPH and AOP

- Provides the new process to stakeholders in the development of LIPH and 3 AOPs for the 2026-2028 cycle
- Contains the updated tools and forms to aid planning across all levels

DM 2023-0125 Implementation of the Revised LGU AOP Monitoring Tool

- Aims to determine the responsiveness of the DOH/MOH-BARMM assistance to the needs of the LGUs as indicated in the AOP
- Determines the implementation of LGU commitment as indicated in the AOP
- Checks how the LIPH and AOP process is being implemented in accordance with the pertinent policy guidelines

Operational Policies for LIPH and AOP

DM 2025-0541 Development of LGU Supplemental AOPs

- Preparation of Supplemental AOPs upon the availability of new revenue sources, additional funds, or funds previously not available, under any of the following conditions:
 - a. New programs, projects, and activities (PPAs) not previously included in the approved AOP;
 - b. Adjustment of local priorities due to updated plans, directives, or performance results; and
 - c. Occurrence of public calamities, public health emergencies, or other unforeseen events requiring new PPAs.
- Contains tools and forms for LGU AOP Supplemental Plan

DM 2024-0483 on Template for the TOP for the Implementation of the AOP beginning 2025 onwards (and 0483-A amendment)

- WHEREAS, previously transferred funds to the Province/City have been liquidated as evidenced by the Certificate of No Unliquidated Cash Advances prepared by the CHD Chief Accountant dated _____, for the applicable line item/s (e.g., Local Health Systems Development and Assistance (LHSDA), health promotion grants);
- WHEREAS, a Certificate on Availability of Fund (CAF) dated _____ in the amount of _____ pesos (P_____.00) under _____ budget line item/s has been issued;

Operational Policies for LIPH and AOP

DM 2026-0200 Guidance on Validation of LG2 and Fin1 for the LHS ML 2026 Monitoring

- Annual issuance guide for CHDs, MOH-BARMM, and LGUs for the given monitoring year of the LHS ML (LG2)
- Contains the Key Result Areas, Means of Verification, frequency of collection, and achievement requirements across LHSML levels

DM 2026-0179 Guidance on the LIPH Indicators in the 2026 LGU HSC

- Annual issuance guide for CHDs and LGUs for the LGU Health Scorecard Implementation for the given year
- Contains the Means of Verifications and specific submission dates for the LIPH Indicator across LGU levels

Ongoing Policy Development for LIPH and AOP

- Ongoing system enhancement of LIPH IS (Q3 2026)
- LIPH Information System Manual and DM (Q3 2026)
- Consolidation of DOH Menu of Assistance for LGUs (Q3 2026)
- Revised AOP Monitoring Process DM (Q4 2026)
- Feedbacking process in the development of LIPH and AOP (Q4 2026)

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

PROGRAM PROFILE

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

- Provide policies, guidelines and strategic direction on the implementation of the Special Health Fund (SHF) of the Province-wide and City-wide Health System (P/CWHS)
- The SHF is a pool of financial resources intended for health services, serving as an augmentation to LGU health funds, and is managed by the P/CWHS
- Coordinate with various agencies (PHIC, DOF, DILG, DBM and COA) on the implementation and monitoring of SHF

► OBJECTIVES AND INTENDED OUTCOMES

- To develop policies, procedures and tools to guide the SHF implementation of the P/CWHS
- To develop capacities of key DOH, LGUs and other stakeholders on SHF implementation
- To monitor, together with other stakeholders, the implementation of SHF

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

► **LEGAL AND POLICY BASIS**

- Section 20 of the UHC Act (RA 11223) states that province/city-wide health system shall pool and manage, through a special health fund, all resources intended for health services to finance population-based and individual-based health services, health system operating costs, capital investments, and remuneration of additional health workers and incentives for all health workers
- DOH-DBM-DOF-DILG-PhilHealth JMC 2021-0001, Guidelines on the allocation, utilization, and monitoring of, and accountability for the SHF
- DOH DM 2023-0168 or Interim Guidelines on the Use and Management of the Special Health Fund (April 24, 2023)
- DOH DC 2025-0120 or Special Health Fund Manual of Procedures (March 13, 2025)

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)



LEGAL AND POLICY BASIS

Issuance	Title
DOH-DBM-DOF-DILG-PHIC Joint Memorandum Circular 2021-0001	Guidelines on the Allocation, Utilization, and Monitoring of, and Accountability for, the Special Health Fund (January 13, 2021)
DOH Department Memorandum 2023-0168	Interim Guidelines on the Use and Management of the Special Health Fund (April 24, 2023); ongoing revision
DOH Department Memorandum 2023-0403	Transfer of DOH Financial Grants to the SHF Account of P/CWHS beginning FY 2024 (November 10, 2023); ongoing revision
DOH Department Circular 2025-0120	Dissemination of the Special Health Fund Manual of Operations (March 13, 2025)

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)**LEGAL AND POLICY BASIS**

Issuance	Title
Commission on Audit Circular 2023-003	Accounting and Reporting Guidelines for the SHF for P/CWHS pursuant to Section 20 of the IRR of RA 11223 (June 14, 2023)
COA Circular No. 2025-002	General Accounting Guidelines on Provider Payment Mechanisms for the Implementation of the PhilHealth's PPM for Health Care Provider Networks (HCPNs) and Directly Contracted Health Facilities (January 28, 2025)
BLGF Memorandum Circular 012.2025	Guidelines on the Submission of Reports on Special Health Fund Utilization of Local Government Units (March 24, 2025)

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

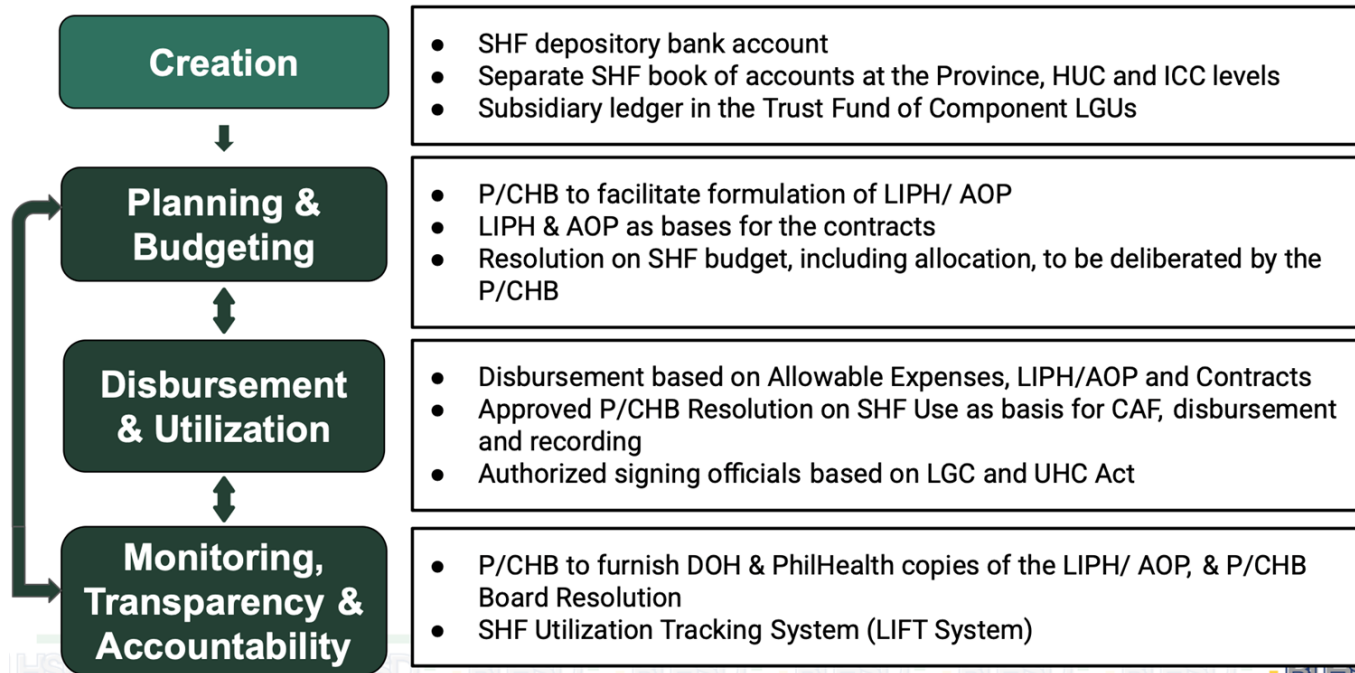
► KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

- SHF is a type of LGU Special Fund deemed automatically appropriated for health expenditures
- Being implemented only by the UHC Integration Sites (Province, HUCs, ICCs only)
- It is being managed by the Provincial/ City Health Board (P/CHB), to be assisted by the Provincial/ City Health Office and Management Support Unit (MSU), in accordance with existing budgeting, accounting and auditing rules and regulation
- The MSU (Accountant, Administrative Officer, Liaison Officer) serves as the Administrative Secretariat of the P/CHB which assists the P/CHB, provide administrative and technical support, and coordinate with necessary stakeholders
- Utilization and disbursement to depend on the following:
 - o Allowable expenses chargeable against the SHF as specified in the JMC 2021-0001 on SHF
 - o LIPH and AOP
 - o Contractual arrangement of the P/CHB with DOH, PhilHealth, CSOs, International Health Partners

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS



LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

► BASIS OF PRIORITIZATION OR TARGETING

- All UHC integration Sites which committed to integrate their LHS into P/CWHS shall exhibit financial integration by FY 2023-2025, hence shall establish and implement SHF
- DOH Financial Assistance to the P/CWHS coursed to the SHF include the following:
 - LHSDA UHC Grants and Fixed Tranche which are computed on the basis of number of population, number of component cities/municipalities, number of unserved and underserved areas, fund utilization and performance
 - Health Promotion Grants for the implementation of HP playbooks and other health-related PPAs
- Section 21 of the UHC Act states that all income derived from PhilHealth payments shall accrue to the Special Health Fund to be allocated by the LGUs exclusively for the improvement of the LGU health system
- Management of the SHF (including resource allocation to component LGUs) is by the P/CHB of the P/CWHS

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)► **PERFORMANCE INDICATORS AND TARGETS**

INDICATORS	2024		2025		2026	
	TARGETS	ACCOMPL.	TARGETS	ACCOMPL.	TARGETS	ACCOMPL.
Number of UHC IS with established SHF Depository bank accounts	58	80 (137%)	58/102	87/102 (85%)	85%	
Number of UHC IS with SHF utilization	58	19 (33%)	58/102	39/102 (38%)	40%	

Total Number of Provinces/ HUC/ ICC: 120

No. of UHC Integration Sites: 102

Note: Reported accomplishments for **2025** are based on data as of **December 31, 2025**, while those for **2026** reflect data as of **March 31, 2026**

Data on utilization is based on LHS ML 2024 Annual Report AND Q2 2025 SHF Report submitted by CHDs

Yearly targets FOR 2024 and 2025 are based on the LHS ML policy

Target for 2026 is based on the HCFS 2026-2028 Midterm Update (DC 2026-0576)

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)► **PERFORMANCE INDICATORS AND TARGETS**

SHF DEPOSITS TOTAL					SHF TOTAL	TOTAL UTILIZATION	% Utilization
Province	Municipalities/ CC	CHD/DOH	PhilHealth	Others			
27,256,630	21,348,290	278,742,867	2,882,196,037	95,809,206	3,305,353,030	1,179,855,412	36%
0.82%	0.65%	8.43%	87.20%	2.90%			

*Based LHS ML CY 2025 Year-End Report (DC 2026-0122) and Reports submitted by CHDs

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)**PROGRAM
PROFILE****► BLHSD PROGRAM ACCOMPLISHMENT (2023-2025)**

	2023	2024	2025
Policy/ Reports	DM 2023-0168: Interim Guidelines on the Use and Management of the Special Health Fund	DM 2023-0403: Transfer of DOH Financial Grants to the SHF Account of Province-wide and City-wide Health Systems beginning FY 2024	DC 2025-0120 Dissemination of the SHF Manual of Operations
Technical Assistance	Orientation on the SHF of CHDs and MOH-BARMM and other key stakeholders (DBM, DILG, DOF, PhilHealth, DOH Offices, Development Partners, etc.)	Continuing provision of TA on SHF implementation to CHDs and MOH-BARMM and other stakeholders	Orientation on the SHF Manual of Operations to SHF of CHDs and MOH-BARMM and other key stakeholders
Monitoring	Initial monitoring of SHF implementation thru the LHS ML	- DM 2024-0412: Request for Updates on the Status of Implementation and Utilization of the SHF by the UHC IS - Initial work on the SHF Utilization Tracking System - Monitoring of SHF implementation thru the LHS ML	- Continuing quarterly monitoring thru submission of CHD reports (per DM 2024-0412) - Monitoring of SHF implementation thru the LHS ML - Consultation & Review of LIPH and SHF implementation

Note: Reported accomplishments for **2024** are based on data as of **December 31, 2024**, while those for **2025** reflect data as of **June 30, 2025**.

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

► CHDs/MOH BARMM PROGRAM ACCOMPLISHMENT (2023-2025)

	ACCOMPLISHMENTS (2025)	CHALLENGES	NEXT STEPS
NCR	● 6/14 UHC IS with SHF account (Marikina, Paranaque, Valenzuela, Pasig, Manila, San Juan) ● 2 UHC IS with Utilized SHF (Paranaque and Manila)	Subsidiary ledgers in the nominal account (Income and Expense) cannot be created on electronic new government accounting system	Can be done manually for the purpose of reporting. LGU may coordinate with Regional COA how SHF be distributed against income source in the disbursement aspect.
CAR	● 7/7 UHC IS with SHF account (Benguet, Ifugao, Kalinga, Mountain Province, Baguio City, Abra, Apayao) ● 6 UHC IS with Utilized SHF (Baguio City, Apayao, Benguet, Mt. Province, Ifugao, Kalinga)	Provision on UHC Act re use of SHF for remuneration of "additional" HRH is limiting its use to existing health personnel	Proposed this amendment for HEAL of the 20th Congress, subject to the Secretary's approval.
Ilocos	● 5/5 UHC IS with SHF account (Ilocos Norte, Ilocos Sur, La Union, Pangasinan, Dagupan City) ● 2 UHC IS with Utilized SHF (La Union, Pangasinan)	-liquidation issues -Provinces still on the process of drafting their own SHF guidelines	CHD, in coordination with other agencies, to provide necessary TA; LGUs to refer to SHF MOP which included sample policies from some LGUs as reference



LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

PROGRAM PROFILE

► CHDs/MOH BARMM PROGRAM ACCOMPLISHMENT (2023-2025)

	ACCOMPLISHMENTS (2025)	CHALLENGES	NEXT STEPS
Cagayan Valley	•6/6 UHC IS with SHF account (Nueva Vizcaya, Quirino, Santiago City, Batanes, Cagayan, Isabela) •5 UHC IS with utilized SHF (Batanes, Cagayan, Nueva Vizcaya, Quirino, Santiago City)	No local policy on allocation and distribution of SHF; Funding limited to grants provided by the DOH; HCPN contracting on hold	CHD, in coordination with other agencies, to provide necessary TA; LGUs to refer to SHF MOP which included sample policies as reference; Refer to PhilHealth CO for clarifications on the reimbursement process for SHF
Central Luzon	•4/7 UHC IS with SHF account (Bataan, Tarlac, Bulacan, Pampanga, Nueva Ecija) •3 UHC IS with utilized SHF (Bataan, Tarlac, Bulacan)	PhilHealth ending of PCPN contracting and HCPN contracting put on hold	Meet with PhilHealth and propose downloading and transfer of PhilHealth reimbursements to SHF
CALABAR ZON	•4/4 UHC IS with SHF account (Batangas, Laguna, Quezon, Lucena City) •2 UHC IS with utilized SHF (Quezon, Batangas)	No local policy on allocation and distribution of SHF; Weak health financing and public financial management in LGU	Ongoing development of HHRDB and BLHSD of the Capacity Development for LHBs; Coordinate with DBM and DOF for capacitation of LGUs on PFM (as per JMC); continue BLHSD learning exchange sessions

Note: Reported accomplishments for 2024 are based on data as of December 31, 2024, while those for 2025 reflect data as of June 30, 2025.



LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

► CHDs/MOH BARMM PROGRAM ACCOMPLISHMENT (2023-2025)

	ACCOMPLISHMENTS (2025)	CHALLENGES	NEXT STEPS
MIMAROPA	6/6 UHC IS with SHF accounts 2 UHC IS with utilized SHF (Occidental Mindoro and Romblon)	-Instruction from PRO4B that reimbursement to follow current process of infusing PhilHealth earning to the Trust Fund for Health.	Refer to PhilHealth CO for clarifications on the reimbursement process for SHF
Bicol	3/5 UHC IS with SHF accounts (Catanduanes, Sorsogon, Camarines Norte) 1 UHC IS with utilized SHF (Sorsogon)	-	-
Western Visayas	5/6 UHC IS with SHF accounts (Aklan, Antique, Capiz, Guimaras, Iloilo) 2 UHC IS with utilized SHF (Aklan, Capiz)	-	-

Note: Reported accomplishments for **2024** are based on data as of **December 31, 2024**, while those for **2025** reflect data as of **June 30, 2025**.

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

► CHDs/MOH BARMM PROGRAM ACCOMPLISHMENT (2023-2025)

	ACCOMPLISHMENTS (2025)	CHALLENGES	NEXT STEPS
Central Visayas	4/5 UHC IS with SHF accounts (Bohol, Cebu, Mandaue City, Lapu-lapu City) 1 UHC IS with utilized SHF (Mandaue City)	Ensure all partners donating or transferring money are informed of the new bank account of LGU for health	LGUs communicated officially and/updated data in forms used for renewal application (ex. Philhealth)
Negros Island Region	2/2 UHC IS with SHF accounts (Negros Occidental, Bacolod City)	-	-
Eastern Visayas	6/6 UHC IS with SHF accounts (Biliran, Northern Samar, Samar, Southern Leyte, Eastern Samar, Leyte) 2 UHC IS with utilized SHF (Biliran and Samar)	liquidation issues; No local policy on allocation and distribution of SHF	CHD, in coordination with other agencies, to provide necessary TA; LGUs to refer to SHF MOP which included sample policies from some LGUs as reference

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)**► CHDs/MOH BARMM PROGRAM ACCOMPLISHMENT (2023-2025)**

	ACCOMPLISHMENTS (2025)	CHALLENGES	NEXT STEPS
Zamboanga Peninsula	• 3/4 UHC IS with SHF accounts (Zamboanga del Norte, Zamboanga del Sur, Zamboanga Sibugay) • 3 UHC IS with utilized SHF (Zamboanga del Norte, Zamboanga del Sur, Zamboanga Sibugay)	-	-
Northern Mindanao	• 6/7 UHC IS with SHF accounts (Lanao del Norte, Misamis Occidental, Misamis Oriental, Cagayan de Oro City, Camiguin, Iligan City) • 1 UHC IS with utilized SHF (CDO)	-	-
Davao	• 6/6 UHC IS with SHF accounts (Davao de Oro, Davao del Norte, Davao Oriental, Davao City, Davao del Sur, Davao Occidental) • 5 UHC IS with utilized SHF (Davao de Oro, Davao del Norte, Davao Oriental, Davao City)	No local policy on allocation and distribution of SHF; Funding limited to grants provided by the DOH; HCPN contracting on hold	CHD, in coordination with other agencies, to provide necessary TA; LGUs to refer to SHF MOP which included sample policies as reference; Refer to PhilHealth CO for clarifications on the reimbursement process for SHF

LOCAL HEALTH FINANCING (SPECIAL HEALTH FUND)

► CHDs/MOH BARMM PROGRAM ACCOMPLISHMENT (2023-2025)

	ACCOMPLISHMENTS (2025)	CHALLENGES	NEXT STEPS
SOCCSKSARGEN	5/5 UHC IS with SHF accounts (Sarangani, South Cotabato, Sultan Kudarat, North Cotabato, General Santos) 1 UHC IS with utilized SHF (South Cotabato)	-liquidation issues, PCPN contracting ended and HCPN contracting is put on hold	CHD, in coordination with other agencies, to provide necessary TA; Refer to PhilHealth CO for clarifications on the reimbursement process for SHF
CARAGA	6/6 UHC IS with SHF accounts (Surigao del Norte, Agusan del Sur, Butuan City, Surigao del Sur, Dinagat Islands, Agusan del Norte)	-	-
BARMM	2/2 UHC IS with SHF accounts (Basilan, Maguindanao) 1 UHC IS with utilized SHF (Maguindanao)	-	-

*Notes: Reported accomplishments for 2024 are based on data as of December 31, 2024, while those for 2025 reflect data as of June 30, 2025.

LOCAL HEALTH FINANCING (LGU HEALTH ACCOUNTS)

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

- Provide policies, guidelines and strategic direction on the implementation of the LGU Health Accounts
- The LGU Health Accounts (LGU HA) is a mechanism to track and analyze health expenditures at the local level to inform LGU planning, resource allocation and spending for health. It aims to answer these questions on health financing: i) Who spent for health care?, ii) Where and to whom was it spent? and, iii) How was it spent?

► OBJECTIVES AND INTENDED OUTCOMES

- To provide guidelines on the tracking and analysis of the expenditures for local health systems
- To provide and assist the LGUs with a tool for evidence-based and equitable planning, resource allocation and spending for health.
- To develop capacities of CHDs and LGUs on LGU Health Accounts

LOCAL HEALTH FINANCING (LGU HEALTH ACCOUNTS))

LHA estimation in the Four UHC Integration Sites: Baguio City, Benguet, Aklan, Guimaras

Objectives

Demonstrate LHA as a tool to inform local health planning and policy development

Produce 2021-2023 LHA estimates in the select P/CWHS

Provide recommendations for improving LHA estimation in line with current health financing directions

Assess feasibility of upscaling LHA estimation in other P/CWHS

Methodology

01

Reconstitution of the TWG

02

Selection and orientation of LGUs

03

LHA estimation

04

Data validation

05

Consolidation and analysis

LOCAL HEALTH FINANCING (LGU HEALTH ACCOUNTS)

LHA estimation in the Four UHC Integration Sites: Baguio City, Benguet, Aklan, Guimaras

Limitations in the Estimation



Dependence on the completeness
and availability of data sources



No local data for OOP (prorated
only from National OOP)



Exclusion of capital outlays



Exclusion of other
sources of funds



Exclusion of private sector health spending



Exclusion of lumped expenditures
which the LGU cannot disaggregate



Years covered (2021-2023) included
pandemic or **COVID period**

LOCAL HEALTH FINANCING (LGU HEALTH ACCOUNTS))

LHA estimation in the Four UHC Integration Sites: Baguio City, Benguet, Aklan, Guimaras

Summary of Results

Findings on the Health Spending

- Dominance of household OOP spending
- Concentration of spending in hospitals and curative services
- Lesser spending on preventive care and governance

Findings on the LHA Estimation Process

- Difficulty in getting and disaggregating data
- Confusion and non-familiarity of the LHA Terms in SHA 2011. LGUs, DOH and other stakeholders need uniform guide on the definition and application of LHA terms and processes
- Exclusion of some fund sources e.g. Trust Fund, Revolving Funds, Consignment, supplemental funds, ILHZ, SHF and capital accounts in the estimation may have undermined the actual local health spending
- No dedicated person/s office/s for LHA

LOCAL HEALTH FINANCING (LGU HEALTH ACCOUNTS))

Recommendations and Proposed Ways Forward

1. **Develop Policy Guidance** on the implementation of local health expenditure tracking (LGU Health Accounts) anchored on the Health Care Financing Strategy (2026)
1. **Inter-agency discussions and high level engagement:** DOH, BLGF, PSA, PhilHealth, and other relevant agencies to support the LGU HA integration into LIFT
1. **Proceed with next iteration of targeted roll-outs** incorporating Process Enhancements (2027)

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

- The Department of Health unit under the BLHSD that focuses on strengthening local leadership and governance in health, particularly to help LGUs and local health boards (LHBs) carry out their roles under the local Government Code and the Universal Health Care Act.

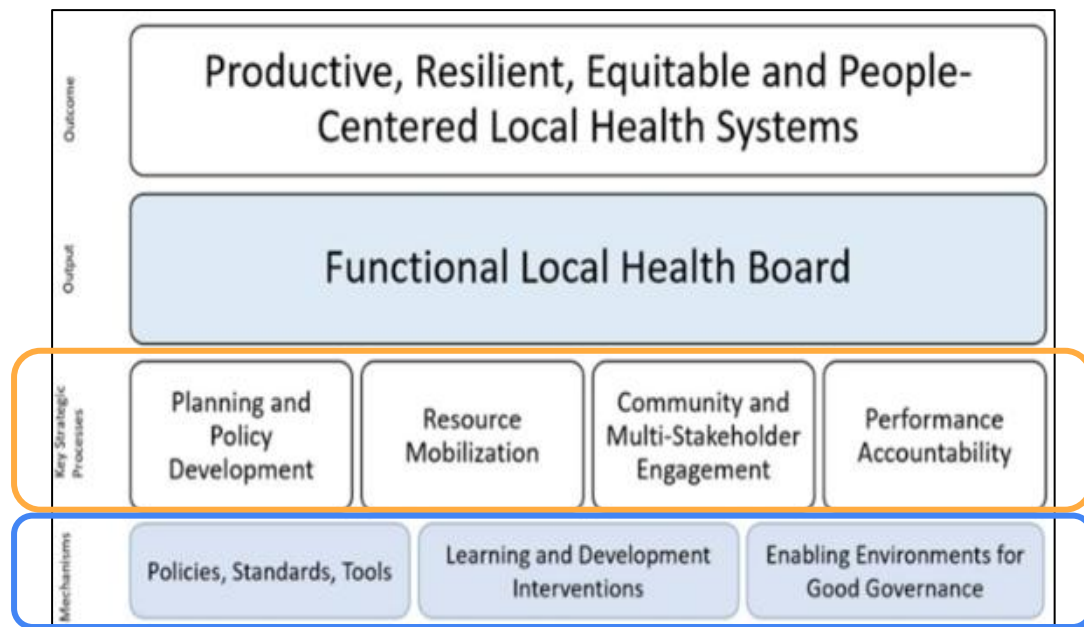
► OBJECTIVES AND INTENDED OUTCOMES

- To strengthen the leadership and governance capacities of LHBs, local chief executives (LCEs), local leaders and the Department of Health representatives in order to improve the governance, performance and responsiveness of local health systems towards the effective implementation of UHC with the following intended outcomes:
 - More functional local health boards
 - Stronger leadership among LCEs, LHB members and DOH representatives
 - Improved responsiveness of local health systems to local population health needs
 - Progress towards productive, equitable, resilient and people-centered local health systems

**LEGAL AND POLICY BASIS**

- *Republic Act No. 7160* or the Local Government Code of 1991
- *Republic Act No. 11223* or the Universal Health Care Act
- DOH Rules and Regulations Implementing the Local Government Code
- *Administrative Order No. 2020-0029* dated July 9, 2020 or Roles, Functions and Responsibilities of DOH Representatives
- *DOH-DILG Joint Administrative Order No. 2022-0001* dated 02 June 2022 or Policy Framework on Leadership and Governance for Health (LeadGov4Health) Towards a Functional Local Health Board
- Partnership Agreement between DOH (HHRDB) and Local Government Academy
- *DILG Memorandum Circular No. 2025-060* or the Guidelines in the Accreditation of Civil Society Organizations and Selection of Representative to the Local Special Bodies
- *DM No. 2025-0024* Interim Guidelines on PuroKalusugan: An Approach to Improving Primary Health Care Service Delivery Directly to the Underserved and Unserved Filipinos
- *DM No. 2025-0123* Support to DOH Representatives in the Implementation of PuroKalusugan
- *DM No. 2025-0421* Guidelines on the Roll-out of the DOH Representative Capacity Development Program
- *DM No. 2025-0455* Strengthening the Reporting System of the Department of Health (DOH) Representatives in their Local Government Unit of Assignment
- *DC No. 2026-0099* Dissemination of the Guidebook on Operationalizing the Local Health Board

► KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS



The LeadGov4Health Framework serves as a guide to the LGUs in achieving a functional LHB towards the achievement of a productive, resilient, equitable and people-centered health system.

Effectiveness of the LeadGov4Health framework and its components shall be measured through the monitoring and evaluation of the functionality of the LHB & its impact on its LHS.

➡ 4 key strategic processes in streamlining the functions and activities of the LHB

➡ These mechanisms are imperative to ensure the functionality of the LHB, and shall be a shared responsibility among the LGUs, DOH, DILG and LGA



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

Operationalized alongside local health systems maturity monitoring, local health planning through the LIPH and monitored through the LGU Health Scorecard

FOR THE LOCAL HEALTH BOARD

- Development of policies, tools, manuals on local health systems related to leadership and governance for health.
- Support to capacity development programs for LHBs
- Partnership and networking with DILG, LGA and other concerned stakeholders to improve local policies/ tools on leadership and governance for health.
- Monitor LHB Functionality through the LGU Health Scorecard and the LHSML
- Development of Guide on Operationalizing the Local Health Board.
- Development of Key Messages to the LCE and LHB on LHS and other key public health programs
- Development of Performance Accountability System for LHB
- Development of LHB Functionality Assessment Tool

► **KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS**

FOR THE DOH REPRESENTATIVES

- Development of policies, tools and manuals essential to provision of technical assistance on leadership and governance for health.
- Upgrading of DOH Rep Assistant Application and Local Health Systems Toolkit.
- Annual provision of fund augmentation for the Provincial DOH Offices.
- Development of Capacity Development Program on Leadership and Governance on Universal Health Care
- Conduct of DOH Representative Convention.

► **PERFORMANCE INDICATORS AND TARGETS**

INDICATORS	2023		2024		2025	
	TARGETS	ACCOMPL.	TARGETS	ACCOMPL.	TARGETS	ACCOMPL.
Number of LGUs with Functional LHBs	Baseline data collection	51% <i>Provinces: 41.98% (34/81)</i> <i>HUC and ICC: 73.68% (28/38)</i> <i>Municipalities: 43.92% (701/1,596)</i>	100%	72% <i>Provinces: 80% (66/82)</i> <i>HUC and ICC: 89% (34/38)</i> <i>Municipalities: 71% (1,142/1,604)</i>	100%	No data yet

Note: Collection of 2025 performance data will be done in 2026

► **BLHSD PROGRAM ACCOMPLISHMENT (2023-2025)**

	2023	2024	2025
Policy/ Reports	Development of Design of Leadership Capacity Development (CapDev) Program for the DOH Representative and its MOP		Issuance of DM No. 2025-0421 <i>Roll-out of the DOH Representative CapDev Program</i>
		Development of the Guidebook on Operationalizing the Local Health Board	
			Issuance of: - DM No. 2025-0123 <i>Support to DOH Representatives in the Implementation of PK</i> - DM No. 2025-0455 <i>Strengthening the Reporting System of the DOH Representatives in their LGU of Assignment</i>
Technical Assistance	Enhancement of the DOH Representative Assistant App, and updating of the Local Health System Toolkit		
		Conduct of the National Convention of DOH Representatives	National implementation of the DOH Representative CapDev Program
Monitoring	Inclusion of LHB Functionality in the LGU HSC and baseline collection of data		

LEADERSHIP AND GOVERNANCE FOR HEALTH (LEADGOV4HEALTH)**PROGRAM
PROFILE****BUDGET SUMMARY**

	FY 2022 (DM 2021-0458)	FY 2023 (DM 2022-0436)	FY 2024 (DM 2023-0350)	FY 2025 (DM 2024-0323)	FY 2026 (GAA-based)	2027 Proposed
Metro Manila	1,700,000	2,600,000	3,367,375	3,367,375	24,316,919	24,316,919
Ilocos	500,000	2,000,000	2,902,268	2,902,268	23,553,033	23,553,033
Cordillera	700,000	2,800,000	4,152,262	4,152,262	24,442,027	24,442,027
Cagayan Valley	600,000	2,400,000	3,544,849	3,544,849	24,135,614	24,135,614
Central Luzon	900,000	2,100,000	2,942,807	2,942,807	25,093,572	25,093,572
Calabarzon	600,000	1,800,000	1,900,000	1,900,000	23,330,765	23,330,765
Mimaropa	600,000	2,100,000	2,200,000	2,200,000	22,009,568	22,009,568
Bicol	700,000	1,600,000	1,929,780	1,929,780	22,639,163	22,639,163
Western Visayas	800,000	2,600,000	3,191,917	2,443,938	24,174,135	24,174,135
NIR				1,147,979	20,658,145	20,658,145
Central Visayas	700,000	1,600,000	2,339,834	1,939,834	22,589,766	22,589,766
Eastern Visayas	800,000	2,600,000	3,574,387	3,574,387	24,464,152	24,464,152
Zamboanga Peninsula	500,000	1,400,000	1,680,672	1,680,672	21,730,437	21,730,437
Northern Mindanao	700,000	1,900,000	2,222,743	2,222,743	23,112,508	23,112,508
Davao	600,000	1,800,000	2,061,667	2,061,667	22,531,432	22,531,432
Soccsksargen	500,000	1,400,000	1,880,952	1,880,952	22,050,717	22,050,717
Caraga	600,000	1,500,000	2,399,053	2,399,053	23,408,818	23,408,818
TOTAL	11,500,000	32,200,000	42,290,566	42,290,566	394,240,771	394,240,771

Equity in Health and Special Concerns

Equity in Health and Special Concerns Quick Links

- PuroKalusugan
 - General Information: <https://bit.ly/PKNotion>
 - 2026 Performance Dashboard: https://bit.ly/purokalusugan_dashboard
 - 2026 Fund Monitoring Dashboard: <http://bit.ly/2026PKfunds>
- Support to Barangay Health Workers
 - BHW Connect: bhw.doh.gov.ph
- Unserved and Underserved Communities for Primary Health Care (UUC PHC)
 - 2025 List of UUC PHC: <https://bit.ly/2025UUC4PHCLIST>

INDIGENOUS PEOPLES' HEALTH

LEGAL AND POLICY BASIS

- United Nations Declaration on the Rights of Indigenous Peoples (UNDRIP)
- Indigenous Peoples' Rights Act (IPRA)
- Universal Health Care Act (RA 11223)
- DOH-NCIP-DILG Joint Memorandum Circular No. 2013-01 dated 19 April 2013, "Guidelines on the Delivery of Basic Health Services for Indigenous Cultural Communities/Indigenous Peoples"
- DOH-NCIP-DILG Joint Memorandum Circular No. 2015-01 dated 2 December 2015, "Creation of Inter-Agency Committees on Indigenous Peoples' Health"
- Administrative Order No. 2020-0023 dated May 27, 2020, "Guidelines on Identifying Geographically-Isolated and Disadvantaged Areas and Strengthening their Health Systems"
- Department Memorandum No. 2024-0505 on Dissemination of the Indigenous Peoples Strategic Plan for Health 2025-2028
- NCIP Memorandum Order No. 0151 "Cascading and Adoption of the IP Strategic Plan for Health 2025-2028 in Support to the JMC 2013-01 of DOH-NCIP-DILG"
- DILG Memorandum dated February 17, 2025 with the subject "Indigenous Peoples Strategic Plan for Health 2025-2028"
- DC No. 2025 - 0572 on Dissemination of the IP Operational Plan for Health 2026

INDIGENOUS PEOPLES' HEALTH



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

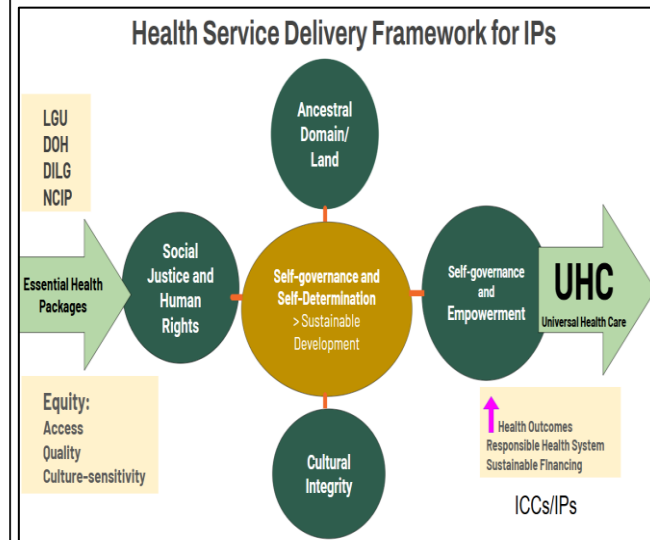
PROGRAM PROFILE

UHC Vision for ICCs/IPs

- Improved health outcomes
- Culturally responsive health systems
- Sustainable health financing for IP health services
- **Key feature:** delivery of Essential Health Packages (EHPs)

Key Considerations

- LGU, DOH, NCIP, and DILG must integrate social, cultural, human rights, and social justice factors in EHP delivery
- Under the IPRA (RA 8371), ICCs/IPs have four (4) rights:
 - Ancestral domain and land
 - Social justice and human rights
 - Self-governance and empowerment
 - Cultural integrity
- Health services must respect these rights and support self-determination and sustainable development



INDIGENOUS PEOPLES' HEALTH



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

Principles of Essential Health Package Delivery

- Improve equity in access, availability, quality, and culturally sensitive health services
- Respect and promote safe and beneficial Indigenous culture and practices
- Integrate medicine and science with ICC/IP culture and rights
- Ensure safe, effective, and sustained quality health outcomes

Sustainability and Empowerment

- Health services must be sustainable
- ICCs/IPs should be empowered to:
 - Influence Essential Health Package delivery
 - Integrate services into community health systems
 - Manage and govern their own health systems

Goal

- Achieve UHC through collaboration between ICCs/IPs and government agencies, etc.

INDIGENOUS PEOPLES' HEALTH

PROGRAM PROFILE

► PROGRAM STATUS/PERFORMANCE

- With existing Culture-Sensitivity and Competency Training Manual use for training of health workers and Program Managers
- Developed and uploaded to the DOH Academy e-learning platform the materials on AO No. 2020-0023, “Guidelines on Identifying Geographically-Isolated and Disadvantaged Areas and Strengthening their Health Systems” for the UHC Implementers’ Course of the DOH Academy.
- Conduct of the National Assessment of IP Strategic Plan for Health 2018-2022
- Issuance of the following:
 - DM No. 2024-0505, “Dissemination of Indigenous Peoples Strategic Plan for Health 2025-2028
 - DC No. 2025-0572, “Dissemination of the Indigenous Peoples (IP) Operational Plan for Health 2026
 - DM No. 2025-0303, “Submission of Regional IP Strat Plan for Health 2025-2028 and its OPlan 2025-2026

INDIGENOUS PEOPLES' HEALTH

PROGRAM PROFILE

► PROGRAM STATUS/PERFORMANCE

- Issuance of the following:
 - DPO No. 2025-1404, “Designation of IP Health Permanent and Alternate Focal Point Persons (DOH-CO, attached agencies, select NGAs)
 - DPO No. 2026-0776 - Cascading of the IP OPlan 2026
 - DPO No. 2026-6875 - Culture Sensitivity Training/Consultation on the Global Plan of Action of IPs and OPlan for Health 2026

INDIGENOUS PEOPLES' HEALTH

PROGRAM PROFILE

► PROGRAM STATUS/PERFORMANCE

Technical Assistance:

- Culture-Sensitivity and Competency Training for the Program Managers of DOH-CO, attached agencies, select NGAs
- Adoption/Regionalization of IP Strategic Plan for Health 2025-2028
- DM No. 2025-0170 - Virtual Cascading of the IP Strat Plan for Health 2025-2028 and IP OPlan for Health 2025-2026
- DM No. 2026-0078 - Virtual Cascading of 2026 IP Operational Plan Health
- Online Consultation on Global Plan of Action for the Health of IPs (DOH Philippines as the co-chair)
- Online Regional Consultation on Global Plan of Action for the Health of IPs (Western Pacific Region)

INDIGENOUS PEOPLES' HEALTH

PROGRAM PROFILE

▶ PROGRAM STATUS/PERFORMANCE

- Number of scholars from IP areas & unserved and underserved communities (UUCs) enrolled in the Pre-Service Scholarship Program:

DOH Scholarship Program as of July 2026 (% HHRDB)		
Cadre	Total Scholars	Scholars from IP Areas & UUCs
Medicine	259	165
Pharmacy	114	107
Medical Technology	97	90

INDIGENOUS PEOPLES' HEALTH

PROGRAM PROFILE

► PROGRAM STATUS/PERFORMANCE

- Technical assistance provision to CHDs and MOH BARMM through Sub-allotment/Transfer of Funds
- HRH Deployment Program

Deployment Program Number of HRH Deployed in UUCs & IP Areas as of July 2026 (c/o HHRDB)			
Doctors	Nurses	Midwives	Total
598	7,366	3,604	11,568

UNSERVED AND UNDERSERVED COMMUNITIES FOR PRIMARY HEALTH CARE (UUC for PHC)

PROGRAM PROFILE

► Unserved and Underserved Areas (UUAs) (Formerly Geographically Isolated and Disadvantaged Areas or GIDA)

Refer to barangays specifically disadvantaged due to the presence of both physical and socio-economic factors:

- **Physical factors** are characteristics that **limit the delivery of and/or access to basic health services** to communities that are **difficult to reach** due to distance, weather conditions, and transportation difficulties.
- Socio-economic factors are **social, cultural, and economic characteristics of the community** that **limit access** to and utilization of health services

► OBJECTIVES AND INTENDED OUTCOMES

Improve access to quality health care through province-wide/city-wide health systems, and equitable and sustainable health financing in UUAs (formerly GIDAs)

UNSERVED AND UNDERSERVED COMMUNITIES FOR PRIMARY HEALTH CARE (UUC for PHC)

PROGRAM PROFILE

► **LEGAL AND POLICY BASIS**

- Universal Health Care Act (RA 11223)
- Administrative Order No. 2020-0023 dated May 27, 2020, “Guidelines on Identifying Geographically-Isolated and Disadvantaged Areas and Strengthening their Health Systems”
- Department Memorandum No. 2024-0508 dated December 23, 2024, Unservd and Underserved Area (UUA) Profiling, Validation, and Encoding Using UUA 2025 Toolkit”
- Department Memorandum No. 2025-0305 dated July 2, 2025, “Update on the 2025 Profiling for the Unservd and Underserved Areas (UUA) Encoding and Extension of Deadline”
- Department Circular No. 2025-0549, “Dissemination of the 2025 Unservd and Underserved Communities for Primary Health Care List”
- Department Memorandum No. 2026-0063, “Annual Updating of the Unservd and Underserved Communities for Primary Health Care for CY 2026”

UNSERVED AND UNDERSERVED COMMUNITIES FOR PRIMARY HEALTH CARE (UUC for PHC)

PROGRAM PROFILE

► KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

Profiling of the barangays is done by Local Government Units with assistance from the Centers for Health Development and the MOH - BARMM **every 3 years**, and is being **updated annually**. The profiling follows the following criteria (AO 2020-0023)

- **Physical Factor**
 - at least 25% of sitios/ puroks have no access to an RHU nor a hospital within 60 minutes of travel in any form of transport, including walking
- **Socio-Economic Factor – at least ONE of the following:**
 - At least 10% of its population are IPs;
 - At least 10% of its population are affected by armed-conflict or internally displaced or barangay is identified as a Communist Terrorist Group (CTG)/ Local Extremist Group (LEG) area by the National Intelligence Coordinating Agency;
 - At least 50% of its population are enrolled in 4Ps/CCT;
 - Performance of barangay in at least 4 indicators is less than their latest provincial data (IMR, U5MR, FIC, CPR, Adolescent Birth Rate, ANC, SBA, Access to improved water supply)

UNSERVED AND UNDERSERVED COMMUNITIES FOR PRIMARY HEALTH CARE (UUC for PHC)

PROGRAM PROFILE

► BASIS OF PRIORITIZATION OR TARGETING

- Once barangays are profiled, encoded, and validated, BLHSD releases a list for the UUAs (formerly GIDAs).
 - **Annual updating and issuance of UUA List** through the DOH website and technical reports based on AO 2020-0023:
 - DM No.2020-0490 : GIDA List 2020
 - DM No. 2021-0525: GIDA List 2021
 - DM No. 2022-0567: GIDA List 2022
 - DM No. 2023-0409: GIDA List 2023
 - DM No. 2024-0459: UUA List 2024
 - DC No. 2025-0549: 2025 UUC for PHC List
- The released list is used by other DOH Offices for **prioritization of health service provision** (e.g. Deployment through the National Health Workforce Support System).
- Several requests are also handled by BLHSD, either through eFOI or via email, for the issuance of the list for different purposes (e.g. research, policy making, etc.)

List of 2025 Unserved and Underserved Areas (UUA)

Distribution of UUC for PHC Barangays by Region (as of 2025 per DC No. 2025-0549)

PROGRAM PROFILE

SUMMARY	
REGION	NUMBER
BANGSAMORO AUTONOMOUS REGION IN MUSLIM MINDANAO (BARMM)	400
CORDILLERA ADMINISTRATIVE REGION (CAR)	609
REGION I (ILOCOS REGION)	262
REGION II (CAGAYAN VALLEY)	227
REGION III (CENTRAL LUZON)	129
REGION IV-A (CALABARZON)	195
MIMAROPA REGION	458
REGION V (BICOL REGION)	757
REGION VI (WESTERN VISAYAS)	397
REGION VII (CENTRAL VISAYAS)	82
REGION VIII (EASTERN VISAYAS)	358
NEGROS ISLAND REGION (NIR)	141
REGION IX (ZAMBOANGA PENINSULA)	523
REGION X (NORTHERN MINDANAO)	440
REGION XI (DAVAO REGION)	437
REGION XII (SOCCSKSARGEN)	304
REGION XIII (CARAGA)	268
TOTAL	5987

PUROKALUSUGAN

PROGRAM PROFILE

▶ PROGRAM/ACTIVITY/PROJECT DESCRIPTION

- Aim to accelerate the realization of the Universal Health Care Act's goals of providing equitable healthcare access to all Filipinos by **strengthening the service delivery at the purok level.**
 - Primary Health Care strategy
 - Multisectoral whole of society approach
 - Community-centered service delivery approach.
 - Focus: Unserved and Underserved.

▶ OBJECTIVES AND INTENDED OUTCOMES

- **Empowering communities** to fast-track achievement of the 8 Priority Health Outcomes.
 - Service Delivery – Accessibility, Responsiveness
 - Local Health Systems – Strengthening, Collaboration
 - Health Outcomes – Equity, Effectiveness

► LEGAL AND POLICY BASIS

1. Universal Health Care Act (RA 11223)
2. Department Circular No. 2025-0011 dated January 14, 2025, “Dissemination of the Interim Guidelines on PuroKalusugan”
3. Department Memorandum No. 2025-0024 dated January 14, 2025, “Interim Guidelines on PuroKalusugan: An Approach to Improving Primary Health Care Service Delivery Directly to the Underserved and Unserved Filipinos”
4. Department Memorandum No. 2025-0115 dated February 24, 2025, “Dissemination of the Barangay Health Worker (BHW) PK Manual”
5. Department Memorandum No. 2025-0123 dated March 5, 2025, “Support to the DOH Representatives in the Implementation of PuroKalusugan”
6. Department Memorandum No. 2025-0272 dated June 13, 2025, “Prioritization of Nutrition in the Implementation of PuroKalusugan”
7. Department Memorandum No. 2025-0377 dated August 18, 2025, “Dissemination of the Monitoring Tools for the Implementation PuroKalusugan”

► LEGAL AND POLICY BASIS

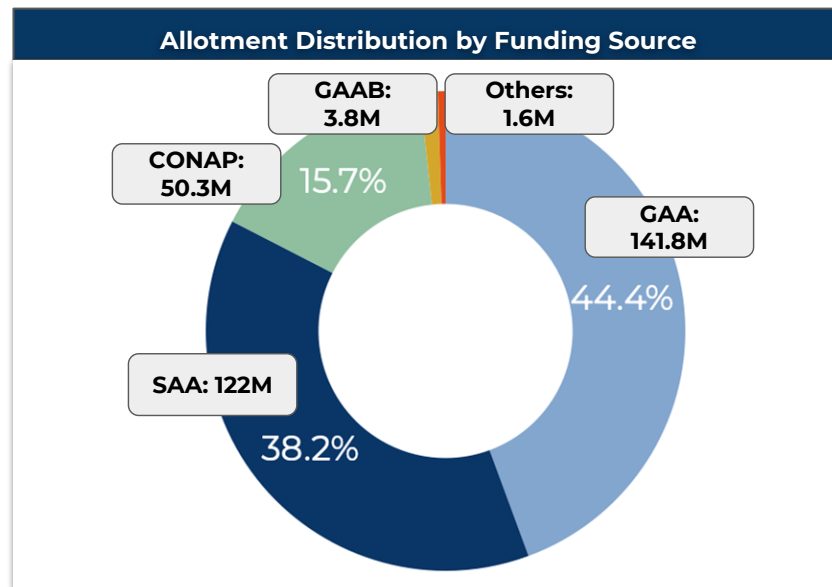
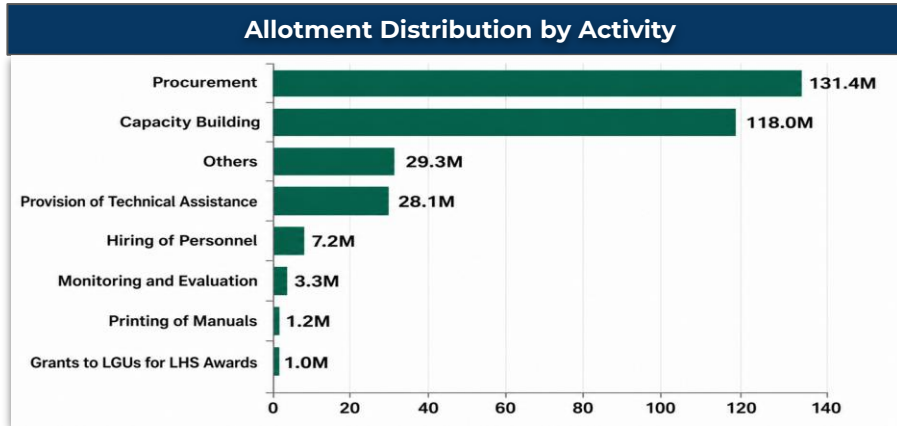
8. Department Memorandum 2026-0012, dated January 8, 2026, “Dissemination of Key Directives from the 1st PuroKalusugan Steering Committee Meeting”
9. Department Memorandum 2026-0060, dated January 15, 2026, “Guidelines of the Utilization of the Lab for ALL Funds under the Local Health SYstems Development Assistance Line Item”
10. Department Memorandum 2026-0071, dated February 4, 2026, “Interim Use of Excel Workbook for the Data Submission of 2026 PuroKalusugan Implementation”
11. Department Circular 2026-0314, dated March 30, 2026, Dissemination of the PuroKalusugan Implementation Report for Calendar Year 2025
12. Department Circular 2026-0315, June 10, 2026, Dissemination of the National and Regional PuroKalusugan Quarterly Performance Bulletins for the First Quarter of 2026
13. Department Memorandum 2026-0323, dated June 4, 2026, “Guidelines on the Reporting and Monitoring of PuroKalusugan Funds for Fiscal Year 2026”

PUROKALUSUGAN



PK Funding of All Regions *(Data as of Quarter 2 2026)*

Total PK Funds	319.5M
% Obligation	36.3%
% Disbursement	47.3%



Link to PK Fund Monitoring Dashboard: bit.ly/2026PKfunds

**AO Development, Consultation, and Processing Timeline**

Timeline	Process
July 2025	Conducted the first consultation with coordinators, BHW representatives, and other stakeholders
August 2025	Requested inputs on the draft PK AO from DOH Central Office Bureaus, CHDs, and MOH-BARMM
November 2025	Submitted the policy memorandum and draft PK AO to the Health Policy Development and Planning Bureau
December 2025	Requested concurrence on the draft PK AO from DOH Central Office bureaus, CHDs, and MOH-BARMM
January 2026	Revised the draft PK AO based on the comments and recommendations received
February 2026	Requested the Cluster Lead's review and clearance of the revised draft PK AO
March–July 2026	Continued coordination with the concerned office regarding the review and clearance of the revised draft AO

PUROKALUSUGAN

PROGRAM PROFILE

Region	HSRP Project Sites
II - Cagayan Valley	Isabela
V - Bicol	Sorsogon
	Masbate
VI - Western Visayas	Iloilo
VII - Central Visayas	Bohol
VIII - Eastern Visayas	Leyte
IX - Zamboanga Peninsula	Zamboanga Sibugay
	Zamboanga del Sur
X - Northern Mindanao	Misamis Oriental
	Lanao Del Norte
XI - Davao	Davao de Oro
	Davao del Norte
XII - SOCCSKSARGEN	Sultan Kudarat
	South Cotabato
XIII - Caraga	Agusan del Sur
BARMM	Maguindano del Norte
	Maguindanao del Sur



**Estimated Total Cost of
Tablet Devices to be
Procured: 993k**

Qty: 40 units

Status: Ongoing
Procurement



**Estimated Total Cost of PK
Kits to be Procured: 100M**

Qty: 10,770 kits

Status: Consolidated into a
single procurement package
(Ongoing PR revision)

Other Pending HSRP-funded projects:

- Consultancy service in BHW Cap Building = 71M
- Consultancy service for Data Systems = 10M

Status: Finalization of TOR



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

- Key Implementation Strategies
 - a. **Resource & Gap Analysis** – Community profiling to identify health needs and tailored strategies for accessibility.
 - b. **Targeted Health Interventions** – Address 8 Priority Health Outcomes (e.g., immunization, maternal health, NCDs). Starting with 3 barangays per municipality (2025).
 - c. **PuroKalusugan Health Teams (PKTs)** – Nurses, midwives, BHWs, and volunteers delivering services twice a week in priority areas.
 - d. **Digital Health Systems** – Timely monitoring via the PuroKalusugan Dashboard, telemedicine, and digital tools

► BASIS OF PRIORITIZATION OR TARGETING

- **Selection of PuroKalusugan Sites:**
 - The site or barangay selection process identifies priority areas for initial implementation **based on measurable health and access indicators.**
 - LGUs, with CHD support, **assess each community using set criteria**, such as **poor performance in DOH's 8 Priority Health Outcomes and low access to primary care facilities.**
 - Each barangay is scored, ranked, and at least three with the highest scores are chosen as initial sites.
 - Implementation can later expand to other ranked areas as resources allow, ensuring the most underserved communities are addressed first.
- **Selection of Priority Programs (Except for Nutrition which is mandatory)**
 - The LGUs, with assistance from CHDs/MOH-BARMM, **use local data to compare health outcomes against national targets and assess disease burden, then ranks programs by urgency.** This guides them to pinpoint which health programs to prioritize in each selected barangay or purok
 - The top priorities are planned, coordinated with partners, and implemented first, with progress monitored and adjustments made as capacity and outcomes improve.

**PERFORMANCE INDICATORS AND TARGETS**

List all indicators used to measure progress, along with baseline and target values.

INDICATORS	2025		2026		2027
	TARGETS	ACCOMPL.	TARGETS	ACCOMPL.	
Number of PK Sites providing primary care services	4,824 brgys	8,432 brgys	16,804 bgrys	21,273 brgys (Q2)	29,408 of brgys
Individuals Reached through PK services	15,641,259	6,343,801	12,175,166	3,645,470 (Q2)	TBD*

Note:

Reported targets and accomplishments for **2026** reflect data as of **June 30, 2026**.

PuroKalusugan was officially launched in 2025 through Department Memorandum No. 2025-0024, hence the accomplishment stated is starting 2025 only.

Individual targets for 2025 is not reflective of actual targets due to changes in the indicators. Moreover, data represents cumulative totals for January to December 2025, with all client encounters included regardless of whether they represent first-time or repeat visits. The 2026 target setting for individuals is done at the brgy/pk site level which is submitted to BLHSD quarterly. PK implementation progress is submitted 7 working days after the end of the quarter. The target setting do not apply to the programs on Mental Health, Road safety, and WASH. Mental Health and Road safety are actual counts while WASH is counted at the household level.

Tier 2 Budget Proposal (LHSDA)

BHW Incentive	Orientation for BHWs	Printing of Manuals	Provision of PK Kits
- 1000 php per month for 12 months - Covers 100% of BHWs per region - Number of BHWs to be given incentive: 277,767 Computation: $277,767 \times 1000 \text{ php} \times 12 \text{ months} = 3,333,204,000.00$	- 500 php per day for a 3-day orientation of BHWs - Number of BHWs to be oriented: 166,617 Computation: $166,617 \times 500 \text{ php} \times 3 \text{ days} = 249,924,900$	- 300 php per manual - Manuals to be printed: 166,617 Computation: $166,617 \times 300 \text{ php} = 49,984,980.00$	- 10,668 pesos per kit based on 2025 Market Scoping - PK Kits to be provided: 12,603 Computation: $12,603 \times 10,668 \text{ php} = 134,704,070$

Support for Barangay Health Workers



Number of BHWs



Source: BHW Connect (December 2025)

Region	BHW Incentives	BHW Orientation	Printing of BHW PK Manuals	PK Kits	Total Allocation
NCR	58,716,000.00	4,403,700	880,740	5,498,976	69,499,416.00
CAR	93,228,000.00	6,992,100	1,398,420	3,777,139	105,395,659.20
Region I (Ilocos)	285,780,000.00	21,433,500	4,286,700	10,475,309	321,975,508.80
Region 2 (Cagayan Valley)	143,592,000.00	10,769,400	2,153,880	7,409,990	163,925,270.40
Region 3 (Central Luzon)	286,944,000.00	21,520,800	4,304,160	9,955,872	322,724,832.00
Region IV-A (CALABARZON)	307,344,000.00	23,050,800	4,610,160	12,803,155	347,808,115.20
MIMAROPA	183,912,000.00	13,793,400	2,758,680	4,681,344	205,145,424.00
Region 5 (Bicol)	213,012,000.00	15,975,900	3,195,180	11,129,414	243,312,494.40
Region 6 (Western Visayas)	266,604,000.00	19,995,300	3,999,060	10,866,490	301,464,849.60
NIR	171,864,000.00	12,889,800	2,577,960	4,338,259	191,670,019.20
Region 7 (Central Visayas)	226,692,000.00	17,001,900	3,400,380	7,413,197	254,507,476.80
Region 8 (Eastern Visayas)	248,748,000.00	18,656,100	3,731,220	13,995,936	285,131,256.00
Region 9 (Zamboanga Peninsula)	106,860,000.00	8,014,500	1,602,900	7,419,610	123,897,009.60
Region 10 (Northern Mindanao)	200,352,000.00	15,026,400	3,005,280	6,483,341	224,867,020.80
Region 11 (Davao)	161,208,000.00	12,090,600	2,418,120	3,725,837	179,442,556.80
Region 12 (SOCCSKSARGEN)	115,632,000.00	8,672,400	1,734,480	3,517,421	129,556,300.80
Region 13 (CARAGA)	166,824,000.00	12,511,800	2,502,360	4,206,797	186,044,956.80
BARMM	95,892,000.00	7,126,500	1,425,300	7,005,984	111,449,784.00
Total:	3,333,204,000.00	249,924,900.00	49,984,980.00	134,704,070.40	3,767,817,950.40

Support for Barangay Health Workers

PROGRAM PROFILE

PROGRAM/ACTIVITY/PROJECT DESCRIPTION

1. **Policy Development** – Formulation and updating of policies and guidelines on Performance-Based Incentives (PBI) for BHWs, also support for the development of policies related to 1. benefits and incentives for BHWs; and 2. their roles and responsibilities in line with RA 7883.
2. **Capacity Building** – support the trainings, workshops, and development of manuals to strengthen BHW competencies. This includes ensuring that the developed manuals explicitly integrate the roles and functions of BHWs and that these roles are reflected across DOH programs and projects, in support of their mandated responsibilities.
3. **BHW Registry Maintenance** – Regular updating, validation, and utilization of the BHW Registry (via BHW Connect) to ensure accurate profiling, effective planning, and performance monitoring, in support of the efficient delivery of BHW functions.

OBJECTIVES AND INTENDED OUTCOMES

A. Objective

To establish clear guidelines and coordinate support programs and projects for Barangay Health Workers (BHWs), ensuring their continuous development and empowerment.

B. Intended Outcomes

1. **Strengthened Capacity** – facilitate/coordinate the enhancement of skills and competencies of BHWs in delivering primary health care services at the community level.
2. **Recognition and Empowerment** – Promote sustained recognition of BHWs as vital partners in achieving UHC goals.
3. **Maintenance of BHW Registry** – Ensure accurate, updated, and comprehensive BHW profiling through BHW Connect to support planning, monitoring, and performance-based incentives.

Support for Barangay Health Workers

DOH Support to BHWs Based on CHD/MOH-BARMM Submissions

CHD/MOH BARMM	2025 (Actual cost and /Based on 2025 WFP)	Proposed for 2026	Remarks
DOH Support to BHWs	60,310,691	46,058,421	
MM CHDs	1,850,000	2,050,000	includes capacity building, provision of logistics supplies, PBI for BHWs, and other support for BHWs
Ilocos Region	3,290,000	2,856,000	
CAR	7,644,533	4,251,621	
Cagayan Valley	1,850,000	2,856,000	
Central Luzon	1,850,000	2,050,000	
Calabarzon	3,707,973	2,050,000	
Mimaropa	9,936,028	2,050,000	
Bicol	1,300,000	2,050,000	
Western Visayas	5,523,750	5,095,000	
Central Visayas	5,977,900	2,050,000	
NIR	1,850,000	2,050,000	
Eastern Visayas	1,847,000	1,849,800	
Zamboanga Peninsula	4,865,000	6,600,000	
Northern Mindanao	1,850,000	2,050,000	
Davao Region	1,880,000	2,050,000	
SOCCSKSARGEN	3,528,707	2,050,000	
CARAGA	1,559,800	2,050,000	
MOH BARMM	442,458,700	2,050,000	
Total			

Support for Barangay Health Workers

PROGRAM PROFILE



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

Key Implementation Strategies

Performance-Based Incentives (PBI) for BHWs

- Implementation of standardized criteria anchored on RA 7883 roles and responsibilities of BHWs.
- Establishment of fair, transparent, and accountable screening, validation, and ranking mechanisms at the CHD and MOH-BARMM level.
- Provision of cash prize grants to Provincial, Highly Urbanized City (HUC), and Independent Component City (ICC) BHW Federations based on their performance, with projects focused on inclusive support to members
- Cash prize of PhP 1,000,000 shall be awarded to the top-performing P/CBHW Federation based on the established ranking criteria by the CHDs/MOH BARMM
- Accredited P/CBHW Federations shall receive the cash prize through a MOA. However, in cases where the top-performing P/CBHW Federation is unable to comply with existing accounting rules and regulations, the fund utilization may be managed by the concerned CHDs or MOH-BARMM.

Support for Barangay Health Workers

PROGRAM
PROFILE



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

Key Implementation Strategies

Capacity Building

- Development of BHW reference Manual and Facilitators guide recommended to LGUs as reference training guide for BHWs, including training, workshops, and mentoring in the context of RAs 7883 and UHC.
- Development of training materials that integrate the roles and responsibilities of BHWs in line to RA 7883.
- Ensuring that related DOH programs and projects explicitly recognize and integrate the roles of BHWs.
- Support for professional growth such as advocacy activities for registration to TESDA BHS NC II and recognition initiatives to sustain motivation and commitment of BHWs.

Support for Barangay Health Workers

PROGRAM PROFILE

► KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

Key Implementation Strategies

BHW Connect (Registry Maintenance and Utilization)

- Continuous updating and enhancement of BHW Connect to maintain an accurate and reliable national BHW Registry.
- Regular encoding, validation, and updating of BHW profiles, including training, status, years of service, honorarium received, and generation of BHW IDs.
- Utilization of registry data for planning, resource allocation, program targeting, monitoring, and recognition of BHW contributions.

Link to NHW Connect: BHW.doh.gov.ph or

Note: The BHW connect is open for encoding and updating of BHW profiles

Support for Barangay Health Workers

► BASIS OF PRIORITIZATION OR TARGETING

- **BHW Connect** - Only registered and accredited Barangay Health Workers (BHWs), duly encoded in the BHW Registry System as mandated by RA 7883.

Registered	Accredited
● Training ● 1 year of voluntary service ● 18 years old; and ● Physically and Mentally fit	● Registered ● 3 years of voluntary service

- **PBI for BHWs** -Annual provision of PBI for BHWs shall be given to BHW Federations that consistently demonstrate the mandated functions of BHWs as community organizers, health educators, and primary health care providers.
- **Capacity Building** - The DOH conducted TOT sessions for the CHDs and MOH-BARMM representatives using the BHW Reference Manual. In return, the CHDs and MOH-BARMM shall ensure the conduct of training for their respective LGUs who will then be responsible for cascading the training to their respective BHWs. To support this process, the DOH shall provide policy manuals, training guides, and standardized tools to ensure uniformity and alignment with RA 7883

Support for Barangay Health Workers

STATUS OF IMPLEMENTATION OF DOH DILG JMC. 2023-001 Retention and Continued Service of BHWs as of **Sept. 18, 2025**

Feedback from CHD/MOH-BARMM	Next Steps
The CHDs/MOH-BARMM disseminated the JMC to the LGUs. There were reports of BHWs being terminated and reinstated back. However, majority of cases were not documented or reported to the CHDs. Therefore, exact number of BHWs who were removed and reinstated back could not be determined at the moment.	1. Status of BHWs will be included in the BHW Connect to get an actual count of those terminated and reinstated back 2. Continuous advocacy through the CHDs/MOH-BARMM ● Feedback from regions in the succeeding slides

Support for Barangay Health Workers

STATUS OF IMPLEMENTATION OF DOH DILG JMC. 2023-001 Retention and Continued Service of BHWs as of Sept. 18, 2025

Region/CHD	No of Reinstated BHWs	Remarks
REGION I (ILOCOS REGION)	0	For 2 provinces, no BHWs were reinstated as per JMC 2023-001. The Circular was not taken into consideration for some barangays in another province.
REGION II (CAGAYAN VALLEY)	0	An investigation has been conducted by the DILG in 1 municipality; however, no formal report was submitted to the CVCHD
REGION III (CENTRAL LUZON)	4	From Bocaue, Bulacan, 4 BHWs who were previously removed by the barangay captain were later reinstated in accordance with the provisions of the Joint Memorandum Circular (JMC)
REGION IV-A (CALABARZON)		Some of our BHW Federation Presidents have raised concerns regarding certain BHWs who were terminated, although there are no documented reports. In some cases, BHWs were dismissed from their assigned barangays but through the use of the JMC they were able to retain their status as BHWs by being reassigned to other barangays. There are also instances where BHWs were terminated despite the
		presence of the JMC.

Support for Barangay Health Workers

STATUS OF IMPLEMENTATION OF DOH DILG JMC. 2023-001 Retention and Continued Service of BHWs as of Sept. 18, 2025

Region/CHD	No of Reinstated BHWs	Remarks
REGION IV-B (MIMAROPA)	0	It is because of the JMC and meeting of BHW Partylist to the Liga ng mga Barangay that they are following the DOH-DILG JMC; the Barangay Captain follow the instructions of DILG and DOH so the BHWs reinstated or not removed.
REGION V (BICOL REGION)	0	The DILG conducted seminars for Barangay Newly Elected Officials (BNEO). The BHW Partylist and DOH sits-in the seminars and they explain the JMC; some brgy captain are following.
REGION VI (WESTERN VISAYAS)		We have already shared the JMC with our counterparts, yet there is one (1) BHW who was terminated and was not given certificate of appointment.
REGION VII (CENTRAL VISAYAS)	0	It is noted that no formal or documented reports were submitted to the CHD. However, there were informal and undocumented instances alleged at the field level.
REGION VIII (EASTERN VISAYAS)		We shared the JMC with our counterparts at the Provincial DOH Offices (PDOHOs), Provincial Health Offices (PHOs) and with the LGUs. Some LGUs have already started implementing it, but others are still facing some issues, especially when it comes to removing BHWs without following due process.

Support for Barangay Health Workers

STATUS OF IMPLEMENTATION OF DOH DILG JMC. 2023-001 Retention and Continued Service of BHWs as of Sept. 18, 2025

Region/CHD	No of Reinstated BHWs	Remarks
REGION IX (ZAMBOANGA PENINSULA)	9394	There are reported cases at the Barangay level, however, these cases have been formally documented or submitted to the CHD for official reporting.
REGION X (NORTHERN MINDANAO)	TBD	There have been reported cases at the LGU level, some of which were brought before the Local Health Board for validation. In fact, these issues were resolved, and the affected BHWs were given their rightful opportunity to retain their services. While we do not have the exact number of reinstated BHWs, the dissemination of the JMC has been a significant help in changing the perspective of the Local Chief Executives.
REGION XI (DAVAO REGION)	TBD	Starting 4th Quarter of 2023, Davao CHD (DCHD) had initiated various dissemination activities of the JMC through Consultative Meetings attended by all BHW Provincial & City Coordinators, BHW Provincial, City & Municipal BHW Federated Presidents, DILG XI representative (though it is the DOH-DCHD REgional BHW P.M. who discussed the JMC. JMC was also circulated through the official regional GC. In the 1st Qtr. of 2024, DOH-DCHD organized a two-day Consultative Workshop funded by the World Health Organization (WHO) and one of the 3 most important agenda was the JMC. This was attended by PHOs, CHOs and ABC Presidents of all LGU's and representatives from the Provincial Governor's Office. There were also series of Fora conducted by the province of Davao del Norte (through the PHO) and some municipalities to ensure massive information dissemination of the JMC. Principal objective was to remind LCEs (specially Brgy. Captains) of the consequences of undue suspension and/or removal of BHWs. Furthermore, even before the approval of the JMC, there were already numerous complaints of illegal removal of BHWs, however no official reports were forwarded to the DCHD.

Support for Barangay Health Workers

STATUS OF IMPLEMENTATION OF DOH DILG JMC. 2023-001 Retention and Continued Service of BHWs as of Sept. 18, 2025

Region/CHD	No of Reinstated BHWs	Remarks
REGION XII (SOCCSKSARGEN)	0	There are reported cases at the LGU level, however, these cases have not been formally documented or submitted to the CHD for official reporting
NATIONAL CAPITAL REGION		The JAO itself was used in NCR to warn barangay officials against removing BHWs without due process, in accordance with RA 7883. Barangay Captains were deterred by the sanction clause; thus, no BHWs were removed without due process.
CORDILLERA ADMINISTRATIVE REGION	0	The JMC was cascaded to LGUs and BHW Federations in 2023. However, the policy was still not considered by some barangay officials after the election. This led to the removal and replacement of some BHWs in the region. However, some BHWs were reinstated not through the use of the JMC but through the consultative meetings conducted by the RHUs.

Support for Barangay Health Workers

STATUS OF IMPLEMENTATION OF DOH DILG JMC. 2023-001 Retention and Continued Service of BHWs as of Sept. 18, 2025

Region/CHD	No of Reinstated BHWs	Remarks
BANGSAMORO AUTONOMOUS REGION IN MUSLIM MINDANAO	8600	Some LGU removed BHW and replaced with new but we emphasized JMC 2023-001 and advised them to report since the fund (incentives) coming from bangsamoro government through Ministry of Health
REGION XIII (CARAGA)	0	The DOH–DILG Joint Memorandum Circular (JMC) 2023-01 on the Retention and Continued Service of BHWs has been disseminated to PDOHOs, PHOs, and LGUs for proper implementation. Reports of cases at the barangay level involving the relief or tenure issues of BHWs were noted; however, these have not been formally documented nor submitted to the CHD for official reporting.

LGU Health Scorecard

PROGRAM PROFILE



PROGRAM/ACTIVITY/PROJECT DESCRIPTION

Primary tool to assess and monitor the performance of LGUs in the implementation of local health reforms within the P/CWHS. It facilitates the reporting of LGUs' progress in meeting the targets based on the priority programs, projects and activities of the Department of Health (DOH)

Strategies:

- Collaboration with KMITS, EB and other DOH M&E systems to streamline and harmonize the processes of data collection and management
- Utilization of results as reference for health planning, performance-based financing, health resource allocations, technical assistance provision, awarding and recognition.



OBJECTIVES AND INTENDED OUTCOMES

- Track the performance of the LGUs in the implementation of local health reforms within the province-wide/city-wide health system
- Guide the implementation of health reforms and priority public health programs at the LGU level

LGU Health Scorecard (LGU HSC)

PROGRAM
PROFILE

► LEGAL AND POLICY BASIS

Mandate Creating Program/Year:

- A.O. No. 2008-0017: Implementing Guidelines for the LGU Scorecard; D.M. No. 2014-0147: Amendments in the LGU SC Indicators 2012-2016
- A.O. No. 2019-0027: Revised Implementation Guidelines for the LGU Health Scorecard
- A.O. No. 2021-0002: Revised Guidelines on the Implementation of the Local Government Unit (LGU) Health Scorecard (HSC)

Goal:

- To measure and track the general performance of LGUs and report their progress in meeting the national health targets based on the priority programs, projects and activities of the DOH

LGU Health Scorecard (LGU HSC)

The LGU HSC makes use of internal and external benchmarking systems to assess and inform LGUs of their performance. The external benchmarking system uses a traffic light color-code scoring to easily communicate to the different stakeholders the performance of an LGU with respect to the average performance of all LGUs and the national target (Table 1). The internal benchmarking system (Table 2) uses directional arrows (and an equal sign) to show how the current year's performance of an LGU fare with respect to its previous year's performance.

Table 1. External Benchmarking System (Color-Code Scoring)

Color Code Score	Interpretation
	"Excellent" - equal to or better than the national target
	"Fair" - lower than the national target but equal to or better than the national average performance
	"Poor" - lower than the national average performance
	- for baseline data collection; no target
	- not applicable
	- no data

Table 2. Internal Benchmarking System (Directional Arrows)

Directional Arrow	Interpretation
Upward-pointing (↑)	- current year's performance result IMPROVED compared to previous year's performance result
Equal sign (=)	- current year's performance result is EQUAL to the previous year's performance result, no change
Downward-pointing (↓)	- current year's performance result DECLINED compared to the previous year's performance result

LGU Health Scorecard (LGU HSC)

PROGRAM
PROFILE

►KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

DM 2020-0275: LGU HSC MANUAL OF PROCEDURES

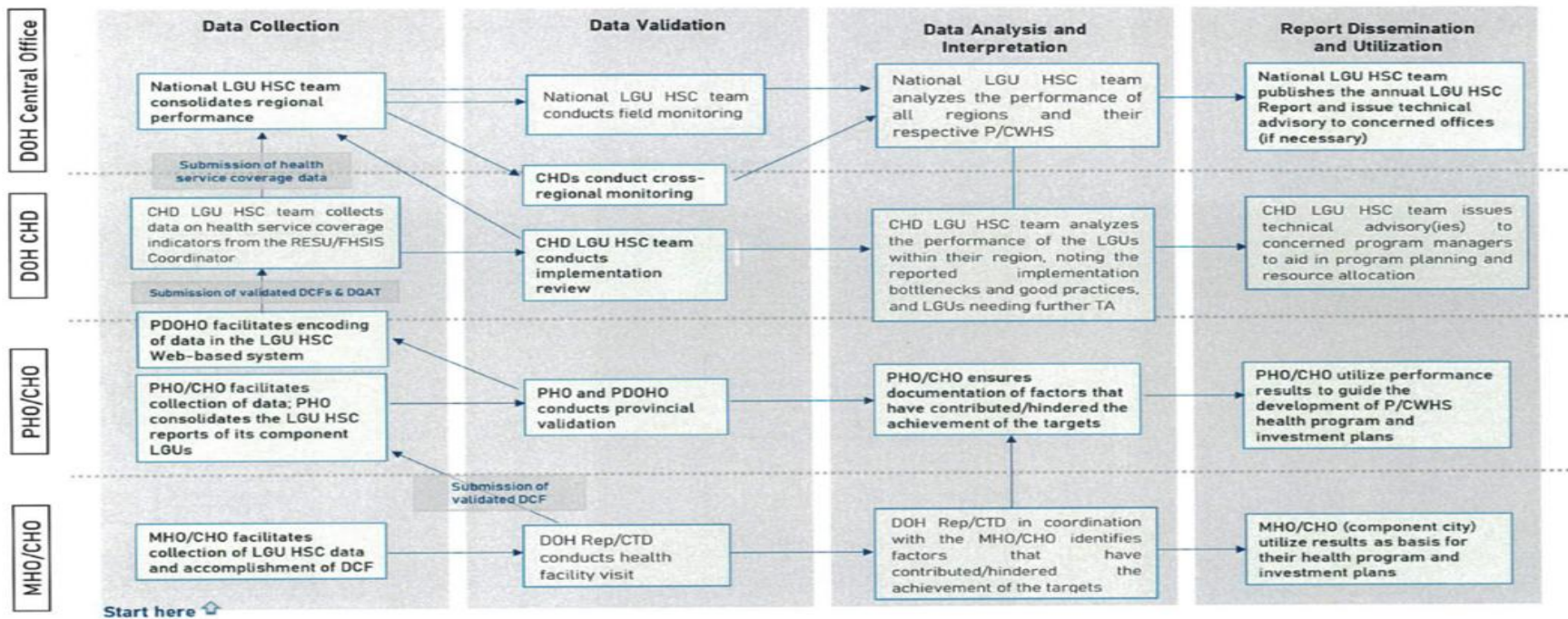
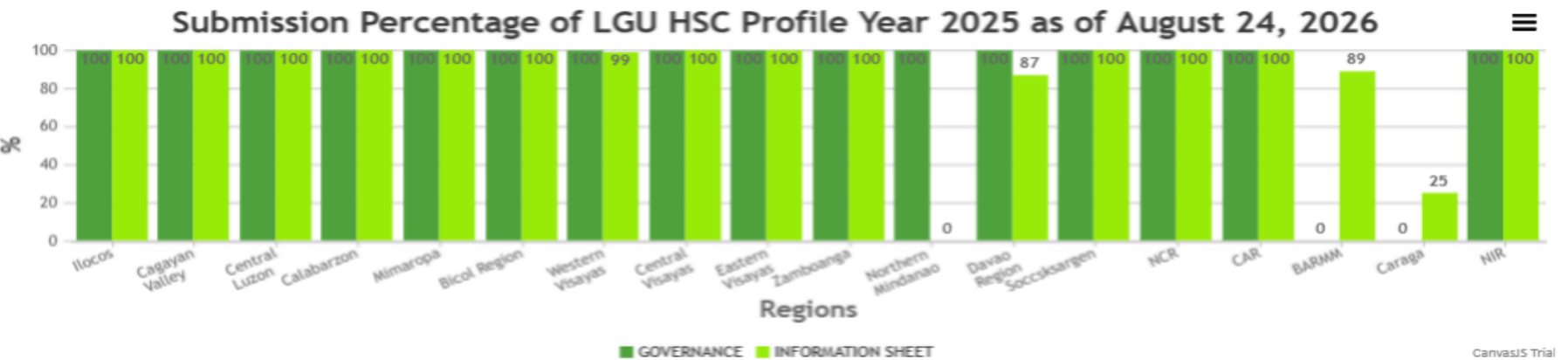


Figure 2. LGU Health Scorecard Flow of Activities

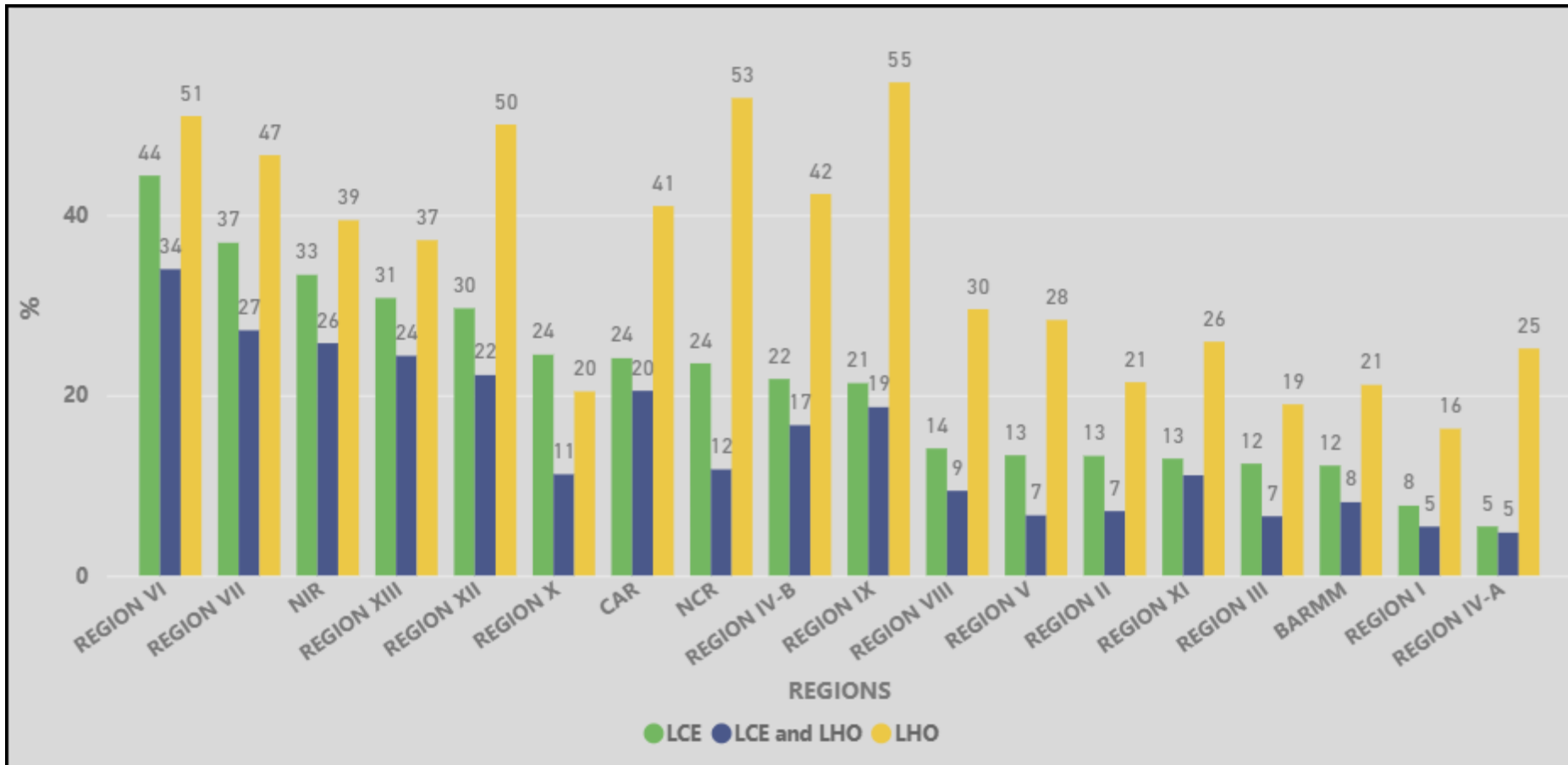
LGU Health Scorecard (LGU HSC)

KEY PERFORMANCE INDICATORS	ACCOMPLISHMENT PER YEAR					
	2022		2023		2024	
	#	% of Target	#	% of Target	#	% of Target
Percentage of LGUs Submitting Data	1714	99.9%	1715	100%	1702	99.2%



*For 2025 LGU HSC, ongoing data collection/validation

LCE and LHO Health Governance Training per Region, 2024



Percentage of LGU budget allocated for Health, Nutrition, and Population Control

It measures the proportion of the budget that is allocated specifically to Health, Nutrition, and Population Control (HNPC) expenditures.

Where:
HNPC covers sector for health program including medical, dental and health services; planning and administration of nutrition programs; population and family planning programs; and administration of these programs.

Region	LGU Budget Allocated For Health, 2024		
	Province	HUC	City
National Average	18.78	7.17	15.27
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	14.46	5.69	13.96
CAGAYAN VALLEY (II)	12.80	9.36	11.79
CENTRAL LUZON (III)	17.84	8.52	16.92
CAR	20.49	7.92	18.30
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	18.78	7.61	18.23
BICOL (V)	22.56	8.00	21.41
NCR	NA	13.17	14.61
MIMAROPA (IV-B)	19.66	7.27	16.64
VISAYAS			
WESTERN VISAYAS (VI)	21.84	7.55	17.36
CENTRAL VISAYAS (VII)	29.85	6.93	17.60
EASTERN VISAYAS (VIII)	31.56	8.55	25.72
MINDANAO			
ZAMBOANGA PENINSULA (IX)	13.33	6.33	11.80
NORTHERN MINDANAO (X)	12.76	6.02	8.66
DAVAO (XI)	10.34	5.54	7.60
SOCCKSARGEN (XII)	20.98	6.89	19.44
BARMM	3.19	2.28	3.10
CARAGA	17.10	6.04	15.46

LGU HSC, 2024

Region		Provincial LGU Budget For Health (%)		
		2023 Results	2024 Results	
National Average		18.99	18.78	↓
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)		14.18	14.46	↑
CAGAYAN VALLEY (II)		15.05	12.80	↑
CENTRAL LUZON (III)		14.74	17.84	↑
CAR		20.10	20.49	↓
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)		19.44	18.78	↓
BICOL (V)		20.97	22.56	↑
NCR		NA	NA	NA
MIMAROPA (IV-B)		25.34	19.66	↓
VISAYAS				
WESTERN VISAYAS (VI)		22.81	21.84	↓
CENTRAL VISAYAS (VII)		30.43	29.85	↓
EASTERN VISAYAS (VIII)		29.93	31.56	↑
MINDANAO				
ZAMBOANGA PENINSULA (IX)		15.84	13.33	↓
NORTHERN MINDANAO (X)		14.78	12.76	↓
DAVAO (XI)		10.22	10.34	↑
SOCCSKSARGEN (XII)		21.53	20.98	↓
BARMM		2.84	3.19	↑
CARAGA		17.97	17.10	↓
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result		
	↓	2024 Performance result is DECLINED compared to the 2023 performance results		
	▬	2024 Performance result is EQUAL compared to the 2023 performance results, no change		
	—	No Comparative Data From 2023		
	N/A	Not Applicable		

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

Region		Highly Urbanized City / Independent Component City Budget Allocated For Health (%)		
		2023 Results	2024 Results	
National Average		12.62	11.90	↓
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)		6.82	7.41	↑
CAGAYAN VALLEY (II)		6.35	5.28	↓
CENTRAL LUZON (III)		15.00	11.15	↓
CAR		9.79	10.38	↑
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)		3.65	5.37	↑
BICOL (V)		7.14	7.78	↑
NCR		16.91	14.61	↓
MIMAROPA (IV-B)		15.15	8.75	↓
VISAYAS				
WESTERN VISAYAS (VI)		4.37	6.98	↑
CENTRAL VISAYAS (VII)		4.35	7.21	↑
EASTERN VISAYAS (VIII)		6.67	6.34	↓
MINDANAO				
ZAMBOANGA PENINSULA (IX)		11.41	9.38	↓
NORTHERN MINDANAO (X)		3.98	3.76	↓
DAVAO (XI)		7.71	5.31	↓
SOCCKSARGEN (XII)		12.53	14.59	↑
BARMIM		1.56	2.49	↑
CARAGA		16.47	10.68	↓
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result		
	↓	2024 Performance result is DECLINED compared to the 2023 performance results		
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change		
	-	No Comparative Data From 2023		
	N/A	Not Applicable		

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC,
2024

Region		Municipal / Component City Budget Allocated For Health (%)		
		2023 Results	2024 Results	
National Average		7.09	7.17	↑
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)		5.86	5.69	↓
CAGAYAN VALLEY (II)		8.24	9.36	↑
CENTRAL LUZON (III)		8.66	8.52	↓
CAR		8.06	7.92	↓
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)		7.13	7.61	↑
BICOL (V)		8.54	8.00	↓
NCR		13.19	13.17	↓
MIMAROPA (IV-B)		7.02	7.27	↑
VISAYAS				
WESTERN VISAYAS (VI)		8.02	7.55	↓
CENTRAL VISAYAS (VII)		6.07	6.93	↑
EASTERN VISAYAS (VIII)		8.21	8.55	↑
MINDANAO				
ZAMBOANGA PENINSULA (IX)		6.31	6.33	↑
NORTHERN MINDANAO (X)		6.04	6.02	↓
DAVAO (XI)		5.85	5.54	↓
SOCCSKSARGEN (XII)		6.75	6.89	↑
BARMM		2.92	2.28	↓
CARAGA		5.80	6.04	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result		
	↓	2024 Performance result is DECLINED compared to the 2023 performance results		
	▬	2024 Performance result is EQUAL compared to the 2023 performance results, no change		
	—	No Comparative Data From 2023		
	N/A	Not Applicable		

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

With complete Local Investment Plan for Health (LIPH) / Annual Operational Plan (AOP)

The LIPH is the LGU’s strategic plan to implement Universal Health Care, with the Annual Operation Plans (AOP) translating its details on a yearly basis. Provinces, HUCs, ICCs, Municipalities and Component Cities craft their respective LIPHs and AOPs following the prescribed processes and timelines.

Region	Percentage of LGUs with complete LIPH / Health Plans		
	Province	HUC, ICC	City
National Average	93	100	99
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	100	100	99
CAGAYAN VALLEY (II)	100	100	99
CENTRAL LUZON (III)	100	100	100
CAR	100	100	97
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	100	100	97
BICOL (V)	50	100	88
NCR	0	100	100
MIMAROPA (IV-B)	100	100	96
VISAYAS			
WESTERN VISAYAS (VI)	100	100	98
CENTRAL VISAYAS (VII)	100	100	92
EASTERN VISAYAS (VIII)	83	100	98
MINDANAO			
ZAMBOANGA PENINSULA (IX)	100	100	99
NORTHERN MINDANAO (X)	100	100	99
DAVAO (XI)	100	100	98
SOCCSKSARGEN (XII)	100	100	100
BARMM	71	100	86
CARAGA	100	100	88

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

Functional Local Health Board

Refers to the extent wherein the Local Health Board (LHB) is able to meet the minimum required members (compliant with RA 7160 and 11223) including civil society organization (CSO) representative (compliant with DILG M.C. 2022-083). The LHB shall be also able to perform its mandated functions as follows:

1. Deliberate and recommend to the Sanggunian the annual budget on health responsive to the needs of the LGU
2. Recommend the development/ updating of local health policy/ies with the aim to strengthen health systems management and service delivery as aligned with UHC
3. Create committees and engage/ involve community/ private sector representatives in selected LHB activities

*For 2025 LGU HSC, ongoing data collection/validation

Region	Functional Local Health Board		
	2023 Results	2024 Results	
National Average	51	73	↑
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	61	80	↑
CAGAYAN VALLEY (II)	65	92	↑
CENTRAL LUZON (III)	72	89	↑
CAR	39	64	↑
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	31	66	↑
BICOL (V)	34	52	↑
NCR	82	94	↑
MIMAROPA (IV-B)	33	72	↑
VISAYAS			
WESTERN VISAYAS (VI)	52	71	↑
CENTRAL VISAYAS (VII)	45	67	↑
EASTERN VISAYAS (VIII)	54	82	↑
MINDANAO			
ZAMBOANGA PENINSULA (IX)	77	89	↑
NORTHERN MINDANAO (X)	44	61	↑
DAVAO (XI)	61	81	↑
SOCCSKSARGEN (XII)	72	91	↑
BARMM	40	46	↑
CARAGA	49	78	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

LGU HSC, 2024

Functional Local AIDS Council (LAC)

Refers to the degree to which LGUs have active local AIDS councils (LACs) which are able to meet the level of functionality as stipulated in Section 19 of IRR of RA 11166 either in the form of advocacy or HIV referral activities. LGU must meet the 3 components:

1. multi-sectoral composition of LAC members,
2. with allocated HIV related budgets either from GAD or other sources, and
3. annual plans of HIV-related programs/activities and projects

A Functional Local AIDS Council signifies that the LGU is actively engaged in addressing the HIV/AIDS epidemic within its jurisdiction. HIV/ AIDS remains a significant public health concern, and local governments play a crucial role in prevention, treatment, and support services, particularly in HIV education in communities pursuant to RA 11166. By having a functional council in place, LGUs can coordinate efforts to raise awareness, provide access to testing and treatment, and support those affected by HIV/AIDS.

*For 2025 LGU HSC, ongoing data collection/validation

Region	Functional Local AIDS Council		
	2023 Results	2024 Results	
National Average	52	71	↑
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	76	86	↑
CAGAYAN VALLEY (II)	64	89	↑
CENTRAL LUZON (III)	60	93	↑
CAR	57	72	↑
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	45	43	↓
BICOL (V)	50	70	↑
NCR	76	100	↑
MIMAROPA (IV-B)	35	68	↑
VISAYAS			
WESTERN VISAYAS (VI)	64	74	↑
CENTRAL VISAYAS (VII)	56	72	↑
EASTERN VISAYAS (VIII)	48	78	↑
MINDANAO			
ZAMBOANGA PENINSULA (IX)	60	81	↑
NORTHERN MINDANAO (X)	50	57	↑
DAVAO (XI)	52	80	↑
SOCCKSARGEN (XII)	45	87	↑
BARMM	11	31	↑
CARAGA	50	82	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

LGU HSC, 2024

Region	Percentage of LGUs with Active Local AIDS Council		
	1. Multi-sectoral composition of LAC members	2. With allocated HIV related budgets either from GAD or other sources, and	3. Annual plans of HIV-related programs/ activities and projects
National Average	91	97	92
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	93	96	91
CAGAYAN VALLEY (II)	91	99	93
CENTRAL LUZON (III)	95	98	96
CAR	80	90	77
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	48	95	82
BICOL (V)	88	84	78
NCR	100	100	100
MIMAROPA (IV-B)	72	92	79
VISAYAS			
WESTERN VISAYAS (VI)	80	96	85
CENTRAL VISAYAS (VII)	80	95	85
EASTERN VISAYAS (VIII)	86	94	85
MINDANAO			
ZAMBOANGA PENINSULA (IX)	84	91	91
NORTHERN MINDANAO (X)	79	82	63
DAVAO (XI)	85	96	81
SOCCKSARGEN (XII)	89	94	87
BARMM	37	46	33
CARAGA	85	92	83

*For 2025 LGU HSC,
ongoing data
collection/validation

LGU HSC, 2024

Provision of **FULL** hazard pay, subsistence and laundry allowances to permanent public health workers (Physician, Public Health Nurse & Midwife) in accordance with RA 7305 (Magna Carta for Public Health Workers)

The salary of the Physician, Public Health Nurse & Midwife complied with the Salary Standardization Law, and benefits are fully given to ALL the permanent LGU-hired health workers:

1. Hazard Allowance
2. Laundry Allowance
3. Subsistence Allowance

(Republic Act No. 7305 “The Magna Carta of Public Health Workers”)

*For 2025 LGU HSC, ongoing data collection/validation

Region		LGUs that Provided Full Magna Carta Benefits to their PHWs	
		2023 Results	2024 Results
National Average		70	77 ↑
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)		64	65 ↑
CAGAYAN VALLEY (II)		40	43 ↑
CENTRAL LUZON (III)		64	66 ↑
CAR		39	41 ↑
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)		71	74 ↑
BICOL (V)		83	85 ↑
NCR		71	71 =
MIMAROPA (IV-B)		94	95 ↑
VISAYAS			
WESTERN VISAYAS (VI)		75	82 ↑
CENTRAL VISAYAS (VII)		91	93 ↑
EASTERN VISAYAS (VIII)		67	74 ↑
MINDANAO			
ZAMBOANGA PENINSULA (IX)		92	95 ↑
NORTHERN MINDANAO (X)		68	70 ↑
DAVAO (XI)		100	98 ↓
SOCCSKSARGEN (XII)		94	96 ↑
BARMM		28	84 ↑
CARAGA		82	91 ↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

LGU HSC, 2024

Region	Percentage of LGUs Provides three components to its Health Workers, 2024		
	Hazard	Subsistence	Laundry Allowance
National Average	61	94	95
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	65	98	99
CAGAYAN VALLEY (II)	46	88	88
CENTRAL LUZON (III)	69	98	95
CAR	42	89	83
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	76	94	93
BICOL (V)	88	97	98
NCR	71	100	100
MIMAROPA (IV-B)	97	97	96
VISAYAS			
WESTERN VISAYAS (VI)	83	95	95
CENTRAL VISAYAS (VII)	94	99	99
EASTERN VISAYAS (VIII)	74	95	95
MINDANAO			
ZAMBOANGA PENINSULA (IX)	95	100	99
NORTHERN MINDANAO (X)	72	91	91
DAVAO (XI)	98	98	98
SOCCSKSARGEN (XII)	96	98	98
BARMM	64	63	63
CARAGA	92	95	96

*For 2025 LGU HSC,
ongoing data
collection/validation

LGU HSC, 2024

Number of Accredited Konsulta Provider

NHIP Konsulta Package – Konsultasyong Sulit at Tama (Konsulta) Package Konsulta Providers - refers to primary care facilities and/ or health care professionals accredited by PhilHealth to provide the Konsulta package

RA 11223

Section 6b PhilHealth shall implement a comprehensive outpatient benefit

Section 6c The DOH and the local government units (LGUs) shall endeavor to provide a health care delivery system that will afford every Filipino a primary care provider that would act as the navigator, coordinator, and initial and continuing point of contact in the health care delivery system

*For 2025 LGU HSC, ongoing data collection/validation

Region	Number of Accredited Konsulta Provider		
	2023 Results	2024 Results	
National Average	2610	3300	↑
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	132	178	↑
CAGAYAN VALLEY (II)	102	115	↑
CENTRAL LUZON (III)	308	400	↑
CAR	97	108	↑
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	269	318	↑
BICOL (V)	140	165	↑
NCR	314	469	↑
MIMAROPA (IV-B)	90	152	↑
VISAYAS			
WESTERN VISAYAS (VI)	166	192	↑
CENTRAL VISAYAS (VII)	207	219	↑
EASTERN VISAYAS (VIII)	139	176	↑
MINDANAO			
ZAMBOANGA PENINSULA (IX)	107	124	↑
NORTHERN MINDANAO (X)	116	160	↑
DAVAO (XI)	122	140	↑
SOCCKSARGEN (XII)	113	153	↑
BARMM	112	123	↑
CARAGA	76	108	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

LGU HSC, 2024

Percentage of Fully Immunized Child

Refers to the number of infants and children in the population that met the definition of FIC, expressed in percentage

FIC is defined as a child who completed their 1 dose of BCG, 3 doses of pentavalent vaccine (DPT-HepB-Hib), 3 doses of OPV, and 2 doses of MMR on or before 1 year of age. 2nd dose of MCV to be given at 12 months old

Region		Fully Immunized Child (%)	
		2023 Results	2024 Results
National Average		62.47	64.85 ↑
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)		71.28	75.10 ↑
CAGAYAN VALLEY (II)		69.00	70.43 ↑
CENTRAL LUZON (III)		70.98	76.04 ↑
CAR		65.03	66.28 ↑
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)		52.46	60.64 ↑
BICOL (V)		51.35	47.84 ↓
NCR		78.57	78.80 ↑
MIMAROPA (IV-B)		58.40	58.87 ↑
VISAYAS			
WESTERN VISAYAS (VI)		61.36	65.82 ↑
CENTRAL VISAYAS (VII)		56.31	61.84 ↑
EASTERN VISAYAS (VIII)		57.97	47.86 ↓
MINDANAO			
ZAMBOANGA PENINSULA (IX)		54.43	63.46 ↑
NORTHERN MINDANAO (X)		70.40	63.47 ↓
DAVAO (XI)		63.13	60.80 ↓
SOCCSKSARGEN (XII)		61.96	64.31 ↑
BARMM		45.38	51.68 ↑
CARAGA		79.34	79.92 ↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

Percentage of pregnant women with at least 4 prenatal check –ups

Refers to the number of pregnant women w/ at least 4 prenatal check-ups

(availed as follows among the total estimated number of pregnant women in the population):

- 1st tri = up to 12 weeks and 6 days AOG (at least 1 check-up)
- 2nd tri = 13-27 weeks and 6 days AOG (at least 1 check-up)
- 3rd tri = 28 weeks AOG and more (at least 2 check-ups)

*For 2025 LGU HSC, ongoing data collection/validation

Region	Percentage of pregnant women with at least 4 prenatal check-ups		
	2023 Results	2024 Results	
National Average	76.68	117.66	↑
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	98.95	88.00	↓
CAGAYAN VALLEY (II)	88.00	103.57	↑
CENTRAL LUZON (III)	99.26	90.72	↓
CAR	66.17	140.86	↑
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	71.71	123.68	↑
BICOL (V)	75.38	137.16	↑
NCR	87.14	109.01	↑
MIMAROPA (IV-B)	46.02	159.89	↑
VISAYAS			
WESTERN VISAYAS (VI)	75.66	119.13	↑
CENTRAL VISAYAS (VII)	28.57	161.26	↑
EASTERN VISAYAS (VIII)	68.79	140.12	↑
MINDANAO			
ZAMBOANGA PENINSULA (IX)	81.59	126.39	↑
NORTHERN MINDANAO (X)	84.11	105.27	↑
DAVAO (XI)	78.84	121.27	↑
SOCCSKSARGEN (XII)	87.72	115.82	↑
BARMM	79.79	123.66	↑
CARAGA	80.68	118.21	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	–	No Comparative Data From 2023	
	N/A	Not Applicable	

LGU HSC, 2024

Percentage of adults 20 years old and above who were risk assessed using the PhilPEN protocol

Refers to the number of adults, 20 years old and above who were risk assessed using the Philippine Package of Essential NCD Interventions (PhilPEN) protocol among the total number of adults 20 years old and above in the total population expressed in percentage.

Region		Adult Risk-Assessed Using PhilPEN Protocol (%)	
		2023 Results	2024 Results
National Average		20.50	24.89
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)		25.37	31.24
CAGAYAN VALLEY (II)		37.12	43.56
CENTRAL LUZON (III)		34.64	49.44
CAR		26.92	35.63
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)		6.76	0.00
BICOL (V)		69.78	34.06
NCR		11.99	16.66
MIMAROPA (IV-B)		10.08	18.89
VISAYAS			
WESTERN VISAYAS (VI)		22.08	32.69
CENTRAL VISAYAS (VII)		11.95	18.84
EASTERN VISAYAS (VIII)		16.53	27.86
MINDANAO			
ZAMBOANGA PENINSULA (IX)		13.76	25.98
NORTHERN MINDANAO (X)		14.61	23.22
DAVAO (XI)		25.00	32.02
SOCCSKSARGEN (XII)		15.33	27.35
BARMM		10.11	21.37
CARAGA		16.73	29.00
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

TB Case Notification Rate

This indicator highlights local government efforts to implement programs for the prevention and control of communicable diseases.

TB Case Notification Rate refers to the number of notified TB, New and Relapse for every 100,000 population.

Notified TB, all forms: include new and relapse (whether bacteriologically confirmed or clinically diagnosed) who were detected, registered, and reported to the MNTPs

Region		TB Case Notification Rate (per 100,000 population)	
		2023 Results	2024 Results
National Average		517	487 ↓(-6)
NORTHERN AND CENTRAL LUZON			
CAR		254	317 ↑(25)
ILOCOS (I)		398	419 ↓(5)
CENTRAL LUZON (III)		538	493 ↓(-8)
CAGAYAN VALLEY (II)		464	448 ↓(-3)
NCR AND SOUTHERN LUZON			
BICOL (V)		534	494 ↓(-7)
NCR		806	722 ↓(-10)
CALABARZON (IV-A)		603	538 ↓(-11)
MIMAROPA (IV-B)		506	474 ↓(-6)
VISAYAS			
CENTRAL VISAYAS (VII)		438	426 ↓(-3)
EASTERN VISAYAS (VIII)		373	354 ↓(-5)
WESTERN VISAYAS (VI)		563	584 ↓(4)
MINDANAO			
BARMM		243	264 ↓(9)
ZAMBOANGA PENINSULA (IX)		399	389 ↓(-3)
CARAGA		438	426 ↓(-3)
NORTHERN MINDANAO (X)		394	378 ↓(-4)
DAVAO (XI)		393	381 ↓(-3)
SOCCSKSARGEN (XII)		478	444 ↓(-7)
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

TB Treatment Success Rate

TB Treatment Success Rate refers to the number of New and Relapse TB cases that were cured or completed treatment out of all those that were notified for the year.

Region	TB Treatment Success Rate (%)		
	2023 Results	2024 Results	
National Average	92.59	93.31	↑
NORTHERN AND CENTRAL LUZON			
CAR	91.46	92.64	↑
ILOCOS (I)	95.01	95.75	↑
CENTRAL LUZON (III)	93.14	94.09	↑
CAGAYAN VALLEY (II)	93.99	95.47	↑
NCR AND SOUTHERN LUZON			
BICOL (V)	91.22	91.75	↑
NCR	91.96	92.80	↑
CALABARZON (IV-A)	93.43	93.95	↑
MIMAROPA (IV-B)	90.21	92.39	↑
VISAYAS			
CENTRAL VISAYAS (VII)	90.56	89.51	↓
EASTERN VISAYAS (VIII)	90.99	92.05	↑
WESTERN VISAYAS (VI)	95.14	95.50	↑
MINDANAO			
BARMM	87.26	84.36	↓
ZAMBOANGA PENINSULA (IX)	92.56	94.43	↑
CARAGA	94.84	95.13	↑
NORTHERN MINDANAO (X)	93.51	95.46	↑
DAVAO (XI)	90.16	92.01	↑
SOCCSKSARGEN (XII)	94.17	95.09	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

Presence of approved Provincial/City/ Municipal Nutrition Action Plan with Target

The Local Nutrition Action Plan (LNAP) is a three-year plan containing the objectives and nutrition interventions that will be implemented to address the malnutrition problem at the provincial, city, municipal and barangay level. The LNAP operationalizes the Philippine Plan of Action for Nutrition (PPAN) and Regional Nutrition Action Plan (RNAP) at the local level. The P/C/M/BNAP is prepared by the respective nutrition committees and should be an integral part of the local development plan and funded in the Annual Investment Program (AIP). The plan is updated yearly to reflect the results of OPT Plus.

*For 2025 LGU HSC, ongoing data collection/validation

Region		Percentage of Presence of Approved Provincial / City / Municipal Nutrition Action Plan		
		2023 Results	2024 Results	
National Average		48	65	↑
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)		8	83	↑
CAGAYAN VALLEY (II)		55	83	↑
CENTRAL LUZON (III)		26	72	↑
CAR		36	73	↑
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)		12	50	↑
BICOL (V)		88	70	↓
NCR		100	100	=
MIMAROPA (IV-B)		64	71	↑
VISAYAS				
WESTERN VISAYAS (VI)		39	47	↑
CENTRAL VISAYAS (VII)		75	66	↓
EASTERN VISAYAS (VIII)		87	87	=
MINDANAO				
ZAMBOANGA PENINSULA (IX)		64	35	↓
NORTHERN MINDANAO (X)		52	74	↑
DAVAO (XI)		50	83	↑
SOCCSKSARGEN (XII)		60	89	↑
BARMM		0	0	=
CARAGA		69	77	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result		
	↓	2024 Performance result is DECLINED compared to the 2023 performance results		
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change		
	—	No Comparative Data From 2023		
	N/A	Not Applicable		

LGU HSC, 2024

Presence of Nutrition Action Officer with at least two (2) staff complement

A full-time or part time designate by the local chief executive either from a separate nutrition office or from among the sectoral departments of the local government unit with at least one technical staff and one support staff.

The Nutrition Action Officer (NAO) serves as focal point for nutrition and coordinates with the local nutrition committee to facilitate the implementation of the LNAP.

NAO should have at least two (2) staff complement (technical and administrative staff support).

*For 2025 LGU HSC,
ongoing data
collection/validation

Region	Percentage of Presence of Nutrition Action Officer with Staff Complement		
	Province	HUC, ICC	City
National Average	34	74	51
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	0	100	8
CAGAYAN VALLEY (II)	40	100	76
CENTRAL LUZON (III)	43	50	77
CAR	100	100	99
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	80	100	70
BICOL (V)	33	100	42
NCR	0	100	100
MIMAROPA (IV-B)	100	100	81
VISAYAS			
WESTERN VISAYAS (VI)	0	100	16
CENTRAL VISAYAS (VII)	0	33	31
EASTERN VISAYAS (VIII)	0	0	23
MINDANAO			
ZAMBOANGA PENINSULA (IX)	0	0	25
NORTHERN MINDANAO (X)	40	50	63
DAVAO (XI)	40	0	63
SOCCKSARGEN (XII)	50	0	79
BARMM	0	0	0
CARAGA	0	100	11

LGU HSC, 2024

Proportion of households using safely managed drinking-water services

Refers to the number of households using safely managed drinking-water services meeting the following 3 criteria. (1) located inside the household or within its premises; (2) available at least 12 hours per day and (3) the water supplied should be free of fecal contamination (optional: priority chemical), among the total number of households expressed as a percentage.

Region	Households Using Safely Managed Drinking Water Services / Sources (%)		
	2023 Results	2024 Results	
National Average	67.36	71.27	↑
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	62.44	76.09	↑
CAGAYAN VALLEY (II)	59.19	63.56	↑
CENTRAL LUZON (III)	91.61	92.53	↑
CAR	62.54	67.84	↑
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	82.73	85.22	↑
BICOL (V)	47.34	52.28	↑
NCR	93.56	94.13	↑
MIMAROPA (IV-B)	49.39	42.19	↓
VISAYAS			
WESTERN VISAYAS (VI)	54.94	58.85	↑
CENTRAL VISAYAS (VII)	58.83	58.79	↓
EASTERN VISAYAS (VIII)	48.69	56.56	↑
MINDANAO			
ZAMBOANGA PENINSULA (IX)	47.35	60.40	↑
NORTHERN MINDANAO (X)	51.20	56.93	↑
DAVAO (XI)	64.49	78.59	↑
SOCCKSARGEN (XII)	49.72	55.24	↑
BARMM	20.14	20.07	↓
CARAGA	54.59	58.75	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

Proportion of Households using safely managed sanitation services

Refers to the number of HHs using safely managed sanitation services among the total number of HHs expressed as a percentage.

Safely Managed Sanitation Services:

- (1) sanitation facility is not shared with other HHs and
- (2) the sewage/excreta should either be:
 - stored in a containment tank and treated (in situ) and application of sanitation by products for reuse/disposal OR
 - stored in a containment tank desludged, transported, treated and disposed off-site and application of sanitation by-products for reuse/disposal OR
 - stored in a containment tank or conveyed through a sewer/sewerage system and treated off-site and application of sanitation by-products for reuse/disposal

*For 2025 LGU HSC, ongoing data collection/validation

Region		Households Using Safely Managed Sanitation Services / Sources (%)		
		2023 Results	2024 Results	
National Average		63.74	68.83	↑
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)		34.64	35.93	↑
CAGAYAN VALLEY (II)		85.66	89.43	↑
CENTRAL LUZON (III)		89.94	92.58	↑
CAR		90.23	91.42	↑
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)		82.23	83.64	↑
BICOL (V)		57.33	65.54	↑
NCR		70.17	91.46	↑
MIMAROPA (IV-B)		40.40	9.12	↓
VISAYAS				
WESTERN VISAYAS (VI)		69.78	75.80	↑
CENTRAL VISAYAS (VII)		73.63	69.75	↓
EASTERN VISAYAS (VIII)		21.97	26.72	↑
MINDANAO				
ZAMBOANGA PENINSULA (IX)		47.21	68.77	↑
NORTHERN MINDANAO (X)		17.83	16.46	↓
DAVAO (XI)		77.10	89.10	↑
SOCCSKSARGEN (XII)		70.44	74.03	↑
BARMM		9.82	14.78	↑
CARAGA		0.00	7.37	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result		
	↓	2024 Performance result is DECLINED compared to the 2023 performance results		
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change		
	-	No Comparative Data From 2023		
	N/A	Not Applicable		

LGU HSC, 2024

Percentage of LGUs with localized MH Program

Percentage of LGUs with localized MH Program

- Local Ordinance/ Executive Order or similar regulation
- A coordinating body
- Complementary Personnel – composed of one (1) MHO and (1) nurse or allied health professional
- Complementary personnel serving as MH service providers trained on MHGap
- Promotion and awareness campaign plans
- All RHUs providing MNS services
- All Primary health care facilities accredited by PhilHealth providing MNS services
- Appropriate allocation of Mental, Neurologic and Substance-Use related commodities
- Budget Allocation for the implementation of MH Program

*For 2025 LGU HSC,
ongoing data
collection/validation

Region		Percentage of LGUs with Localized MH Program, 2024		
		2023 Results	2024 Results	
National Average		14	9	↓
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)		25	2	↓
CAGAYAN VALLEY (II)		29	4	↓
CENTRAL LUZON (III)		8	9	↑
CAR		16	23	↑
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)		7	6	↓
BICOL (V)		13	5	↓
NCR		0	0	=
MIMAROPA (IV-B)		9	6	↓
VISAYAS				
WESTERN VISAYAS (VI)		23	19	↓
CENTRAL VISAYAS (VII)		14	14	=
EASTERN VISAYAS (VIII)		5	1	↓
MINDANAO				
ZAMBOANGA PENINSULA (IX)		33	31	↓
NORTHERN MINDANAO (X)		5	3	↓
DAVAO (XI)		13	6	↓
SOCCSKSARGEN (XII)		9	8	↓
BARMM		8	6	↓
CARAGA		8	10	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result		
	↓	2024 Performance result is DECLINED compared to the 2023 performance results		
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change		
	-	No Comparative Data From 2023		
	N/A	Not Applicable		

LGU HSC, 2024

Region	Percentage of LGUs with Localized MH Program, 2024								
	1. presence and child coordinating	2. Community coordinating (Barangay/ Local coordinating Program)	3. complementary Infant and Children Health Immunization	4. serving and vape use prevention and control	5. promotion access of minors to alcoholic beverages	6. allrhu and sanitation	7. primary health serving (primary Health Law)	8. appropriate and injury prevention (Gender-based appropriate, appropriate against women, and appropriate against children)	9. budget mechanisms for Barangay Health Workers
National Average	41	73	67	85	75	89	9	91	87
NORTHERN AND CENTRAL LUZON									
ILOCOS (I)	71	73	69	93	72	90	5	91	87
CAGAYAN VALLEY (II)	14	76	71	74	82	91	5	92	91
CENTRAL LUZON (III)	34	73	64	72	68	69	15	69	74
CAR	71	70	69	88	88	88	30	81	87
NCR AND SOUTHERN LUZON									
CALABARZON (IV-A)	50	54	71	82	73	84	12	78	82
BICOL (V)	43	64	65	77	74	88	7	78	77
NCR	82	88	94	94	76	82	6	88	88
MIMAROPA (IV-B)	54	56	58	76	56	78	19	82	81
VISAYAS									
WESTERN VISAYAS (VI)	68	64	76	84	67	89	29	87	84
CENTRAL VISAYAS (VII)	75	82	85	81	81	90	19	90	90
EASTERN VISAYAS (VIII)	10	64	66	83	55	97	3	89	89
MINDANAO									
ZAMBOANGA PENINSULA (IX)	72	75	91	91	79	92	41	88	84
NORTHERN MINDANAO (X)	81	72	78	83	71	82	7	81	78
DAVAO (XI)	24	72	69	80	76	81	9	85	89
SOCCSKSARGEN (XII)	62	64	68	72	53	83	13	81	77
BARMM	34	34	45	74	51	67	25	52	42
CARAGA	69	78	64	86	67	81	14	77	71

*For 2025 LGU HSC,
ongoing data
collection/validation

LGU HSC, 2024

EMR utilization for digitalized service delivery at public primary care facilities

To pass the indicator, public primary care facilities (i.e. Rural Health Units and Health Centers only) in the City/Municipality shall conform with ALL parameters:

1. Presence of Electronic Medical Record at ALL Rural Health Units/Health Centers/
2. Digitalization of Work Process: The facility integrated EMR usage into their daily workflows of delivering the services.
3. Statistical Report Generation: FHSIS reports fully generated using the EMR.

*For 2025 LGU HSC, ongoing data collection/validation

Region	EMR utilization for digitalized service delivery at public primary care facilities (%)		
	2023 Results	2024 Results	
National Average	14.07	30.36	—
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	63.58	40.63	—
CAGAYAN VALLEY (II)	14.43	48.94	—
CENTRAL LUZON (III)	6.05	43.15	—
CAR	2.17	18.81	—
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	8.66	21.52	—
BICOL (V)	9.65	17.97	—
NCR	53.96	84.77	—
MIMAROPA (IV-B)	3.80	10.63	—
VISAYAS			
WESTERN VISAYAS (VI)	0.00	22.22	—
CENTRAL VISAYAS (VII)	11.24	52.76	—
EASTERN VISAYAS (VIII)	15.83	47.62	—
MINDANAO			
ZAMBOANGA PENINSULA (IX)	6.26	40.39	—
NORTHERN MINDANAO (X)	19.95	68.28	—
DAVAO (XI)	50.00	95.83	—
SOCSESKSARGEN (XII)	3.70	0.00	—
BARM	5.43	4.78	—
CARAGA	2.51	28.00	—
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	—	No Comparative Data From 2023	
	N/A	Not Applicable	

LGU HSC, 2024

Percent of Local Government Units with institutionalized Disaster Risk Reduction and Management in Health (DRRM-H) System

Institutionalized DRRM-H System means having all the four (4) minimum requirements for the Province/City/Municipality:

1. Approved, updated, disseminated and tested Disaster Risk Reduction and Management in Health (DRRM-H) Plan
2. Organized and trained Health Emergency Response Team on minimum required trainings: Basic Life Support (BLS), Standard First Aid (SFA)
3. Available and accessible within 24 hours post impact of emergency or disaster essential health emergency commodities e.g. medicines such as anti-infectives, analgesics, antipyretics, fluids/ electrolytes, respiratory drugs, dietary/nutritional products
4. Health Operations Center or Emergency Operations Center, functional (1) Command and Control, (2) Coordination, (3) and Communication

*For 2025 LGU HSC, ongoing data collection/validation

Region		Percentage of LGUs with Institutionalized DRRM-H System		
		2023 Results	2024 Results	
National Average		68	78	↑
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)		88	94	↑
CAGAYAN VALLEY (II)		63	81	↑
CENTRAL LUZON (III)		97	100	↑
CAR		71	86	↑
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)		84	90	↑
BICOL (V)		31	51	↑
NCR		100	100	=
MIMAROPA (IV-B)		26	46	↑
VISAYAS				
WESTERN VISAYAS (VI)		66	81	↑
CENTRAL VISAYAS (VII)		68	79	↑
EASTERN VISAYAS (VIII)		91	97	↑
MINDANAO				
ZAMBOANGA PENINSULA (IX)		88	93	↑
NORTHERN MINDANAO (X)		76	87	↑
DAVAO (XI)		56	63	↑
SOCCSKSARGEN (XII)		64	57	↓
BARMM		26	29	↑
CARAGA		68	83	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result		
	↓	2024 Performance result is DECLINED compared to the 2023 performance results		
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change		
	-	No Comparative Data From 2023		
	N/A	Not Applicable		

LGU HSC, 2024

Region	Percentage of LGUs with DRRM-H Components, 2024			
	DRRM-H Plan	Organized and Trained Hert	Essential Health Emergency Commodities	Emergency Operation Center
National Average	94	93	97	96
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)	97	96	98	98
CAGAYAN VALLEY (II)	88	87	96	92
CENTRAL LUZON (III)	100	100	100	100
CAR	93	89	99	94
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)	93	95	97	96
BICOL (V)	65	69	77	70
NCR	100	100	100	100
MIMAROPA (IV-B)	50	68	86	88
VISAYAS				
WESTERN VISAYAS (VI)	88	89	91	93
CENTRAL VISAYAS (VII)	85	93	94	91
EASTERN VISAYAS (VIII)	97	99	100	99
MINDANAO				
ZAMBOANGA PENINSULA (IX)	97	96	99	97
NORTHERN MINDANAO (X)	92	88	92	90
DAVAO (XI)	76	70	87	76
SOCCSKSARGEN (XII)	77	60	87	85
BARMM	34	40	43	42
CARAGA	88	86	91	87

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

	DRRM-H Plan			Organized and Trained Hert			Essential Health Emergency Commodities			Emergency Operation Center		
	2023 Results	2024 Results		2023 Results	2024 Results		2023 Results	2024 Results		2023 Results	2024 Results	
National Average	77	83	↑	76	85	↑	89	90	↑	81	88	↑
NORTHERN AND CENTRAL LUZON												
ILOCOS (I)	90	97	↑	93	96	↑	97	98	↑	95	98	↑
CAGAYAN VALLEY (II)	74	88	↑	72	87	↑	95	96	↑	73	92	↑
CENTRAL LUZON (III)	100	100	▬	97	100	↑	100	100	▬	100	100	▬
CAR	84	93	↑	73	89	↑	99	99	▬	88	94	↑
NCR AND SOUTHERN LUZON												
CALABARZON (IV-A)	87	93	↑	88	95	↑	95	97	↑	90	96	↑
BICOL (V)	59	65	↑	40	69	↑	75	77	↑	60	70	↑
NCR	100	100	▬	100	100	▬	100	100	▬	100	100	▬
MIMAROPA (IV-B)	32	50	↑	55	68	↑	76	86	↑	72	88	↑
VISAYAS												
WESTERN VISAYAS (VI)	81	88	↑	78	89	↑	95	91	↓	86	93	↑
CENTRAL VISAYAS (VII)	77	85	↑	82	93	↑	96	94	↓	88	91	↑
EASTERN VISAYAS (VIII)	91	97	↑	93	99	↑	95	100	↑	94	99	↑
MINDANAO												
ZAMBOANGA PENINSULA (IX)	92	97	↑	96	96	▬	96	99	↑	89	97	↑
NORTHERN MINDANAO (X)	80	92	↑	79	88	↑	84	92	↑	78	90	↑
DAVAO (XI)	76	76	▬	59	70	↑	91	87	↓	76	76	▬
SOCCSKSARGEN (XII)	77	77	▬	68	60	↓	83	87	↑	72	85	↑
BARMM	39	34	↓	43	40	↓	54	43	↓	37	42	↑
CARAGA	74	88	↑	77	86	↑	88	91	↑	76	87	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result										
	↓	2024 Performance result is DECLINED compared to the 2023 performance results										
	▬	2024 Performance result is EQUAL compared to the 2023 performance results, no change										
	—	No Comparative Data From 2023										
	N/A	Not Applicable										

*For 2025 LGU HSC, ongoing data collection/ validation

LGU HSC, 2024

Presence of a Functioning Epidemiology and Surveillance Unit (ESU)

A Functional ESU is characterized by successfully fulfilling the minimum indicators specified for a Functional ESU in the Integrated Monitoring and Evaluation Tool (IMET), indicating the operational readiness and capacity of the ESU to effectively carry out its surveillance and response functions.

Region		Presence of a Functional Epidemiology and Surveillance Unit (%)		
		2023 Results	2024 Results	
National Average		35.83	68.07	↑
NORTHERN AND CENTRAL LUZON				
ILOCOS (I)		60.00	60.00	=
CAGAYAN VALLEY (II)		83.33	83.33	=
CENTRAL LUZON (III)		11.11	77.78	↑
CAR		0.00	71.43	↑
NCR AND SOUTHERN LUZON				
CALABARZON (IV-A)		66.67	83.33	↑
BICOL (V)		0.00	0.00	=
NCR		31.25	68.75	↑
MIMAROPA (IV-B)		50.00	80.00	↑
VISAYAS				
WESTERN VISAYAS (VI)		0.00	100.00	↑
CENTRAL VISAYAS (VII)		85.71	85.71	=
EASTERN VISAYAS (VIII)		87.50	75.00	↓
MINDANAO				
ZAMBOANGA PENINSULA (IX)		0.00	100.00	↑
NORTHERN MINDANAO (X)		57.14	42.86	↓
DAVAO (XI)		66.67	66.67	=
SOCCSKSARGEN (XII)		20.00	100.00	↑
BARMM		0.00	0.00	=
CARAGA		0.00	83.33	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result		
	↓	2024 Performance result is DECLINED compared to the 2023 performance results		
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change		
	—	No Comparative Data From 2023		
	N/A	Not Applicable		

*For 2025 LGU HSC, ongoing data collection/validation

LGU HSC, 2024

Presence of a Functioning Epidemiology and Surveillance Unit (ESU)

A Functioning ESU is characterized by successfully meeting the minimum indicators specified in the Seal of Good Local Governance - Health Compliance and Responsiveness, demonstrating the operational readiness and capacity of the ESU to effectively carry out its surveillance and response functions. A Functioning ESU is defined as having passed all 5 components of the ESU.

1. Presence of a signed Ordinance/Executive Order/Sangguniang Panlalawigan for the creation of the ESU
2. Presence of a signed policy issuance designating or appointment letter hiring at least one Disease Surveillance Officer and one epidemiology assistant of an allied health profession for the ESU
3. Existence of a list of dedicated ESU surveillance staff with training certificates issued by the Epidemiology Bureau (EB) or Regional ESU (RESU) in Basic Epidemiology. Attachment: List of ESU Surveillance Personnel with attached copies of Basic Epidemiology training certificates
4. Presence of budget allocation. (Attachment: Copy of the ordinance or work and financial plan or Local Investment Plan for Health (LIPH) or Annual Operations Plan (AOP))
5. Submits and encodes timely reports (FHSIS, category I notifiable diseases, and category II notifiable diseases)

*For 2025 LGU HSC,
ongoing data collection/validation

Region	Presence of a Functional Epidemiology and Surveillance Unit (%)		
	2023 Results	2024 Results	
National Average	35.83	68.07	↑
NORTHERN AND CENTRAL LUZON			
ILOCOS (I)	60.00	60.00	=
CAGAYAN VALLEY (II)	83.33	83.33	=
CENTRAL LUZON (III)	11.11	77.78	↑
CAR	0.00	71.43	↑
NCR AND SOUTHERN LUZON			
CALABARZON (IV-A)	66.67	83.33	↑
BICOL (V)	0.00	0.00	=
NCR	31.25	68.75	↑
MIMAROPA (IV-B)	50.00	80.00	↑
VISAYAS			
WESTERN VISAYAS (VI)	0.00	100.00	↑
CENTRAL VISAYAS (VII)	85.71	85.71	=
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NORTHERN MINDANAO (X)	57.14	42.86	↓
DAVAO (XI)	66.67	66.67	=
SOCCKSARGEN (XII)	20.00	100.00	↑
BARMM	0.00	0.00	=
CARAGA	0.00	83.33	↑
Internal Benchmark Scores	↑	2024 Performance result IMPROVED compared to 2023 performance result	
	↓	2024 Performance result is DECLINED compared to the 2023 performance results	
	=	2024 Performance result is EQUAL compared to the 2023 performance results, no change	
	-	No Comparative Data From 2023	
	N/A	Not Applicable	

SEAL OF GOOD LOCAL GOVERNANCE (SGLG) HEALTH COMPLIANCE AND RESPONSIVENESS AREA (HCR)

PROGRAM PROFILE

► PROGRAM/ACTIVITY/PROJECT DESCRIPTION

SGLG is an award, incentive, honor and recognition-based program for all LGUs, and is a continuing commitment for LGUs to continually progress and improve their performance. (Section 3 of Republic Act 11292 or The Seal of Good Local Governance Act of 2019).

HEALTH COMPLIANCE AND RESPONSIVENESS AREA (HCR) is one of the Governance Areas under the SGLG that the LGU must pass.

NOTE: SGLG is undergoing reprogramming per directive of DILG. Awarding will now be term-based.

SEAL OF GOOD LOCAL GOVERNANCE (SGLG) HEALTH COMPLIANCE AND RESPONSIVENESS AREA (HCR)

SGLG Criteria: Health Compliance and Responsiveness (Rule VI, Section 4)

“This refers to the LGU's capability to **advocate, effectively implement and deliver health services and information systems**, as well as, pursue evidenced-based **health promotion and protection policies** scientifically proven and accepted to advance population health and individual well-being, reduce the prevalence of **non-communicable diseases and its risk factors**, lower the incidence of **infectious diseases**, address **mental health issues** and improve health status throughout the life course.”

SEAL OF GOOD LOCAL GOVERNANCE (SGLG)

HEALTH COMPLIANCE AND RESPONSIVENESS AREA (HCR)

SGLG Criteria: Health Compliance and Responsiveness (Rule VI, Section 4)

Indicators shall include, but are not limited to:

- a) Ensuring **access to nutritious, safe and affordable food**;
- b) Regularly conducting **physical activities in communities and schools**;
- c) Strictly enforcing laws and ordinances that **regulate tobacco sale and use, harmful alcohol consumption, and use of illicit drugs**;
- d) Providing access to **safe water and sanitation**;
- e) Implementing programs for **immunization, prevention and control of communicable diseases, and health emergency management**;
- f) Ensuring access to essential health commodities and primary health care including **prevention programs for hypertension, diabetes, cancer, and other non-communicable diseases**, and programs that **promote mental health**; and
- g) Establishing **local surveillance and epidemiology systems** to monitor health service delivery.

SGLG HCR Percent of Passers (2024 data)

LGU Level	No. of Passers	Percentage of Passers
Province	66	80%
HUC-ICC	37	97%
Municipality-CC	1460	91%

SGLG HCR Criteria & Percent of Passers

PROGRAM
PROFILE

2023*

2024**

1. With complete Local Investment Plan for Health (LIPH)
2. Proportion of Households Using Safely Managed Drinking-water services
3. Proportion of Households using Safely Managed Sanitation Services
4. TB Case Notification Rate
5. TB Treatment Success Rate
6. Prevalence of Stunting among children under 5 years old
7. Institutionalized Disaster Risk Reduction Management in Health (DRRM-H) System
8. Functional Local Epidemiology and Surveillance Unit (ESU)

1. 2023 Indicators PLUS the following:
2. Fully-immunized child (FIC) Coverage
3. Proportion of pregnant women with at least 4 prenatal check -ups
4. Functional Local Health Board
5. Provision of FULL hazard pay, subsistence and laundry allowances to permanent public health workers under the Magna Carta for Public Health Workers
6. Percentage of adults 20 years old and above who were risk assessed using the PhilPEN protocol
7. Number of Accredited Konsulta Provider

* Bonus Point- if the LGU has documented Good Practice in Health Approved and Endorsed by the CHD

Criteria for Passing:

Province/HUC/ICC: LIPH Plus Two (2) other indicators
Municipality/CC: LIPH Plus One (1) other indicators

Criteria for Passing:

Province/HUC/ICC: LIPH Plus Six (6) other indicators
Municipality/CC: LIPH Plus Four (4) other indicators

Percentage of Passers (*2021 data):

Provinces: 88% HUCs/ICCs: 97%



Percentage of Passers (**2022 data):

Provinces: 84% HUCs/ICCs: 95%
88%

Munis/CCs:



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2026 SUBMITTED INDICATORS FOR THE REPROGRAMMED SEAL OF GOOD LOCAL GOVERNANCE (SGLG) HEALTH COMPLIANCE AND RESPONSIVENESS AREA (HCRA)

FOUNDATIONAL (INPUT) Indicators

1. Presence of Health Plan
 - a. LIPH inclusive of investments for priority health outcomes
 - b. Annual Operation Plan inclusive of budget for priority health outcomes
2. Presence of Functional Local Health Board

2026 SUBMITTED INDICATORS FOR THE REPROGRAMMED SEAL OF GOOD LOCAL GOVERNANCE (SGLG) HEALTH COMPLIANCE AND RESPONSIVENESS AREA (HCRA)

RUNNING (OUTPUT) Indicators

1. Accreditation of Healthcare Facilities to provide special benefit packages
2. Presence of functional ESU and FHSIS
3. Institutionalized DRRM-H System
4. Operation Timbang (OPT) plus coverage:
 - a. Province
 - b. City
 - c. Municipality

2026 SUBMITTED INDICATORS FOR THE REPROGRAMMED SEAL OF GOOD LOCAL GOVERNANCE (SGLG) HEALTH COMPLIANCE AND RESPONSIVENESS AREA (HCRA)

OUTCOME Indicators

1. Percentage of FIC

2. Prevalence of Malnutrition among children below five (5) years of age:

a. Stunting

b. Wasting

c. Overweight and Obese

3. First Patient Encounter

**LEGAL AND POLICY BASIS****Mandate Creating Program/Year:**

- A.O. No. 2008-0006: Integrating Replication Strategies in DOH Operations
- A.O. No. 2021-0061: Guidelines on the Identification, Documentation, and Replication of Local Government Unit Good Practices in Health

Goal:

- Provide an environment that promotes the exchange of knowledge and good practices among the levels of the health care delivery system

Objectives:

- Provide guidance to help our local implementers in the health sector in localizing Health Programs in the context of their communities through the implementation and replication of documented good practices.
- Identify, document, replicate and contextualize evidence-based interventions in the community setting through combination of intervention, capability development, technical assistance provision, and other strategies aimed at sustainability of interventions through promoting buy-ins among local health managers, community and other key stakeholders.

Strategies:

- The identification, documentation and replication of good practices in health shall be a monitoring and knowledge management strategy and shall be one of the references on the development of Local Investment Planning for Health (LIPH)/ Annual Operation Plan (AOP)/ Annual Investment Plan (AIP) by the LGUs.
- Shall serve as reference for strategic and program planning, implementation research, program implementation reviews, capacity building activities, or as a criterion for recognition.



KEY PROCESSES AND IMPLEMENTATION ARRANGEMENTS

Describe the major steps, workflows, or mechanisms involved in implementing the PAP. Include details such as planning cycles, coordination with partners or stakeholders, fund flow, monitoring processes, or reporting arrangements.

1. Identification of Good Practices in Health using criteria in AO 2021-0061
2. Documentation
3. Replication

Criteria:

- LGU-initiated (mandatory)
- Technically feasible
- Effective and successful
- Innovative
- Responsive
- Sustainable
- Culturally sensitive
- Gender sensitive

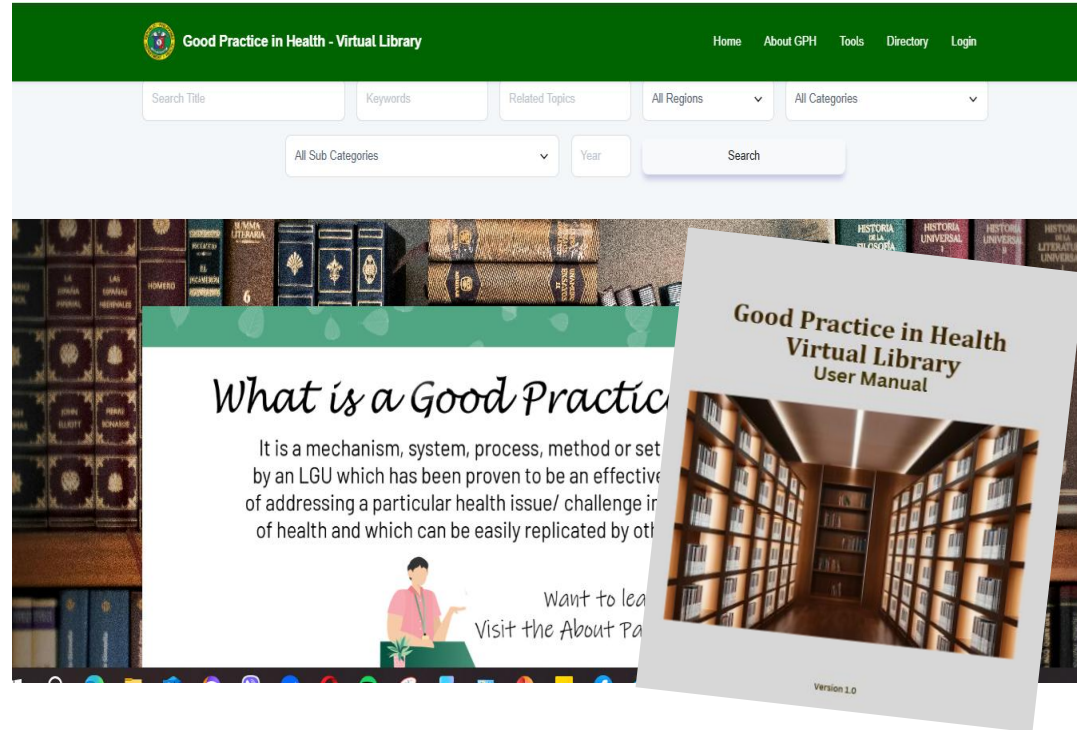
BLHSD

Good Practices in Health

PROGRAM PROFILE

Status:

- Ongoing uploading of Good Practice in Health in Virtual Library
- DC 2026-0170 User Manual
- Ongoing submission of CHDs/MOH-BARMM of validated Good Practice in Health
- Continuous advocacy and technical assistance to LGUs to document Good Practice in Health



Number of Submitted Good Practice per Region as of August 19, 2026

REGION	COMPLETE	INCOMPLETE
ILOCOS	3	0
CAGAYAN	2	1
CENTRAL LUZON	11	0
CALABARZON	4	0
BICOL	1	1
WESTERN VISAYAS	1	18
CENTRAL VISAYAS	2	12
NORTHERN MINDANAO	0	58
DAVAO	7	0
SOCCSKSARGEN	12	0
TOTAL	43	90